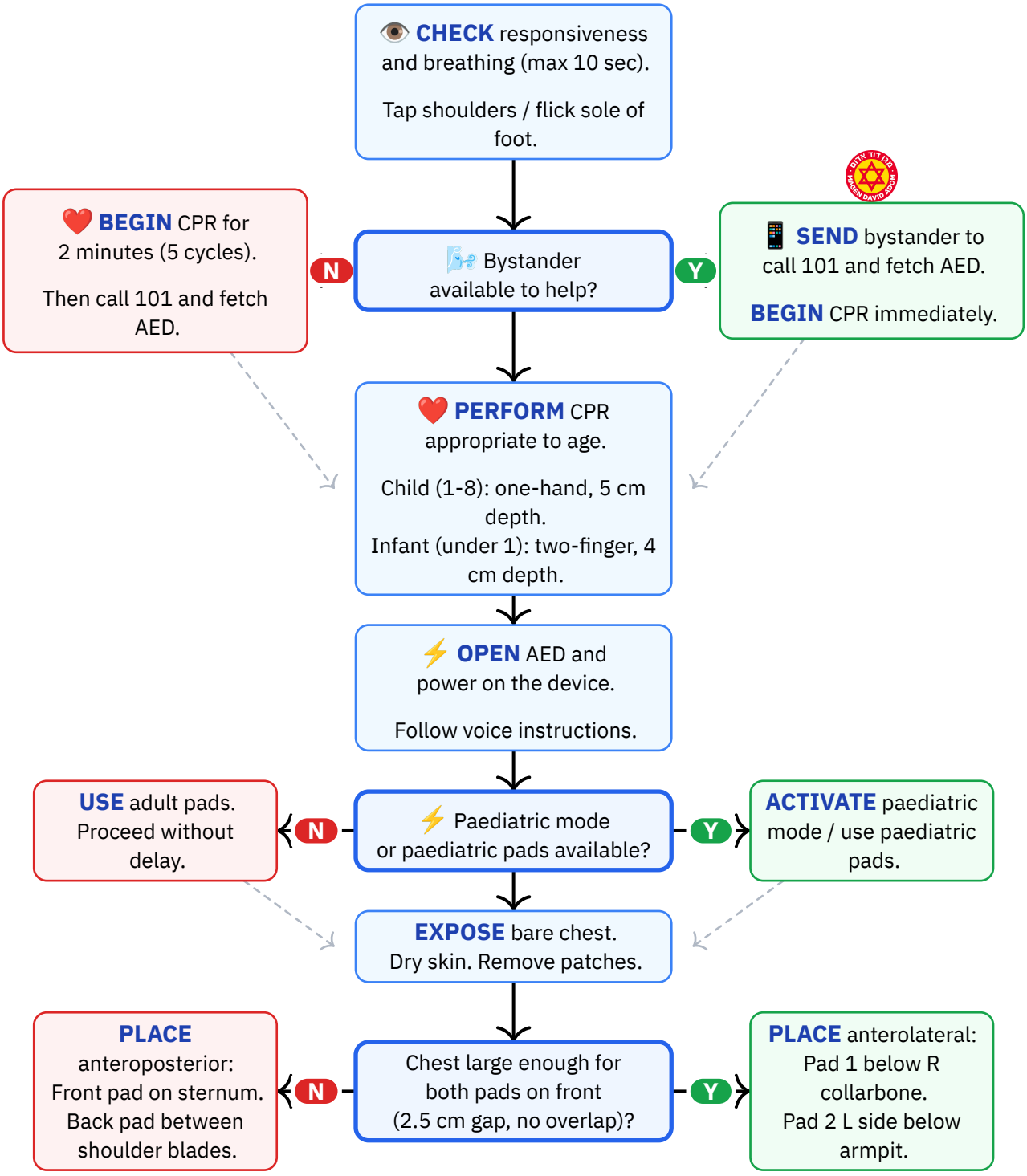
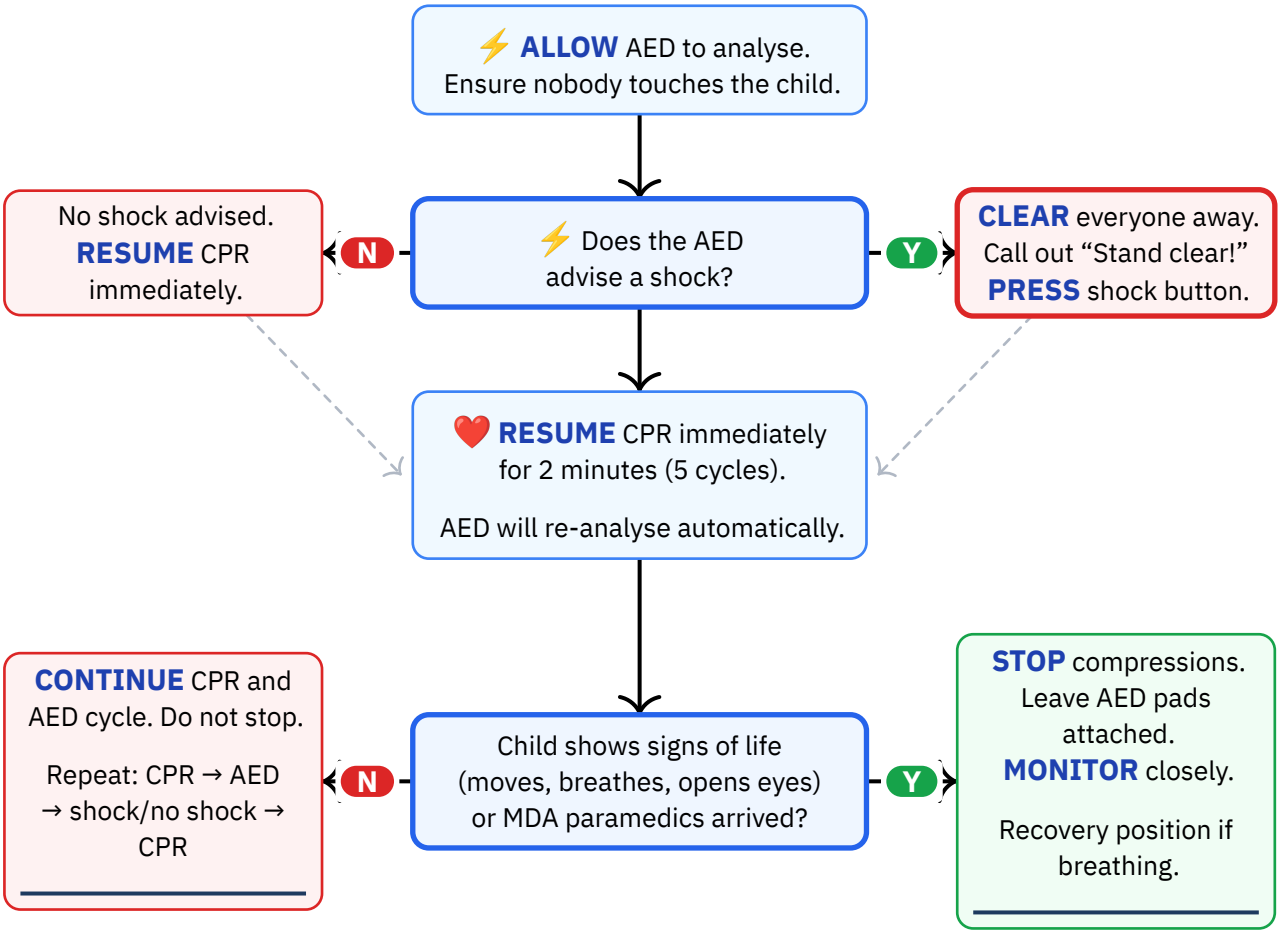


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AED Analysis & Shock Cycle



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Reference Notes

DO NOT:

✕ **Do NOT use an AED on a conscious, normally-breathing child** – AEDs are only for cardiac arrest.
✕ **Do NOT use an AED in water** – move the child to dry ground first before applying pads.
✕ **Do NOT delay defibrillation** because paediatric pads are unavailable – use adult pads with anteroposterior placement.
✕ **Do NOT allow pads to touch or overlap** – use anteroposterior placement if the chest is too small.
✕ **Do NOT touch the child** during AED rhythm analysis or shock delivery.
✕ **Do NOT turn off the AED or remove pads** once applied, even if the child regains consciousness.
✕ **Do NOT perform adult-depth compressions** on a child or infant – compress to one-third of chest depth only.
✕ **Do NOT use an AED near flammable gases** or oxygen-enriched atmospheres.

Equipment needed:

— AED (Automated External Defibrillator)
— Paediatric AED pads (preferred for children under 8 / under 25 kg)
— Adult AED pads (if paediatric pads unavailable)
— Scissors or shears (to cut away clothing)
— Towel or cloth (to dry the chest)

Hebrew Terminology — מונחים בעברית

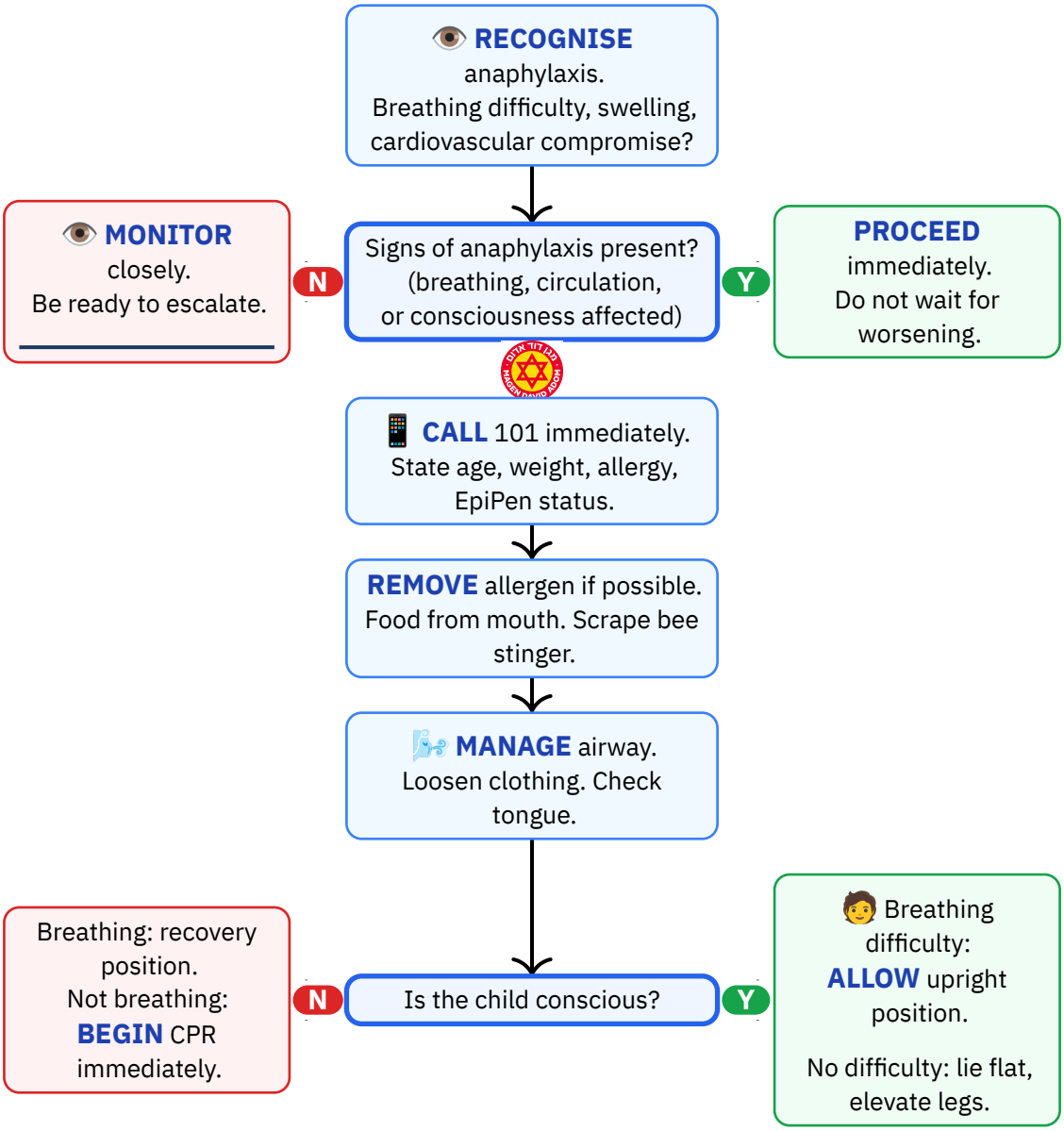
English	עברית	Pronunciation
Defibrillator	דפיברילטור	de-fib-ri-LA-tor
AED	מכשיר החייאה	makh-SHIR ha-kha-ya-A
Electrode pads	אלקטרודות	e-lek-TRO-dot
Shock	הלם / שוק	HE-lem / shok
Cardiac arrest	דום לב	dom lev
CPR	החייאה	ha-kha-ya-A
Stand clear!	התרחק!	hit-ra-KHU!
Child	ילד / ילדה	YE-led / yal-DA
Infant	תינוק / תינוקת	ti-NOK / ti-NO-ket
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Not breathing	לא נושם	lo no-SHEM

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Source: Magen David Adom (MDA) — AED Operation for Children and Infants (AHA 2025 Guidelines as adopted by MDA)
URL: https://www.mdais.org/kursim/defibrillator
Imported: 2026-03-16 · **Last verified:** 2026-03-16

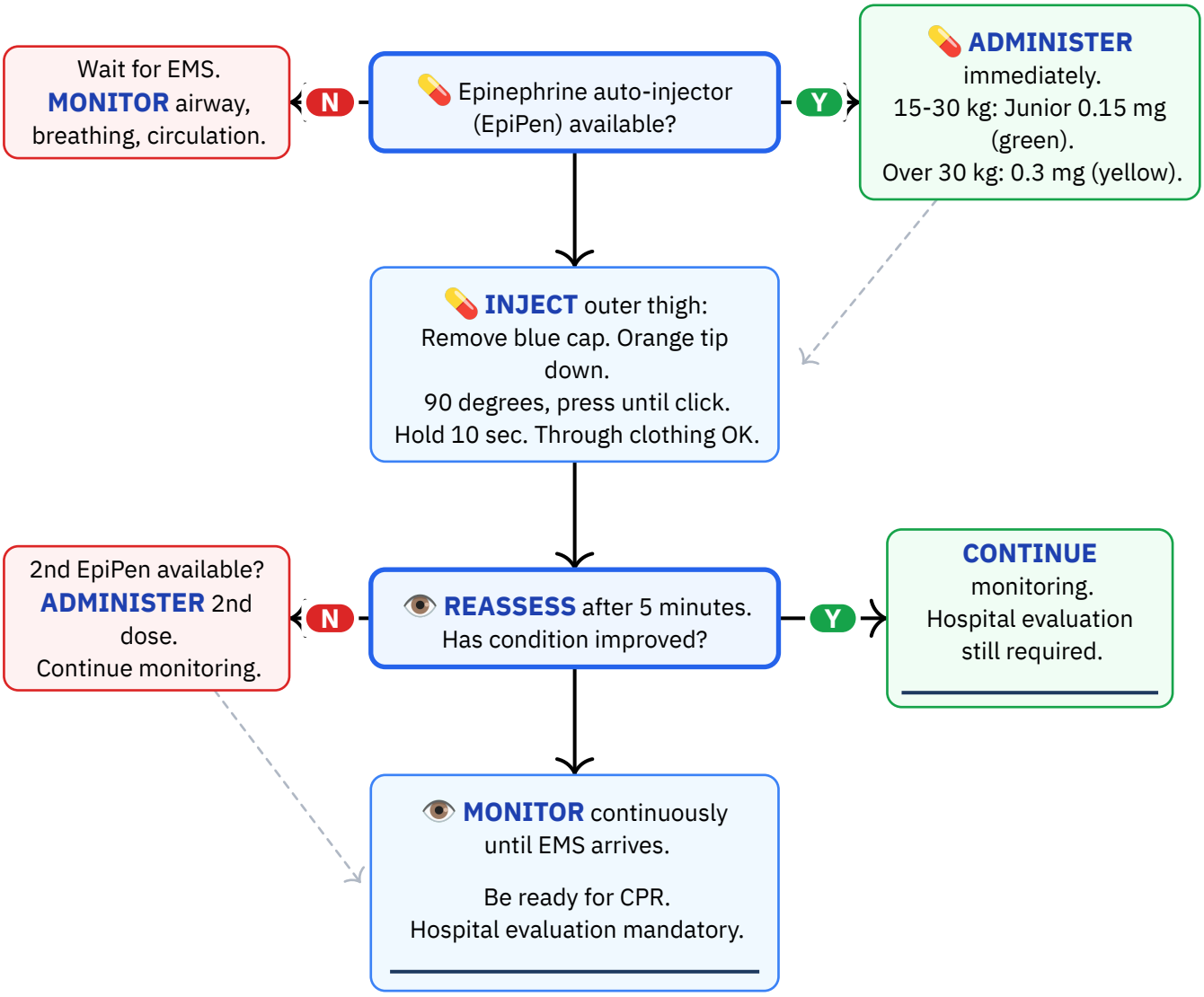
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EpiPen Administration & Monitoring



EpiPen Dosing by Weight

15-30 kg: EpiPen Junior 0.15 mg (green label)
Over 30 kg: EpiPen 0.3 mg (yellow label)
Under 15 kg: No auto-injector for this range, but 0.15 mg Junior is still recommended over withholding treatment.
Note: Some Israeli allergists prescribe 0.3 mg from 20 kg – follow individual prescription.

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Reference Notes

DO NOT:

✕ **Do NOT delay epinephrine administration** – it is the only effective treatment for anaphylaxis.
✕ **Do NOT substitute antihistamines** (Benadryl/diphenhydramine) for epinephrine in anaphylaxis.
✕ **Do NOT force the child to lie down** if having breathing difficulty – allow upright position of comfort.
✕ **Do NOT over-extend an infant's neck** when managing the airway – use neutral position only.
✕ **Do NOT inject epinephrine into fingers, hands, feet, or buttocks** – outer thigh only.
✕ **Do NOT touch the orange tip** of the EpiPen.
✕ **Do NOT induce vomiting** if the allergen was ingested.
✕ **Do NOT allow a child with anaphylaxis to leave** without hospital evaluation – biphasic reactions can be fatal.
✕ **Do NOT store EpiPen in refrigerator, car, or direct sunlight** – Israeli heat makes degradation a particular risk.
✕ **Do NOT give oral medications** to a child with throat swelling or difficulty swallowing.

Equipment needed:

— EpiPen Junior 0.15 mg (green label) -- for children 15-30 kg
— EpiPen 0.3 mg (yellow label) -- for children over 30 kg
— Phone to call MDA at 101

Hebrew Terminology — מונחים בעברית

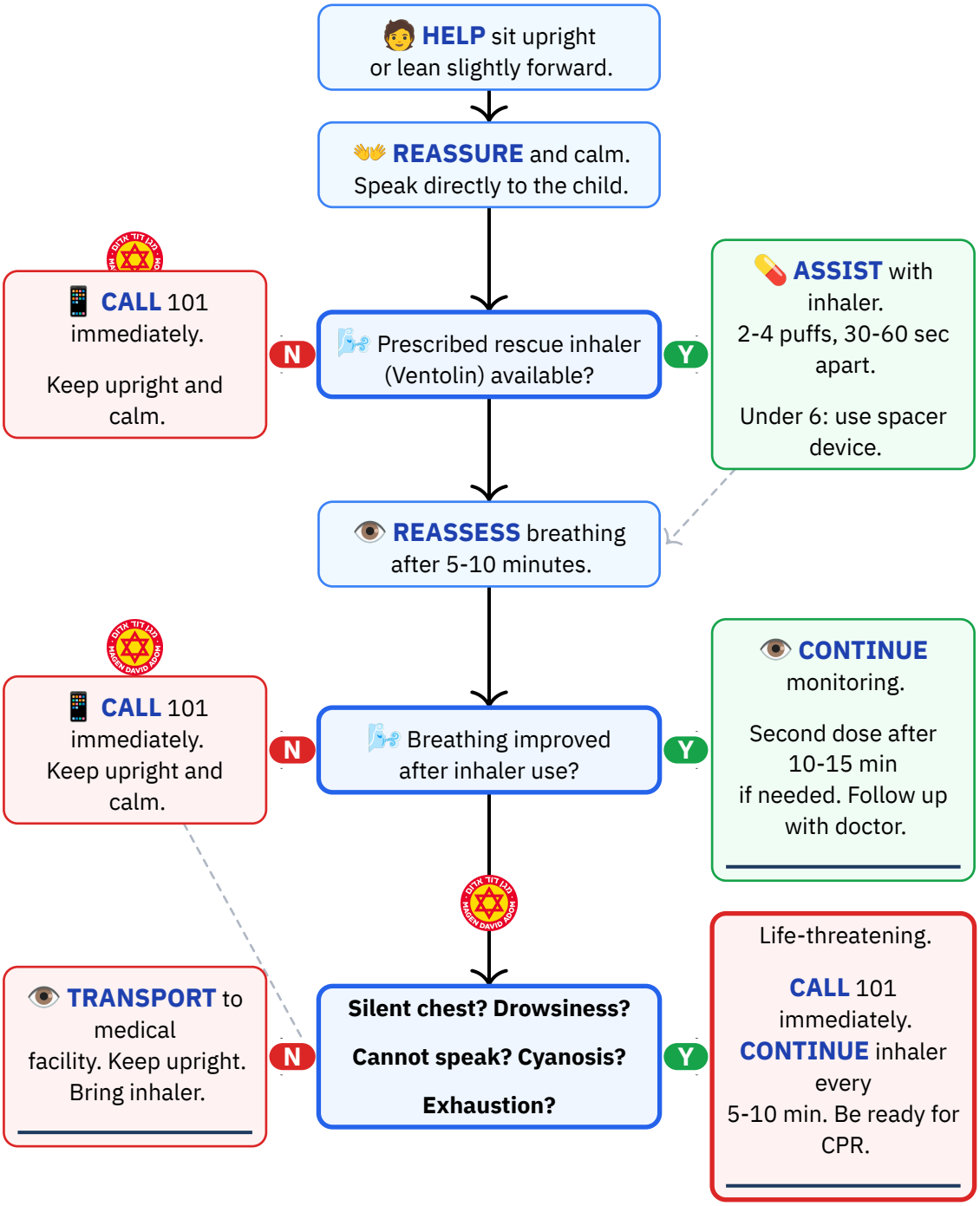
English	עברית	Pronunciation
Allergic reaction	תגובה אלרגית	tgu-VA a-LER-git
Anaphylaxis	אנפילקסיס	a-na-fi-LAK-sis
EpiPen	אפיפן	E-pi-pen
Swelling	נפיחות	ne-fi-KHUT
Hard to breathe	קשה לנשום	ka-SHE lin-SHOM
Thigh	ירך	ya-REKH
Allergy	אלרגיה	a-ler-GI-ya
Bee sting	עקיצת דבורה	a-ki-TSAT dvo-RA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Does the child have an EpiPen?	יש לילד אפיפן?	yesh la-YE-led E-pi-pen?

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Source: Magen David Adom (MDA) — ALS Manual 2024 / BLS Guide 2016
URL: https://www.mdais.org/media/5033/als-2024.pdf
Publication date: 2024-04-01
Imported: 2026-03-15 · **Last verified:** 2026-03-16

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HELP sit upright or lean slightly forward.
REASSURE and calm. Speak directly to the child.
Prescribed rescue inhaler (Ventolin) available?
N
CALL 101 immediately.
Keep upright and calm.
Y
ASSIST with inhaler.
2-4 puffs, 30-60 sec apart.
Under 6: use spacer device.
REASSESS breathing after 5-10 minutes.
Breathing improved after inhaler use?
N
CALL 101 immediately.
Keep upright and calm.
Y
CONTINUE monitoring.
Second dose after 10-15 min if needed. Follow up with doctor.
Silent chest? Drowsiness?
Cannot speak? Cyanosis?
Exhaustion?
N
TRANSPORT to medical facility. Keep upright. Bring inhaler.
Y
Life-threatening.
CALL 101 immediately.
CONTINUE inhaler every 5-10 min. Be ready for CPR.

Reference Notes

DO NOT:

✕ **Do NOT lay the child down** – upright position is essential for breathing.
✕ **Do NOT give cold drinks** during an attack – cold can worsen bronchospasm.
✕ **Do NOT leave the child alone** during an attack.
✕ **Do NOT administer someone else's prescription inhaler** without medical direction.
✕ **Do NOT delay calling 101** if the child is not improving.
✕ **Do NOT ignore drowsiness** during an asthma attack – dangerous sign of CO2 buildup requiring immediate intervention.
✕ **Do NOT assume a quiet chest means improvement** – a silent chest during distress is a life-threatening sign.

Equipment needed:

— Rescue inhaler (Ventolin/salbutamol) -- child's own prescribed inhaler
— Spacer device (for children under 6)
— Supplemental oxygen and face mask (if available)
— Pulse oximeter (if available)
— Phone (to call 101)

Hebrew Terminology — מונחים בעברית

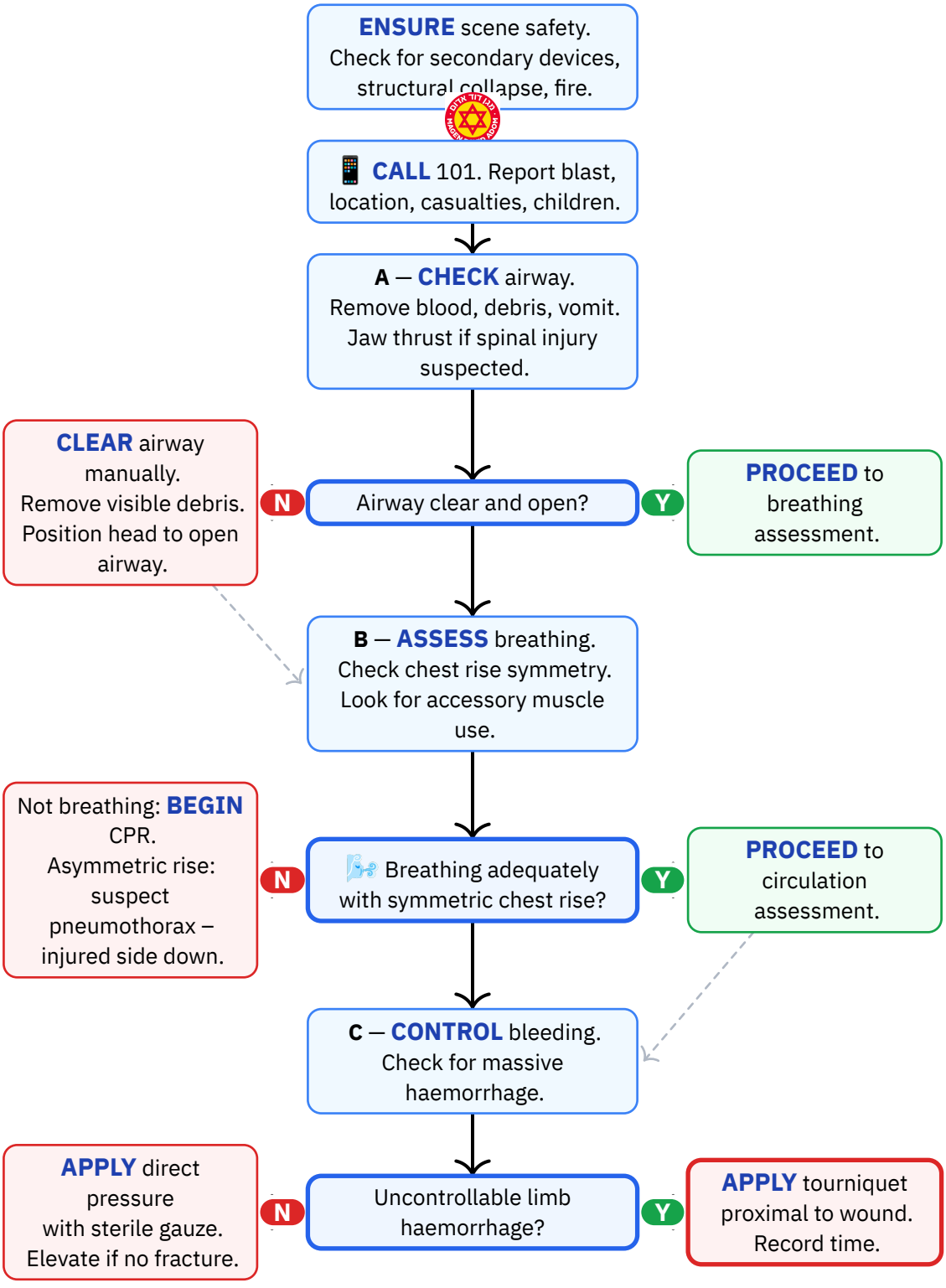
English	עברית	Pronunciation
Inhaler	משאף	mash-AF
Spacer	מרחב	mer-KHAV
Hard to breathe	קשה לנשום	ka-SHE lin-SHOM
Asthma	אסתמה	AST-ma
Asthma attack	התקף אסתמה	het-KEF AST-ma
Wheezing	צפצופים	tsif-tsu-FIM
Chest tightness	לחץ בחזה	LA-khats ba-kha-ZE
Puffs (doses)	שאיפות	she-i-FOT
Child	ילד / ילדה	YE-led / yal-DA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Can you breathe?	אתה יכול לנשום?	a-TA ya-KHOL lin-SHOM?
Where is the inhaler?	איפה המשאף?	EI-fo ha-mash-AF?

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Source: Magen David Adom (MDA) – ALS Professional Manual (April 2024), supplemented by Ichud Hatzalah guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16

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Disability, Exposure & Ongoing Care

D — ASSESS neurological status (AVPU scale).
Check pupil size and reactivity.

↓

E — EXPOSE and examine for additional wounds, burns, embedded objects, fractures.

Then **COVER** immediately to prevent hypothermia.

↓

COVER open wounds with sterile dressings.
STABILISE embedded objects with padding around them.

↓

👐 **KEEP** child with parent or caregiver.
Speak calmly and honestly.

↓

👁️ **MONITOR** continuously.
Keep warm and covered.
Be prepared for CPR.

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Reference Notes

DO NOT:

- ✕ **Do NOT enter an unsafe blast scene** – risk of secondary devices and structural collapse.
- ✕ **Do NOT remove embedded objects** (shrapnel, fragments) – stabilise them in place.
- ✕ **Do NOT assume a quiet child is uninjured** – children may be in psychological shock.
- ✕ **Do NOT remove a tourniquet once applied** – hospital personnel will manage removal.
- ✕ **Do NOT forget to prevent hypothermia** after examination – children lose heat rapidly.
- ✕ **Do NOT separate the child from their parent** or caregiver unnecessarily.
- ✕ **Do NOT ignore asymmetric chest rise** – indicates thoracic trauma requiring urgent attention.
- ✕ **Do NOT give food or drink** if significant injuries present – may need anaesthesia at hospital.

Equipment needed:

— Tourniquet (CAT or equivalent -- see tourniquet protocol for child-specific sizing)
— Sterile gauze and bandages
— Blankets (for hypothermia prevention)
— Rigid splints (for fracture immobilisation)
— Phone (to call 101)
— Supplemental oxygen and mask (if available)

Hebrew Terminology — מונחים בעברית

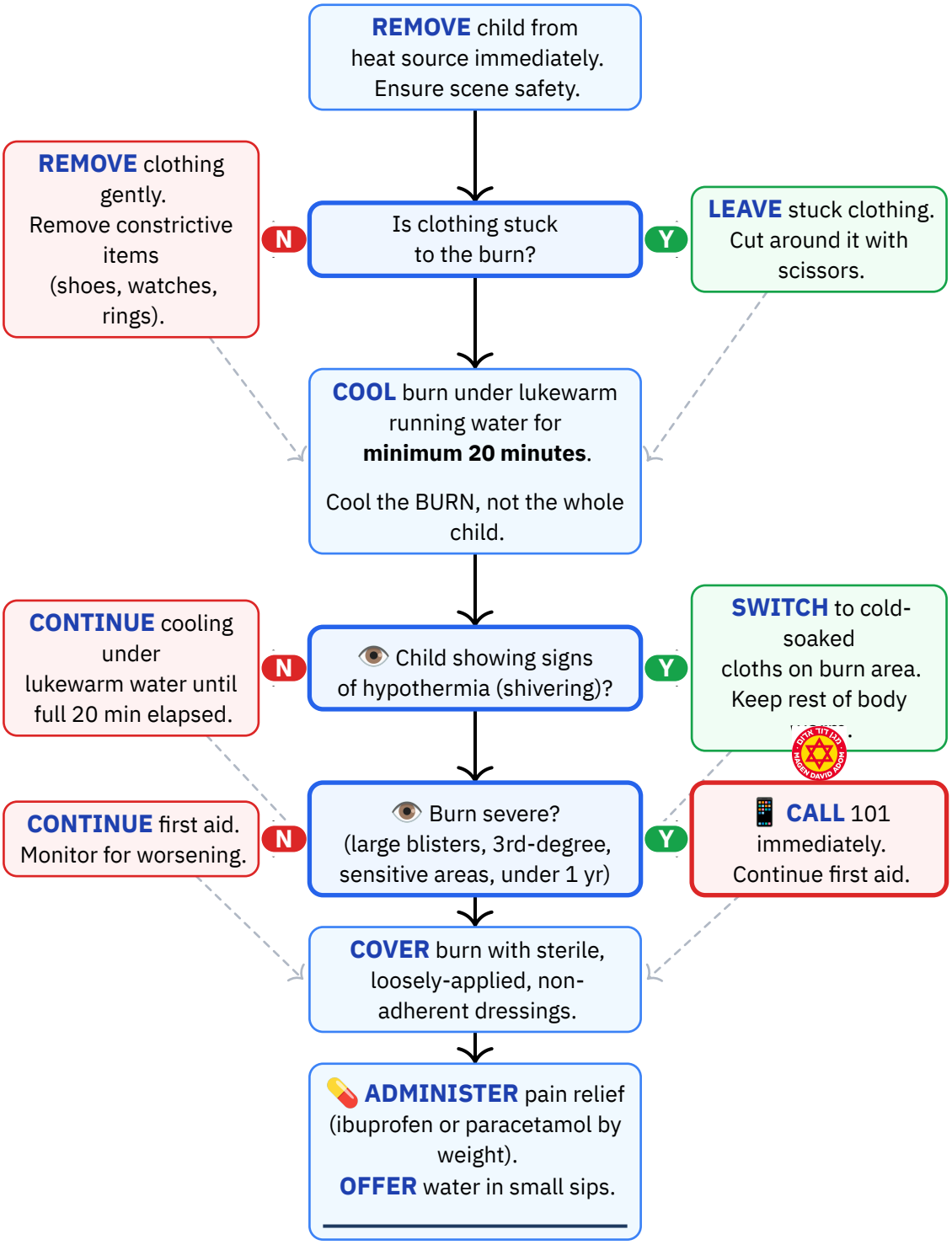
English	עברית	Pronunciation
Explosion	פיצוץ	pi-TSUTS
Shrapnel	רסיסים	re-si-SIM
Rocket / missile	רקטה / טיל	ra-KE-ta / til
Shelter (safe room)	מקלט / ממ"ד	mik-LAT / ma-MAD
Bleeding	דימום	di-MUM
Tourniquet	חוסם עורקים	kho-SEM or-KIM
Wound	פצע	PE-tsa
Breathing	נשימה	ne-shi-MA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Are you hurt?	אתה פצוע?	a-TA pa-TSU-a?
Where does it hurt?	איפה כואב?	EI-fo ko-EV?

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Source: Magen David Adom (MDA) – PHTLS-aligned trauma protocols, supplemented by Ichud Hatzalah missile strike response guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16

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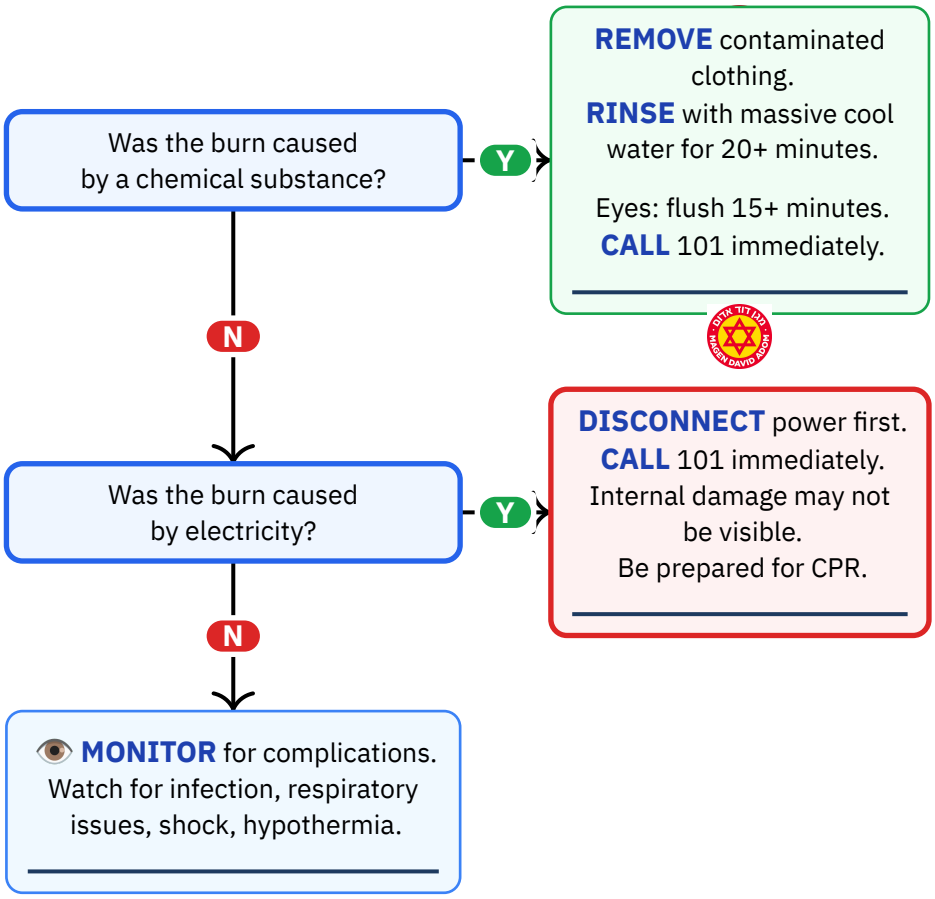
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Chemical & Electrical Burns



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Reference Notes

DO NOT:

✕ **Do NOT apply ice or ice-cold water** – extreme cold causes additional tissue and blood vessel damage.
✕ **Do NOT apply toothpaste, coffee grounds, raw eggs, yogurt, or butter** – these create a heat-trapping layer and increase infection risk.
✕ **Do NOT puncture or pop blisters** – the fluid protects tissue during healing.
✕ **Do NOT apply ointments, oils, or powders** to serious burns before medical evaluation.
✕ **Do NOT use gauze pads or cotton wool directly on burns** – fibres adhere to damaged skin.
✕ **Do NOT remove clothing stuck to the burn** – cut around it instead.
✕ **Do NOT immerse the whole child in cold water** – cool the burn, not the child. High hypothermia risk.
✕ **Do NOT attempt chemical neutralisation** on chemical burns.
✕ **Do NOT leave the child unattended if unconscious** – call 101 immediately.

Equipment needed:

— Cool (lukewarm) running water source
— Sterile non-adherent dressings or clean dry sheet
— Scissors (to cut clothing around stuck areas)
— Blanket or towel (warmth during burn cooling)
— Age-appropriate pain relief (ibuprofen or paracetamol, dosed by weight)
— Drinking water (for hydration)

Hebrew Terminology — מונחים בעברית

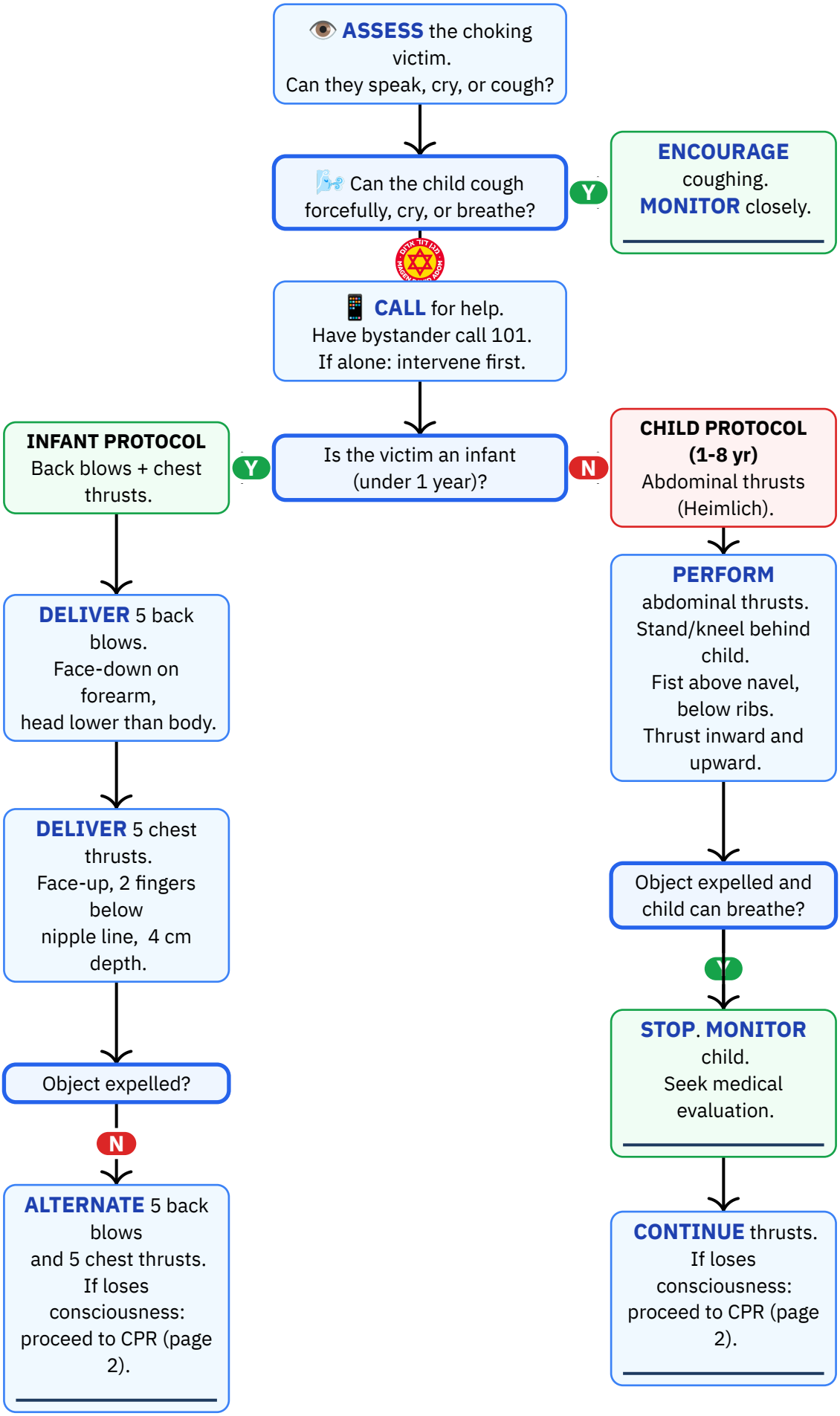
English	עברית	Pronunciation
Burn	כוויה	kvi-YA
Scald	כוויה מנוזל חם	kvi-YA mi-no-ZEL kham
Cool water	מים פושרים	MA-yim posh-RIM
Blister	שלפוחית	shal-pu-KHIT
Bandage / dressing	תחבושת	takh-BO-shet
Pain	כאב	ke-EV
Child	ילד / ילדה	YE-led / yal-DA
Infant	תינוק / תינוקת	ti-NOK / ti-NO-ket
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
It hurts	כואב	ko-EV

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Source: Magen David Adom (MDA) – Burns First Aid Guidelines
URL: https://www.mdais.org/101/burns
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-16
Key child difference: Minimum 20 minutes cooling (vs 10 for adults). Cool the burn, not the whole child. All burns in infants under 1 year require immediate medical attention.

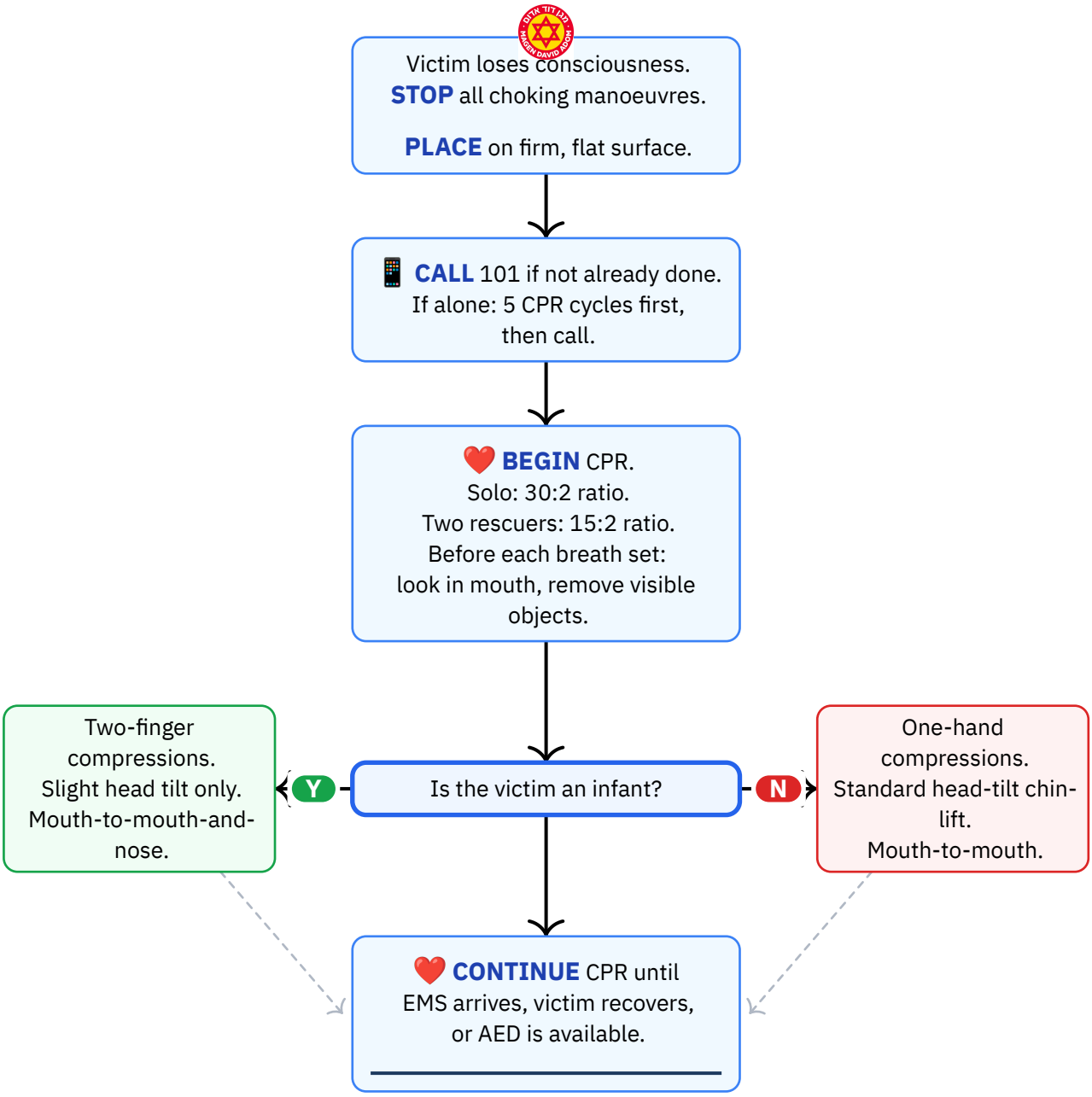
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CPR Transition (Unconscious Victim)



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Reference Notes

DO NOT:

✕ **Do NOT perform abdominal thrusts on infants under 1 year** – use only back blows and chest thrusts. Risk of organ damage.
✕ **Do NOT insert fingers blindly into the mouth** (blind finger sweeps) – only remove objects you can clearly see.
✕ **Do NOT force anything into the mouth** – risk of pushing the object deeper.
✕ **Do NOT give food or fluids** to a choking victim.
✕ **Do NOT intervene if the child is coughing effectively** – allow the body to clear the obstruction.
✕ **Do NOT use this protocol for choking caused by asthma, infection, or head injury** – seek immediate medical care.
✕ **Do NOT use excessive force** during abdominal thrusts that would lift the child off the ground.
✕ **Do NOT fully extend an infant's head** when opening the airway – use slight tilt position only.

Equipment needed:

— AED with paediatric pads (if available, for CPR transition)

Hebrew Terminology — מונחים בעברית

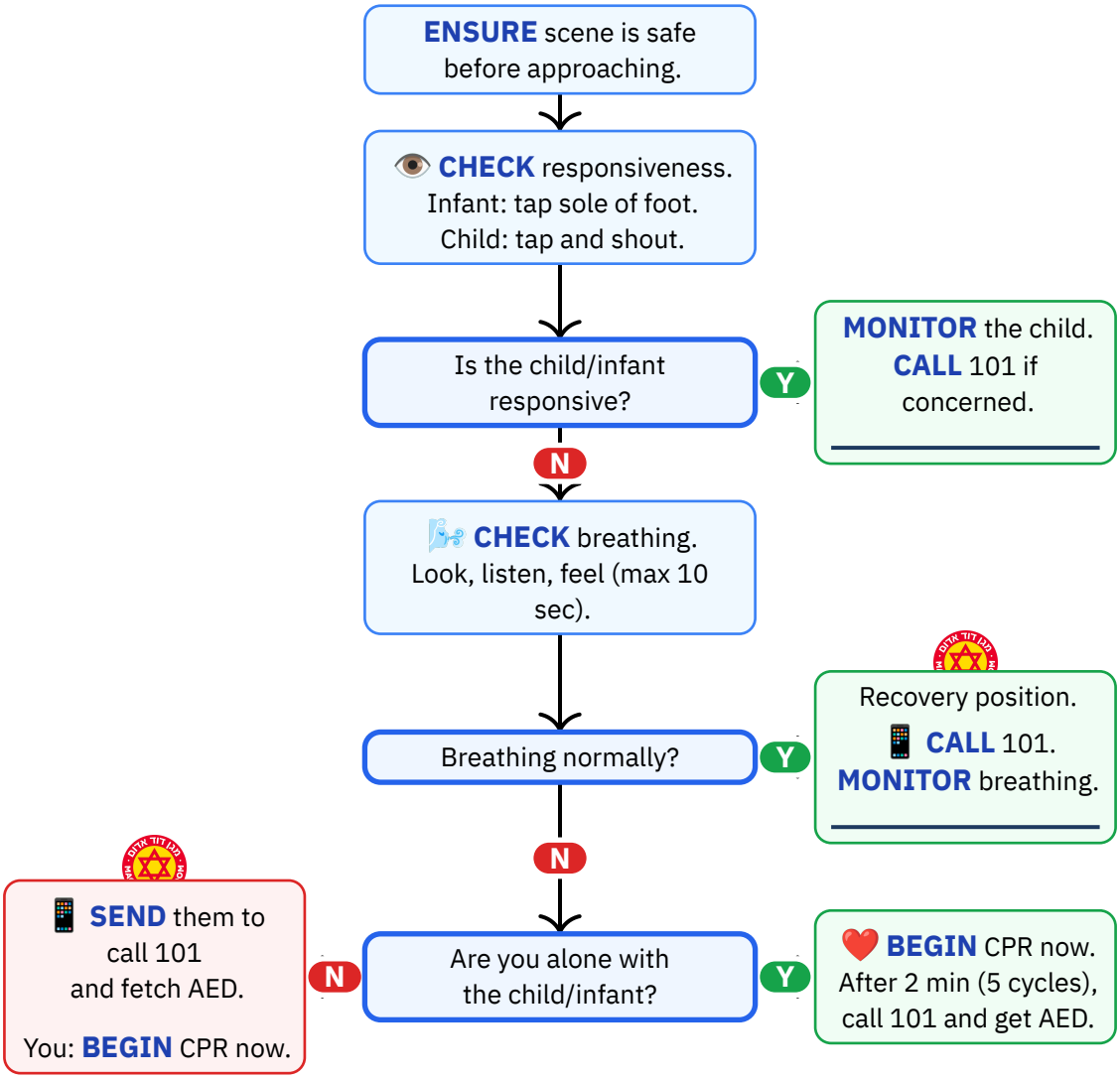
English	עברית	Pronunciation
Choking	חנק	KHE-nek
Foreign body	גוף זר	guf zar
Coughing	שיעול	shi-UL
Back blows	מכות גב	ma-KOT gav
Chest thrusts	לחיצות חזה	le-khi-TSOT kha-ZE
Abdominal thrusts	לחיצות בטן	le-khi-TSOT BE-ten
Breathing	נשימה	ne-shi-MA
CPR	החייאה	ha-kha-ya-A
Child	ילד / ילדה	YE-led / yal-DA
Infant	תינוק / תינוקת	ti-NOK / ti-NO-ket
The child is choking!	הילד נחנק!	ha-YE-led nekh-NAK!
Ambulance	אמבולנס	am-bu-LANS

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Source: Magen David Adom (MDA) – MDA 101 Public First Aid Guidance Series
URL: https://www.mdais.org/101/asphyxia
Imported: 2026-03-15 · **Last verified:** 2026-03-16
Notes: Infant (0-1 year), child (1-8 years). Physical size should guide technique selection, not age alone.

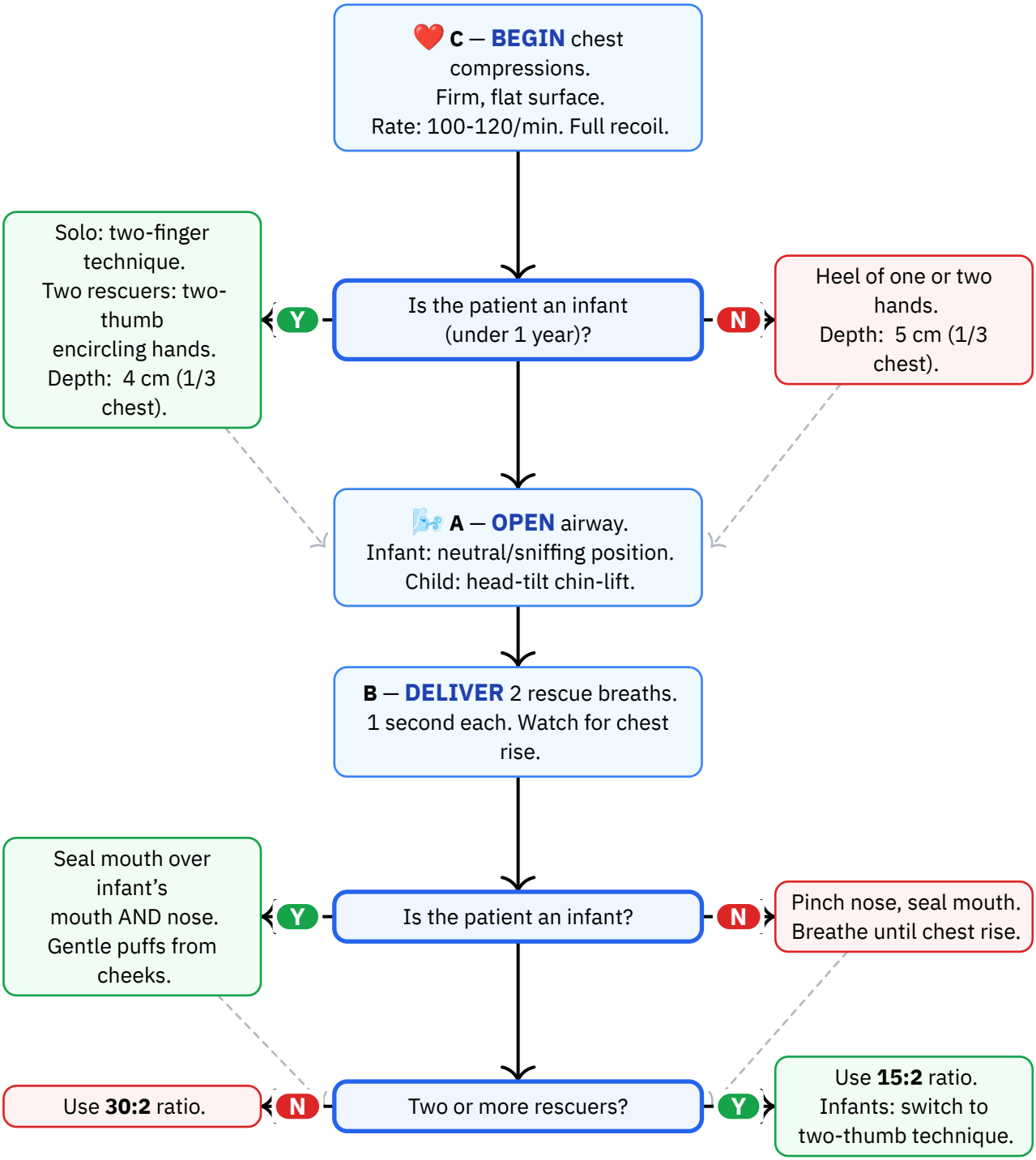
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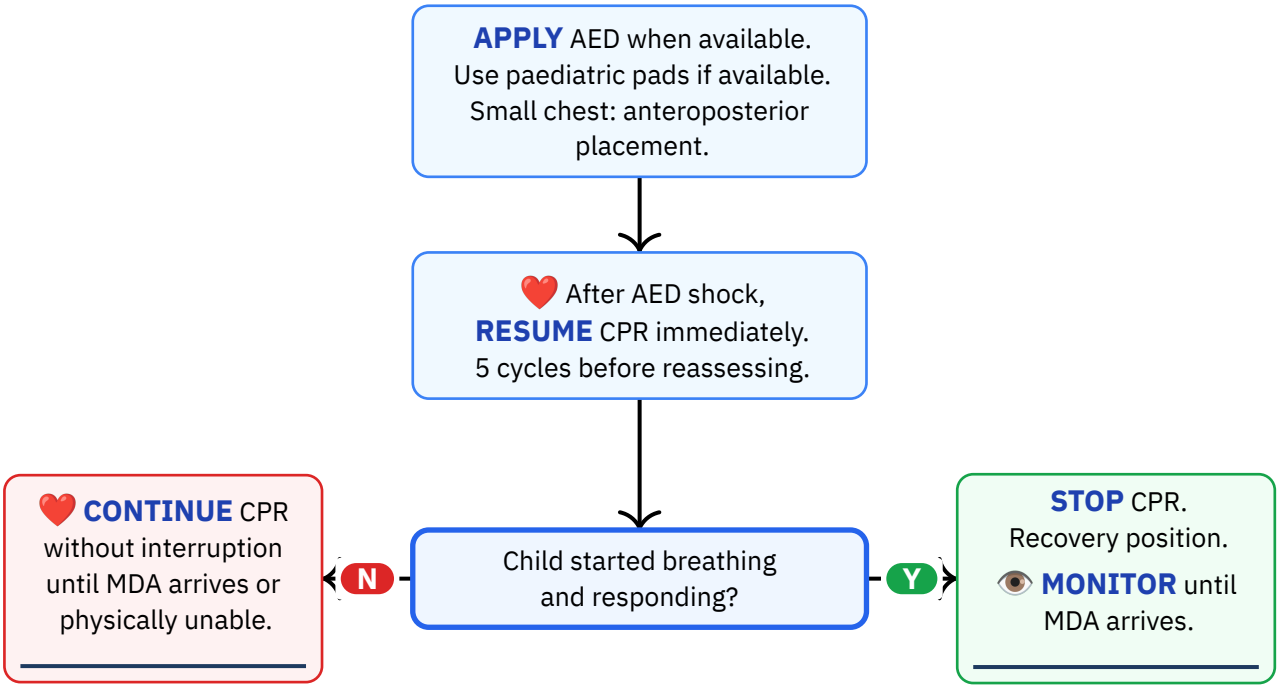
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CPR Technique (C-A-B)



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AED & Continuation



Key Paediatric CPR Parameters

Compression rate: 100-120/min
Infant depth: 4 cm (1/3 chest)
Two-rescuer ratio: 15:2
Lone rescuer: 2 min CPR, then call 101
Child depth: 5 cm (1/3 chest)
Solo ratio: 30:2
Breath duration: 1 second each
Emergency number: 101 (MDA)

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Reference Notes

DO NOT:

✕ **Do NOT hyperextend an infant's neck** when opening the airway – use neutral/sniffing position only.
✕ **Do NOT insert fingers into the mouth** (risk of pushing objects deeper).
✕ **Do NOT deliver deep lung breaths to infants** – use gentle puffs from your cheeks.
✕ **Do NOT use excessive volume or pressure** when delivering rescue breaths (risk of gastric distension).
✕ **Do NOT skip rescue breaths** in paediatric CPR – compression-only CPR is NOT adequate for children and infants.
✕ **Do NOT delay CPR to call for help when alone** – perform 2 minutes of CPR first, then call 101.
✕ **Do NOT allow AED pads to touch or overlap** on a small child or infant.
✕ **Do NOT stop CPR to check for pulse** unless there are clear signs of recovery (breathing, movement).
✕ **Do NOT compress over the xiphoid process** (the very bottom tip of the sternum).

Equipment needed:

— AED with paediatric pads or paediatric dose attenuator (if available)
— Barrier device or pocket mask (optional, for rescue breaths)
— Bag-valve-mask -- infant size (250-350 cc) or paediatric size (if available)

Hebrew Terminology — מונחים בעברית

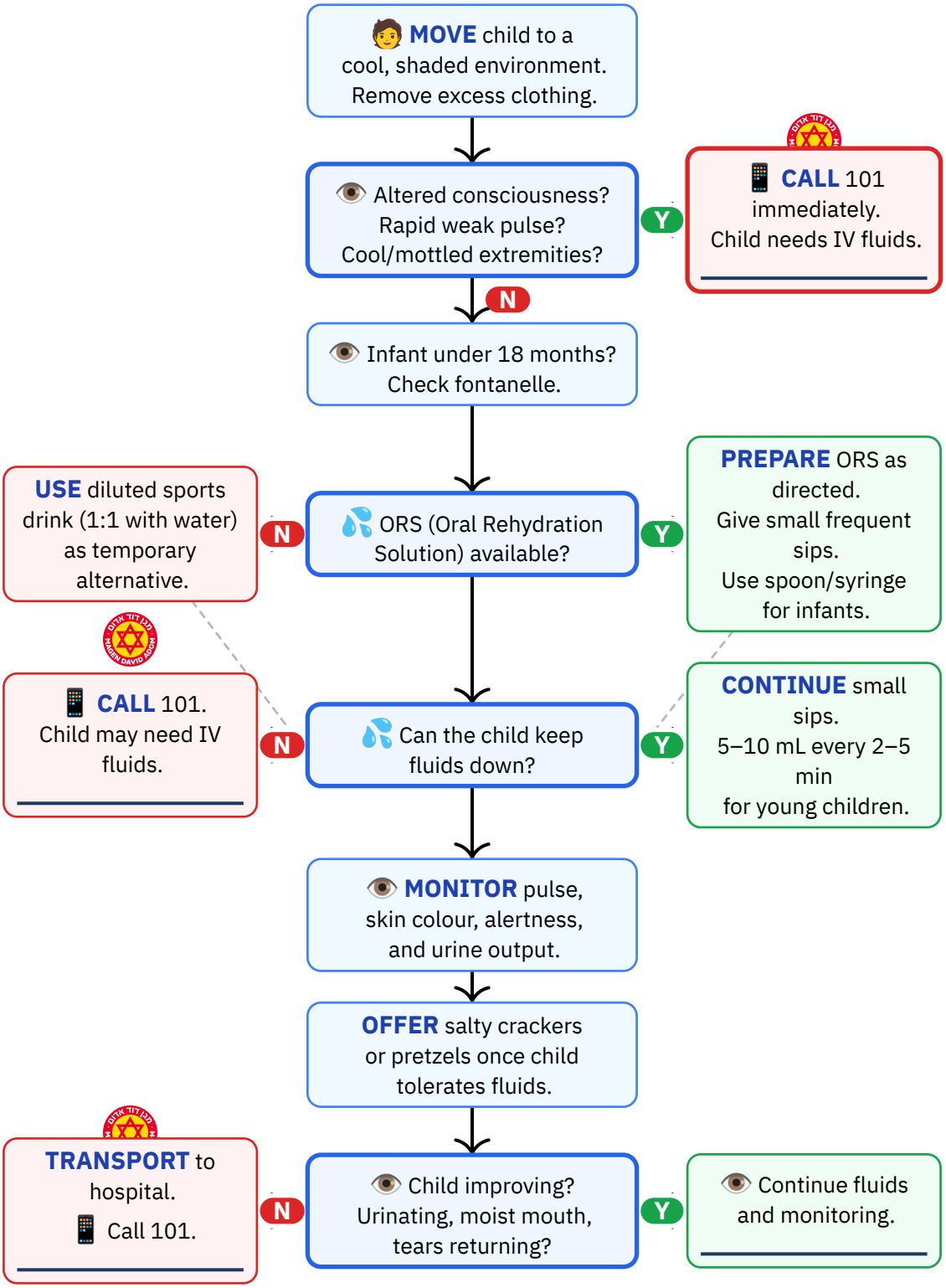
English	עברית	Pronunciation
CPR	החייאה	ha-kha-ya-A
Cardiac arrest	דום לב	dom lev
Chest compressions	לחיצות חזה	le-khi-TSOT kha-ZE
Rescue breaths	הנשמות	han-sha-MOT
Not breathing	לא נושם	lo no-SHEM
Unconscious	מחוסר הכרה	me-khu-SAR ha-ka-RA
Defibrillator	דפיברילטור	de-fib-ri-LA-tor
Recovery position	תנוחת התאוששות	tnu-KHAT hit-o-she-SHUT
Child	ילד / ילדה	YE-led / yal-DA
Infant	תינוק / תינוקת	ti-NOK / ti-NO-ket
Ambulance	אמבולנס	am-bu-LANS
The child is not breathing!	הילד לא נושם!	ha-YE-led lo no-SHEM!

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Source: Magen David Adom (MDA) — BLS Guide 2016 / ALS Professional Handbook 2024
URL: https://www.mdais.org/kursim/cpr-children
Publication date: 2024-04-01
Imported: 2026-03-15 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah. In children, cardiac arrest is almost always secondary to respiratory failure – rescue breaths are critically important.

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Reference Notes

DO NOT:

✕ **Do NOT give cola, fruit juice, plain water alone, or tea** — these do not replace electrolytes and can worsen dehydration.
✕ **Do NOT use homemade salt/sugar solutions** unless ORS is completely unavailable.
✕ **Do NOT force large volumes of fluid at once** — small frequent sips are more effective.
✕ **Do NOT delay calling 101 for an infant under 3 months** with any signs of dehydration.
✕ **Do NOT ignore a sunken fontanelle** in an infant — this is a key dehydration indicator.
✕ **Do NOT give anti-diarrhoea medications** to children without medical direction.
✕ **Do NOT stop breastfeeding** — continue alongside ORS.

Equipment needed:

— ORS (Oral Rehydration Solution) packets
— Spoon or oral syringe (for infant fluid administration)
— Phone (to call 101)
— Thermometer (to check for fever)

Hebrew Terminology — מונחים בעברית

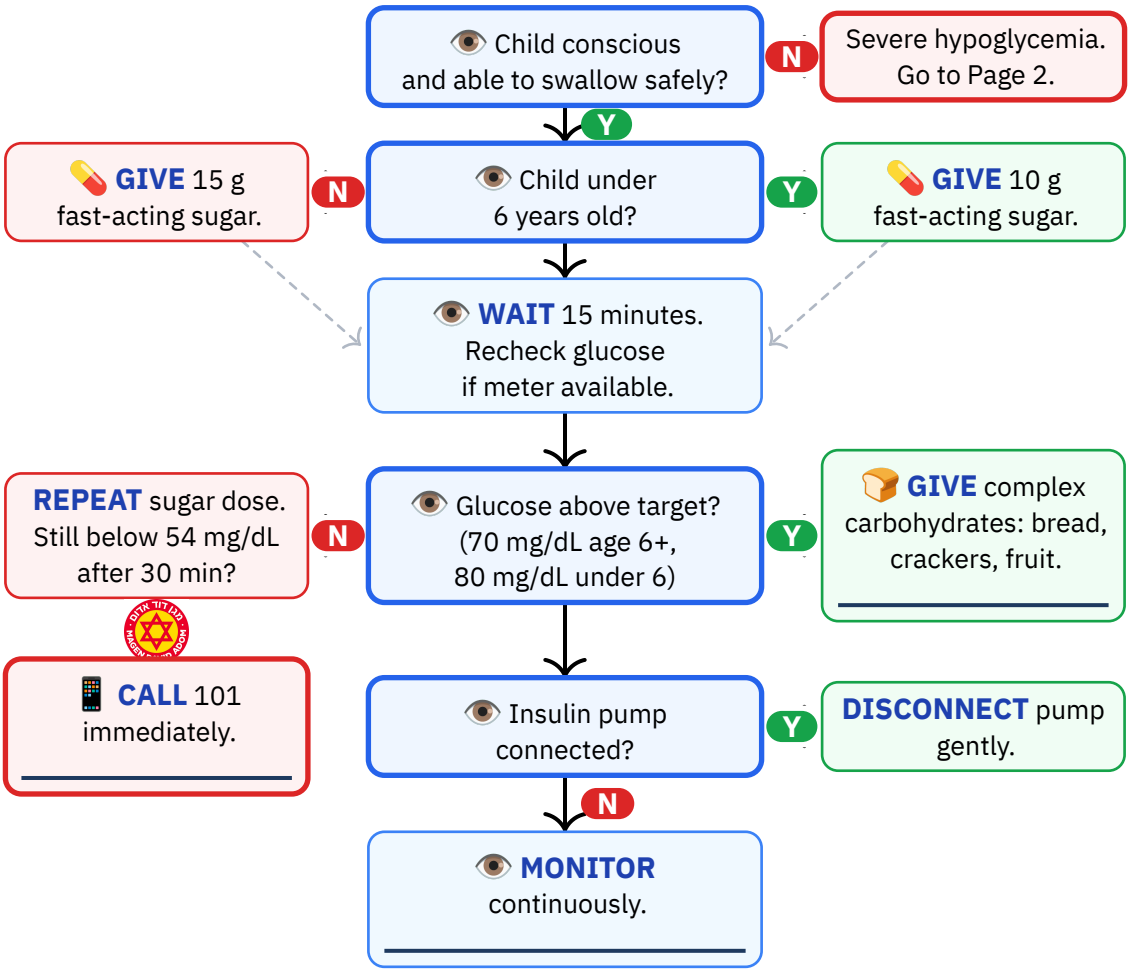
English	עברית	Pronunciation
Dehydration	התייבשות	hit-yab-SHUT
Oral rehydration	שתייה מחדש	shti-YA me-kha-DASH
ORS / electrolytes	תמיסת מלחים	tmi-SAT mla-KHIM
Fontanelle	מרפס	mar-PES
Vomiting	הקאות	ha-ka-OT
Diarrhoea	שלשולים	shil-shu-LIM
Thirsty	צמא	tsa-ME
Pharmacy	בית מרקחת	beit mir-KA-khat
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child is dehydrated	הילד מיובש	ha-YE-led me-yu-BASH
He/she is vomiting	הוא/היא מקיא/ה	hu/hi ma-KI/ma-ki-A

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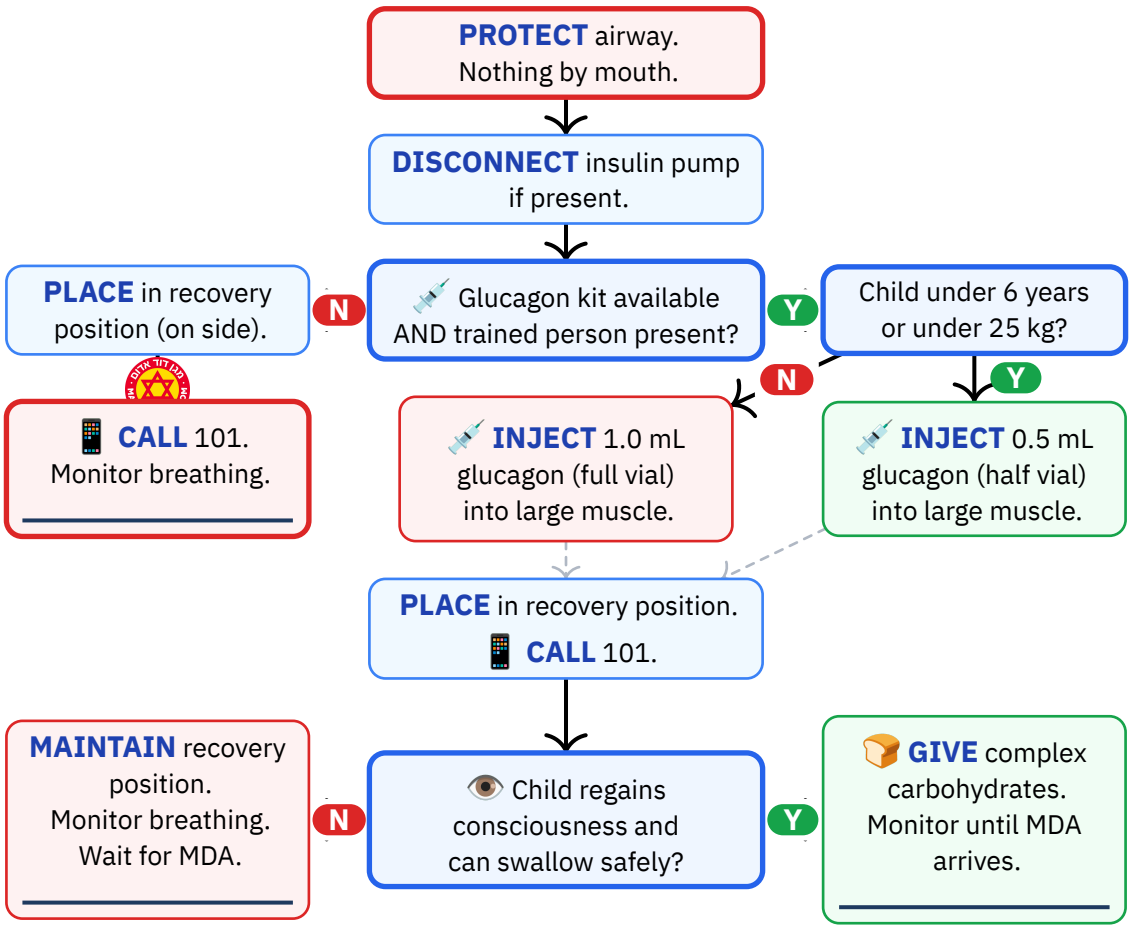
Source: Magen David Adom (MDA) — MDA Dehydration First Aid Guidelines — Pediatric
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16

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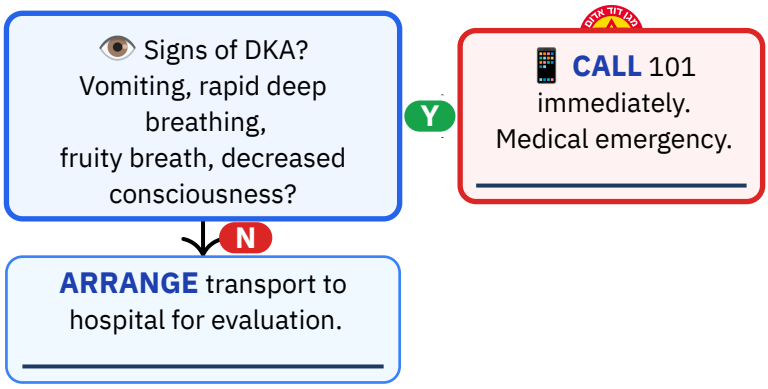

Child conscious and able to swallow safely?
N
Severe hypoglycemia. Go to Page 2.
Y
GIVE 15 g fast-acting sugar.
N
Child under 6 years old?
Y
GIVE 10 g fast-acting sugar.
WAIT 15 minutes. Recheck glucose if meter available.
REPEAT sugar dose. Still below 54 mg/dL after 30 min?
N
Glucose above target? (70 mg/dL age 6+, 80 mg/dL under 6)
Y
GIVE complex carbohydrates: bread, crackers, fruit.
CALL 101 immediately.
Insulin pump connected?
Y
DISCONNECT pump gently.
N
MONITOR continuously.

Severe Hypoglycemia — Unconscious / Seizing



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DKA Assessment & Uncertainty Rule



When in Doubt — Give Sugar

If uncertain whether hypoglycemia or hyperglycemia: **give sugar**.

Sugar for a hypoglycemic child is life-saving.
Sugar for a hyperglycemic child will not cause immediate harm.
Withholding sugar from a hypoglycemic child can be fatal.

This applies only when the child is conscious and can swallow safely.

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Reference Notes

DO NOT:

- ✕ **Do NOT administer insulin** — this is strictly a medical intervention and can be fatal if given inappropriately.
- ✕ **Do NOT give food, drink, or oral medication to an unconscious or seizing child** — choking hazard.
- ✕ **Do NOT use chocolate as the initial sugar source** — fat and protein slow absorption and delay correction.
- ✕ **Do NOT administer glucagon unless trained and authorised** to do so.
- ✕ **Do NOT delay calling 101** for severe hypoglycemia, seizures, or loss of consciousness.
- ✕ **Do NOT attempt to manage hyperglycemia or DKA with first aid** — requires medical intervention.
- ✕ **Do NOT use diet drinks or sugar-free products** as a sugar source.

Equipment needed:

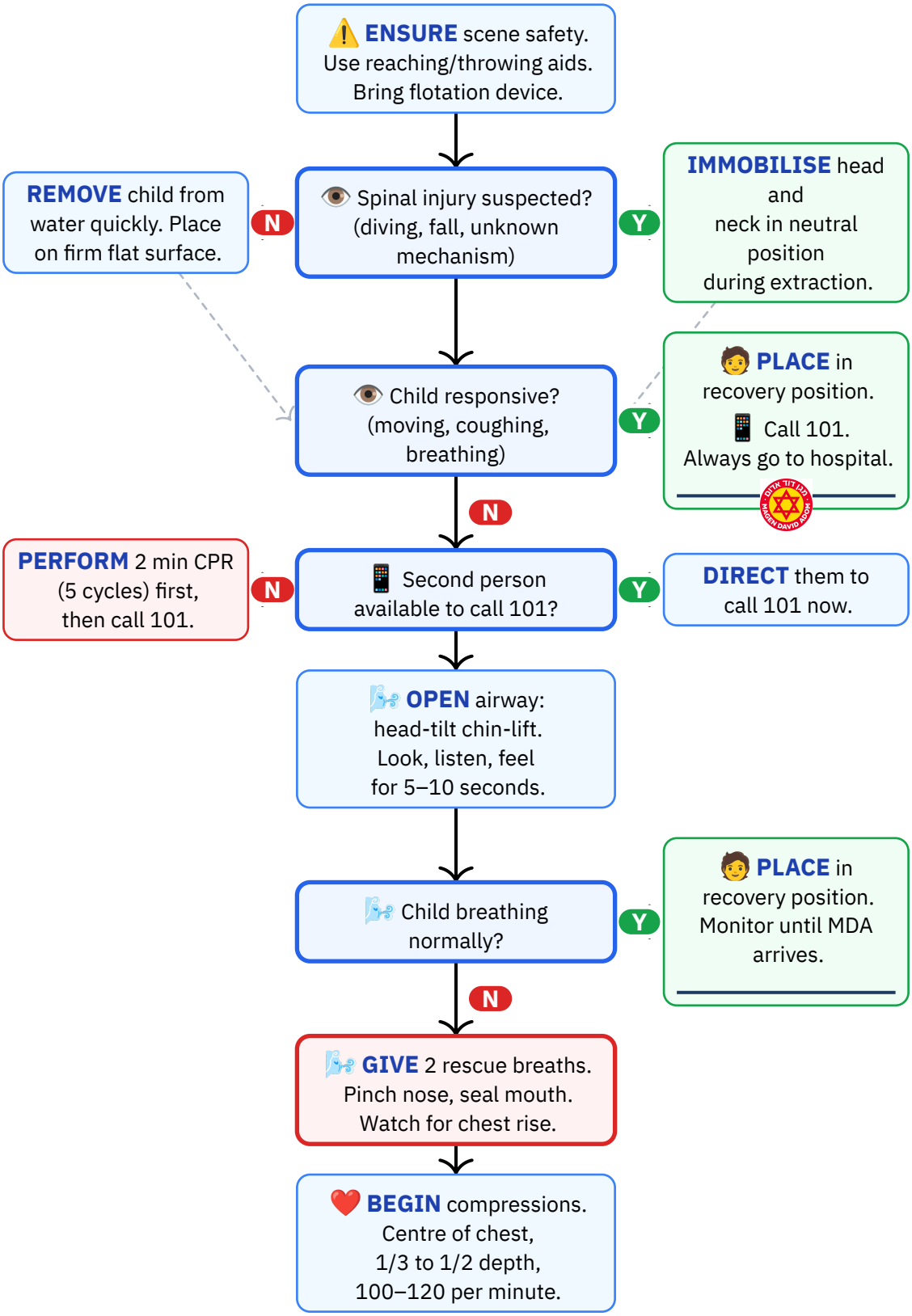
— Glucose gel syringe (15 g glucose)
— Fast-acting sugar source (juice, glucose tablets, sugar cubes, honey)
— Complex carbohydrate snack (bread, crackers, fruit)
— Glucagon emergency kit (Hypo-Kit) if available and authorised
— Blood glucose meter (glucometer) if available

Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Diabetes	סוכרת	su-KE-ret
Low blood sugar	סוכר נמוך	SU-kar na-MUKH
High blood sugar	סוכר גבוה	SU-kar ga-VO-ah
Sugar	סוכר	SU-kar
Glucagon	גלוקגון	glu-ka-GON
Insulin pump	משאבת אינסולין	mash-E-vet in-su-LIN
Seizure	פרכוס	pir-KUS
Unconscious	חסר הכרה	kha-SAR ha-ka-RA
Sweet drink	שתייה מתוקה	shti-YA me-tu-KA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Does the child have diabetes?	יש לילד סוכרת?	yesh la-YE-led su-KE-ret?

Source: Magen David Adom (MDA) — How to Treat a Diabetes Patient
URL: https://www.mdais.org/101/diabetes
Imported: 2026-03-15 · **Last verified:** 2026-03-16

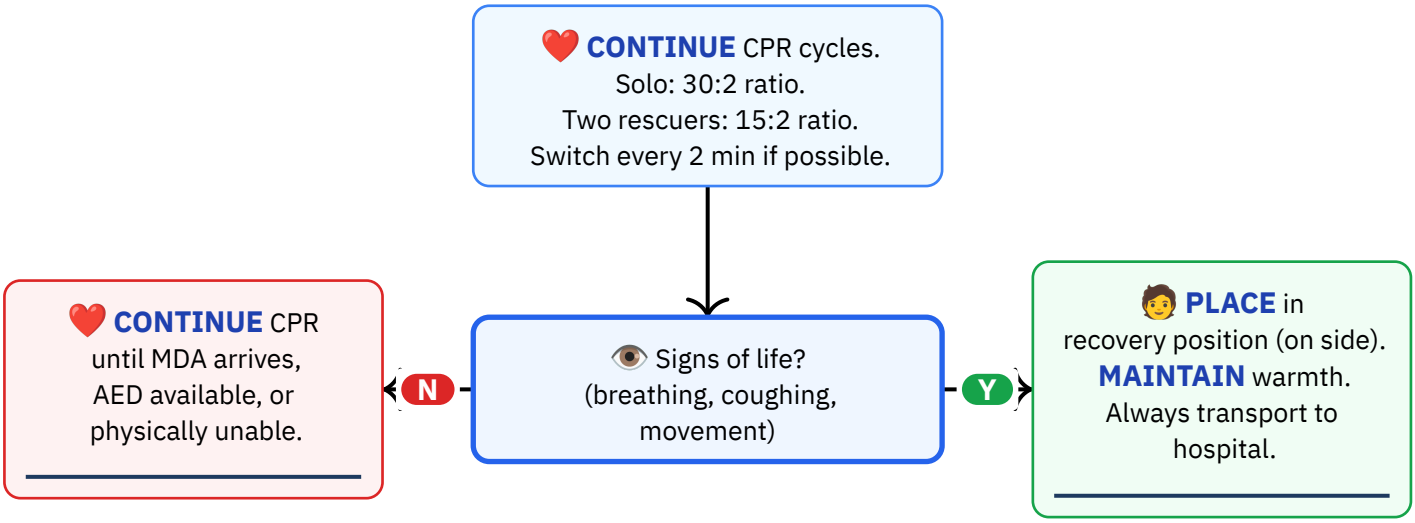
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CPR Continuation & Recovery



All drowning victims MUST go to hospital

Secondary drowning (delayed pulmonary oedema) can develop hours after the event, even if the child initially appears to recover fully. Electrolyte imbalances can also occur.

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Reference Notes

DO NOT:

✕ **Do NOT attempt to remove water from the lungs** — this is impossible and can endanger the victim.
✕ **Do NOT perform abdominal thrusts (Heimlich)** to expel water — not effective for drowning and risks aspiration.
✕ **Do NOT delay CPR** to attempt water removal.
✕ **Do NOT use compression-only CPR for drowning** — rescue breaths are essential (hypoxic emergency).
✕ **Do NOT assume the child is fine** if they regain consciousness — always transport to hospital.
✕ **Do NOT enter the water** unless you are a trained water rescuer with a flotation device.
✕ **Do NOT use excessive ventilation force** — can cause gastric distention in children.

Equipment needed:

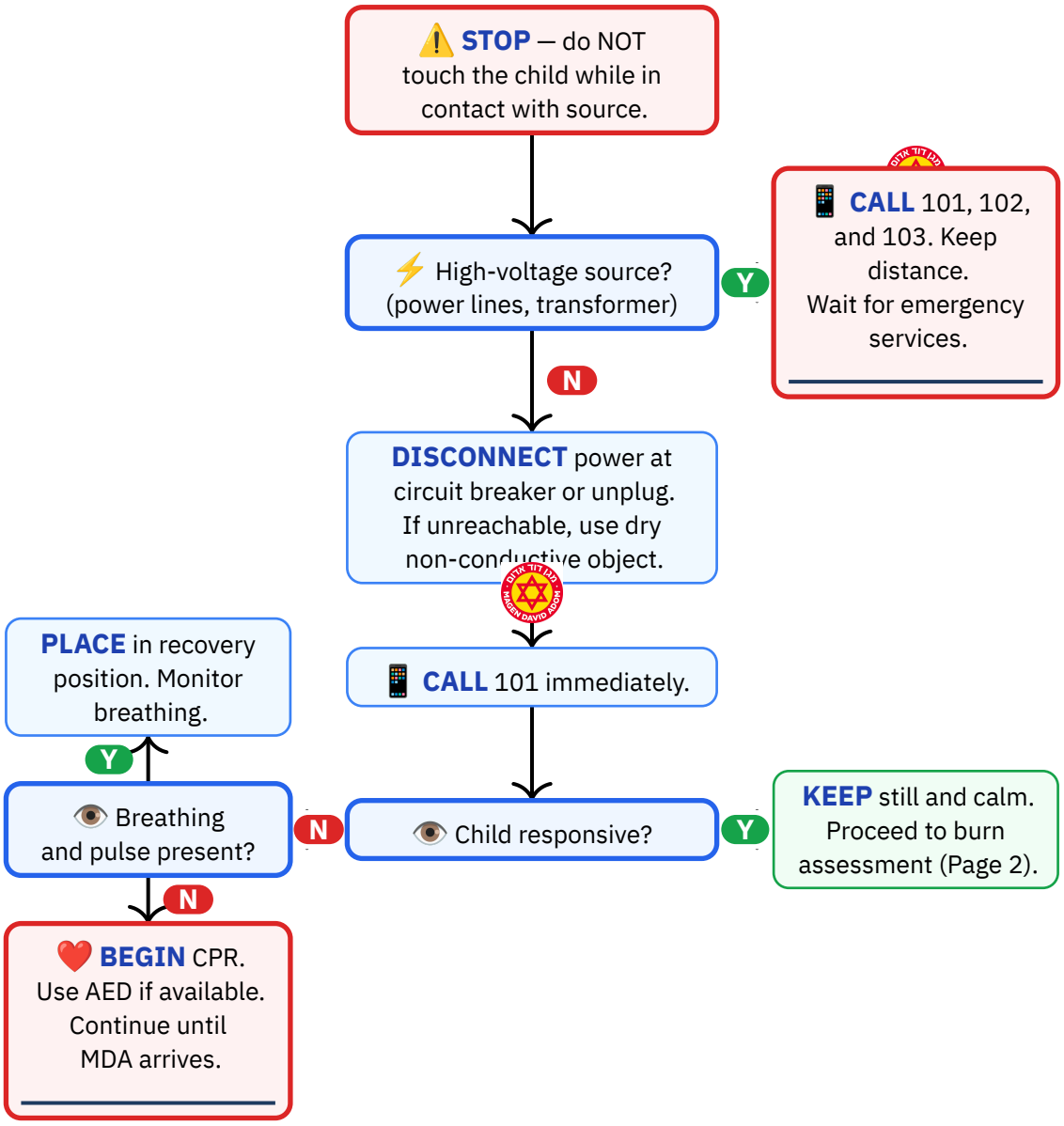
— Reaching aid (pole, rope, towel) or throwing aid (flotation device)
— Firm, flat surface for CPR
— Blanket or covering for warmth (hypothermia prevention)

Hebrew Terminology — מונחים בעברית

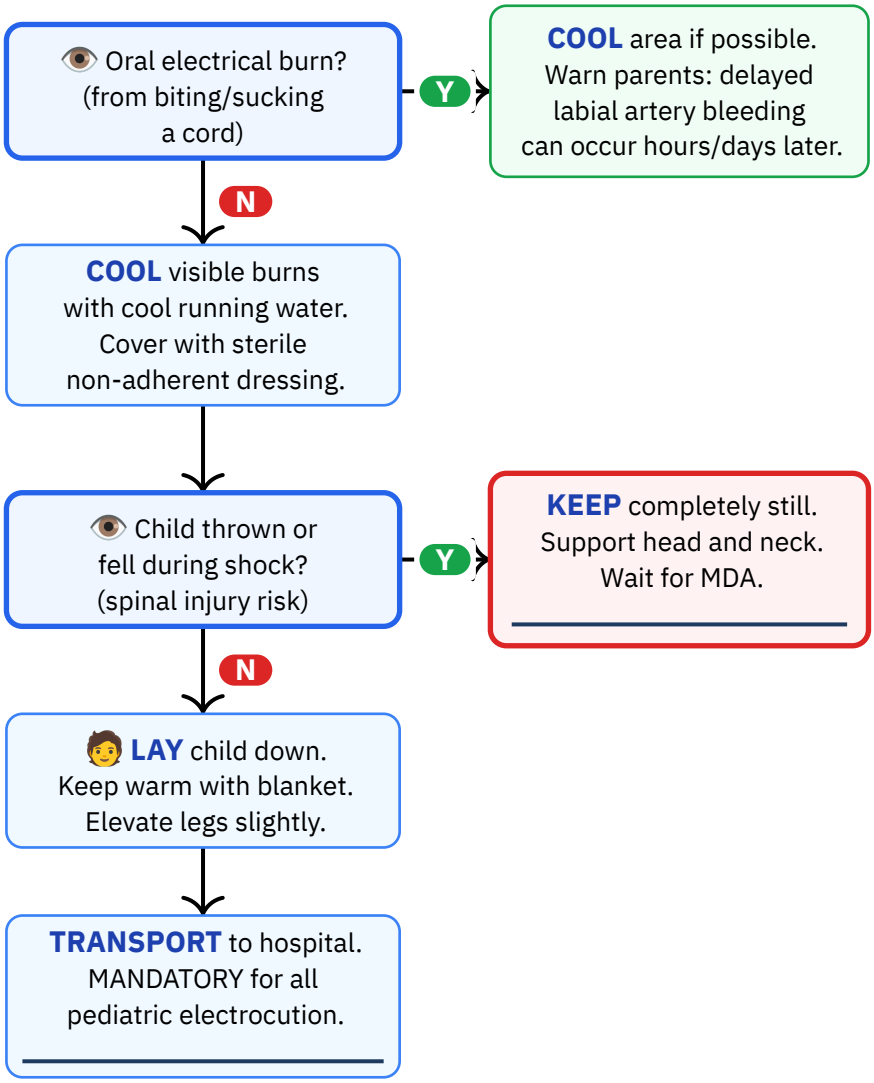
English	עברית	Pronunciation
Drowning	טביעה	tvi-A
Water	מים	MA-yim
Swimming pool	בריכת שחייה	bre-KHAT skhi-YA
CPR	החייאה	ha-kha-ya-A
Rescue breaths	הנשמות	han-sha-MOT
Chest compressions	לחיצות חזה	lkhi-TSOT kha-ZE
Breathing	נשימה	nshi-MA
Recovery position	תנוחת התאוששות	tnu-KHAT hit-o-she-SHUT
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child is not breathing	הילד לא נושם	ha-YE-led lo no-SHEM
Call an ambulance	תזמין אמבולנס	taz-MIN am-bu-LANS

Source: Magen David Adom (MDA) — Drowning First Aid Protocol for Children
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-16

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⚠️ STOP — do NOT touch the child while in contact with source.
⚡ High-voltage source? (power lines, transformer)
Y
📱 CALL 101, 102, and 103. Keep distance. Wait for emergency services.
N
DISCONNECT power at circuit breaker or unplug. If unreachable, use dry non-conductive object.
📱 CALL 101 immediately.
👁️ Child responsive?
Y
KEEP still and calm. Proceed to burn assessment (Page 2).
N
👁️ Breathing and pulse present?
Y
PLACE in recovery position. Monitor breathing.
N
❤️ BEGIN CPR. Use AED if available. Continue until MDA arrives.

Burn Assessment & Shock Management



Reference Notes

DO NOT:

✕ **Do NOT touch the child while in contact with the electrical source** — chain electrocution risk.
✕ **Do NOT use metal objects or wet/damp materials** to separate the child from the source.
✕ **Do NOT apply ice to electrical burns.**
✕ **Do NOT apply creams, ointments, toothpaste, or home remedies** to electrical burns.
✕ **Do NOT assume the child is fine** because external injuries appear minor — internal damage may be hidden.
✕ **Do NOT delay calling 101** — every pediatric electrocution requires hospital assessment.
✕ **Do NOT move the child unnecessarily** — spinal/traumatic injuries may be present.
✕ **Do NOT approach high-voltage sources** — call 101, 102, and 103; maintain safe distance.
✕ **Do NOT stop CPR** based on apparent lack of vital signs — electrocution victims can be resuscitated after prolonged efforts.
✕ **Do NOT give food or drink** to a child in shock after electrocution.

Equipment needed:

— Non-conductive object (wooden broom handle, dry stick, plastic chair)
— AED / defibrillator (with pediatric pads if available)
— Cool running water source (for burn cooling)
— Sterile non-adherent dressings
— Blanket (for warmth and shock treatment)

Hebrew Terminology — מונחים בעברית

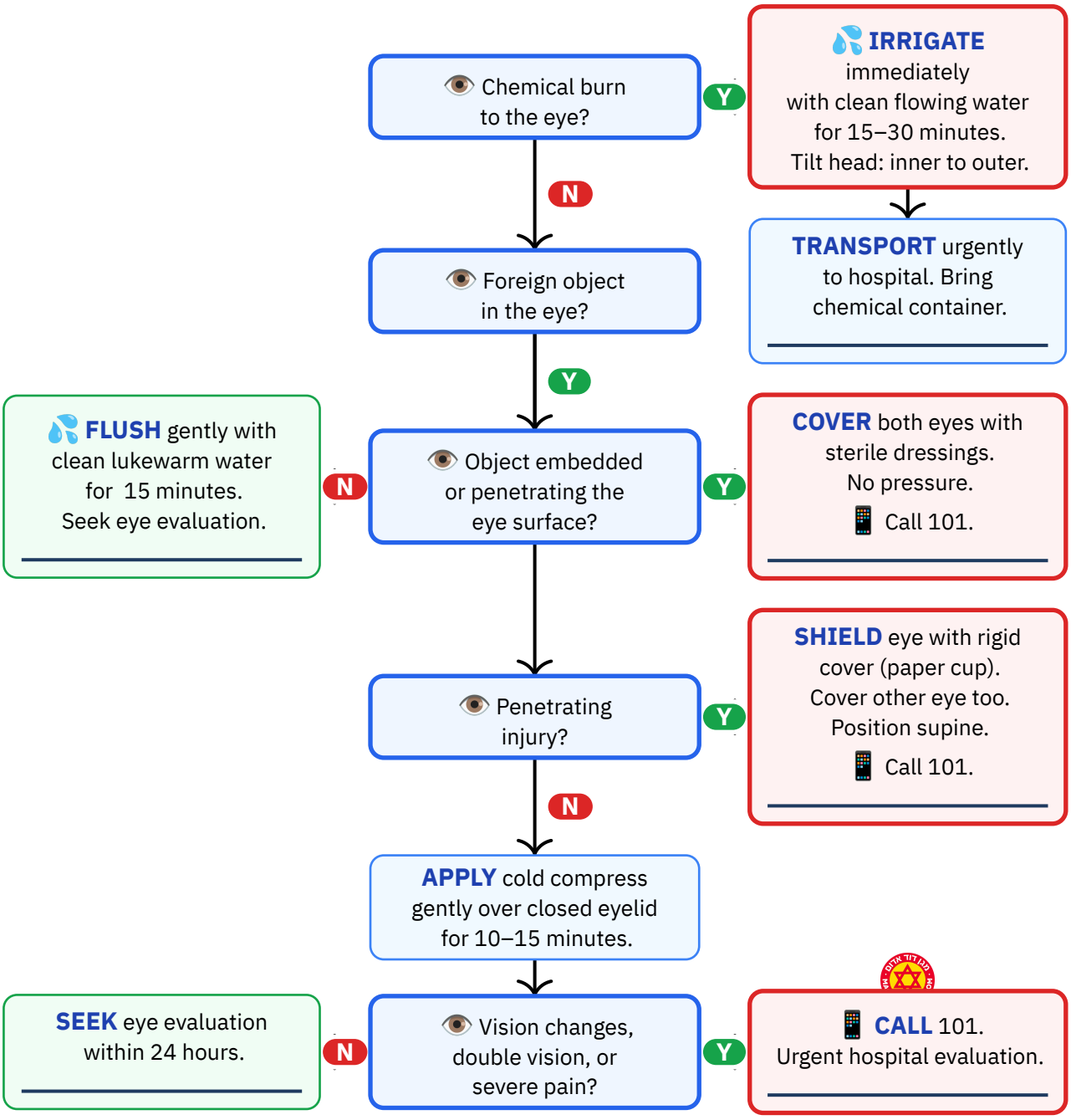
English	עברית	Pronunciation
Electrocution	התחשמלות	hit-khash-me-LUT
Electric shock	מכת חשמל	ma-KAT khash-MAL
Circuit breaker	מפסק ראשי	maf-SEK ra-shi
Power line	קו חשמל	kav khash-MAL
Burn	כוויה	kvi-YA
Shock (medical)	הלם	HE-lem
CPR	החייאה	ha-kha-ya-A
Defibrillator	דפיברילטור	de-fib-ri-LA-tor
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Turn off the electricity	תכבה את החשמל	te-kha-BE et ha-khash-MAL
The child was electrocuted	הילד התחשמל	ha-YE-led hit-khash-MEL

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Source: Magen David Adom (MDA) — Electrocution First Aid Guidelines — Pediatric
URL: https://www.mdais.org/101/electrocution
Publication date: 2026-01-01
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👁️ Chemical burn to the eye?
Y
💦 IRRIGATE immediately with clean flowing water for 15–30 minutes. Tilt head: inner to outer.
TRANSPORT urgently to hospital. Bring chemical container.
N
👁️ Foreign object in the eye?
Y
💦 FLUSH gently with clean lukewarm water for 15 minutes. Seek eye evaluation.
N
👁️ Object embedded or penetrating the eye surface?
Y
COVER both eyes with sterile dressings. No pressure.
📱 Call 101.
👁️ Penetrating injury?
Y
SHIELD eye with rigid cover (paper cup). Cover other eye too. Position supine.
📱 Call 101.
N
APPLY cold compress gently over closed eyelid for 10–15 minutes.
SEEK eye evaluation within 24 hours.
N
👁️ Vision changes, double vision, or severe pain?
Y
📱 CALL 101. Urgent hospital evaluation.

Reference Notes

DO NOT:

✕ **Do NOT rub the injured eye** — worsens damage from foreign objects and chemical burns.
✕ **Do NOT apply pressure or bandages to a penetrating eye injury.**
✕ **Do NOT attempt to remove an embedded object** from the eye.
✕ **Do NOT delay irrigation for chemical burns** — begin immediately.
✕ **Do NOT use any eye drops or ointments** without medical direction.
✕ **Do NOT allow the child to rub or touch the injured eye.**
✕ **Do NOT apply antiseptic or alcohol** to the eye.

Equipment needed:

— Clean water source (tap water or sterile water for irrigation)
— Rigid eye shield or paper cup
— Sterile gauze or clean cloth
— Cold compress or ice wrapped in cloth
— Phone (to call 101)

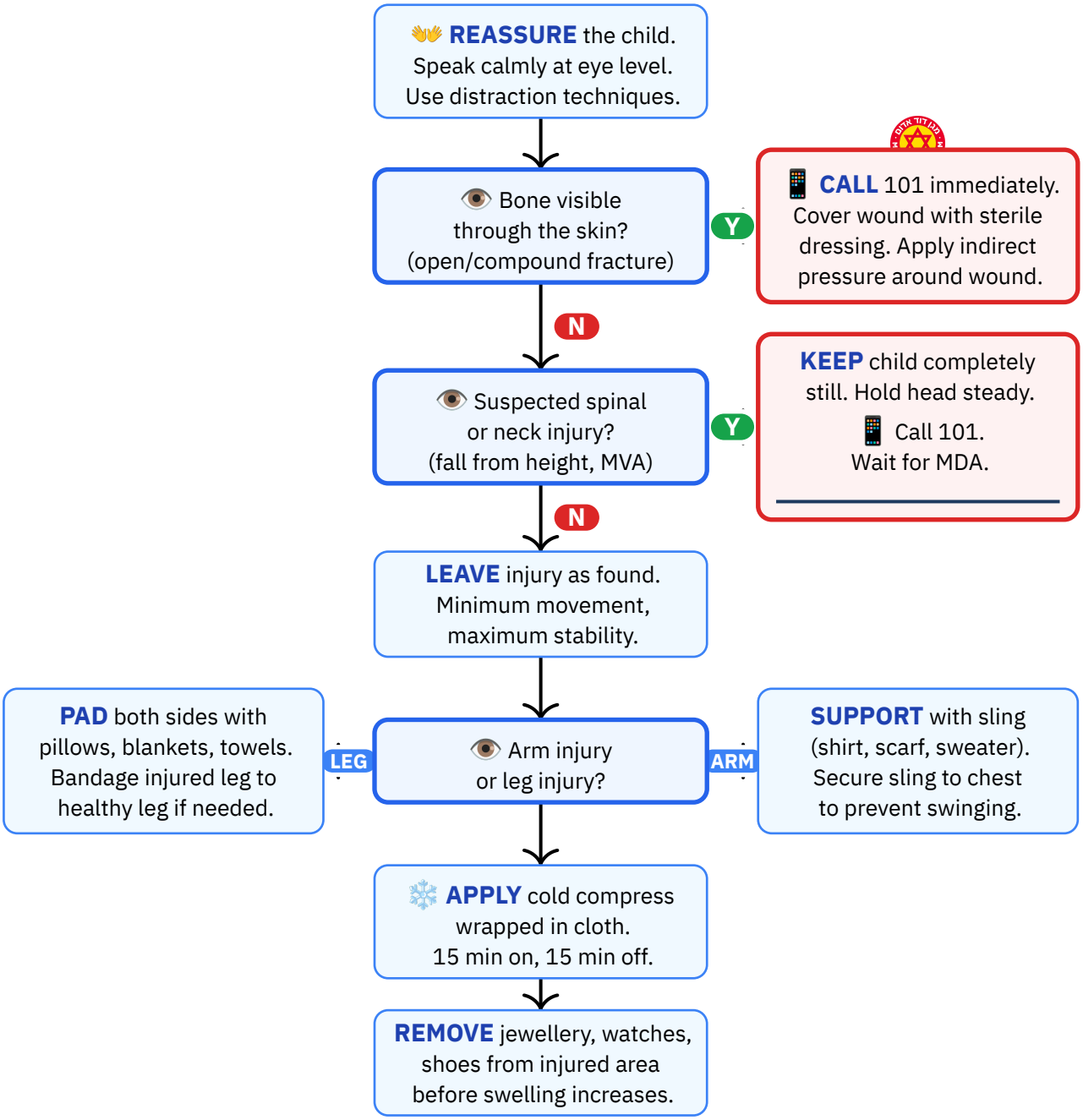
Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Eye	עין	A-yin
Eye injury	פגיעה בעין	pgi-A ba-A-yin
Chemical burn	כוויה כימית	kvi-YA khi-MIT
Foreign object	גוף זר	guf ZAR
Irrigate / rinse	לשטוף	lish-TOF
Water	מים	MA-yim
Bandage	תחבושת	takh-BO-shet
Vision	ראייה	re-i-YA
Pain	כאב	ke-EV
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Something got in the eye	נכנס משהו לעין	nikh-NAS MA-she-hu la-A-yin
Rinse with water	תשטוף עם מים	tish-TOF im MA-yim

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Source: Magen David Adom (MDA) — Eye Injury First Aid Guidelines — Pediatric
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
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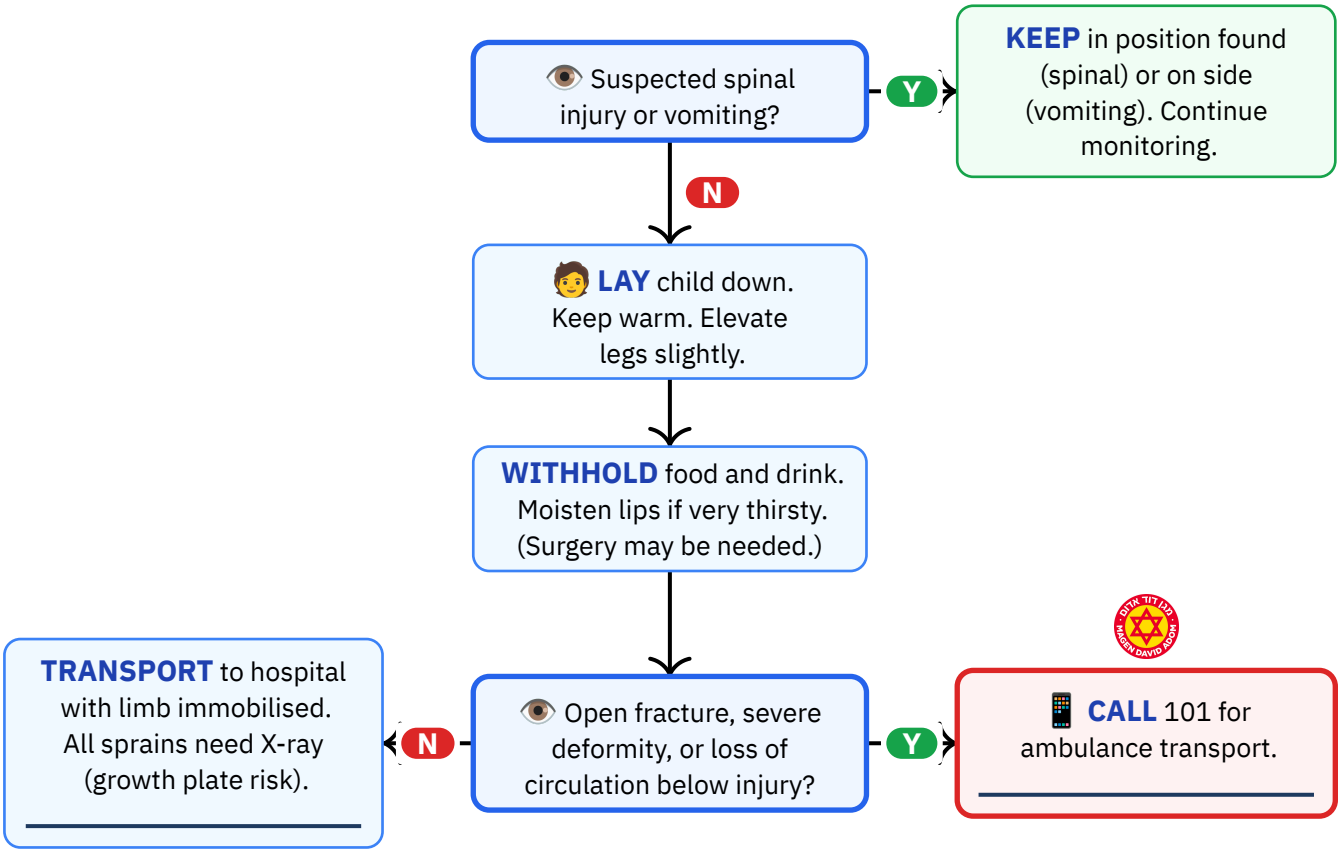
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Shock Prevention & Transport



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Reference Notes

DO NOT:

✕ **Do NOT attempt to straighten, pull, or realign a broken bone** — can damage arteries, nerves, and growth plates.
✕ **Do NOT move the child unnecessarily**, especially if spinal injury is suspected.
✕ **Do NOT apply heat** to the injury (no hot compresses, no warm water).
✕ **Do NOT apply ice directly to skin** — always wrap in cloth first.
✕ **Do NOT tie splints or bandages too tightly** — monitor circulation below the injury.
✕ **Do NOT allow the child to bear weight** on an injured leg.
✕ **Do NOT remove clothing by pulling over the injury** — cut it away if necessary.
✕ **Do NOT give aspirin to children** (Reye's syndrome risk); use paracetamol (Acamol) or ibuprofen (Nurofen for Children).
✕ **Do NOT push a protruding bone back** into the wound in an open fracture.
✕ **Do NOT give the child food or drink** — surgery under anaesthesia may be required.

Equipment needed:

— Improvised sling material (shirt, scarf, sweater, triangular bandage)
— Improvised splint material (cardboard, rolled newspapers, rulers)
— Padding material (clothing, towels, blankets)
— Ice pack or cold compress (or bag of frozen vegetables)
— Cloth or towel for wrapping ice pack
— Sterile dressings or clean cloth (for open fractures)
— Bandages or fabric strips for securing splints
— Blanket for warmth and shock prevention
— Tourniquet (for severe uncontrollable bleeding, if available)

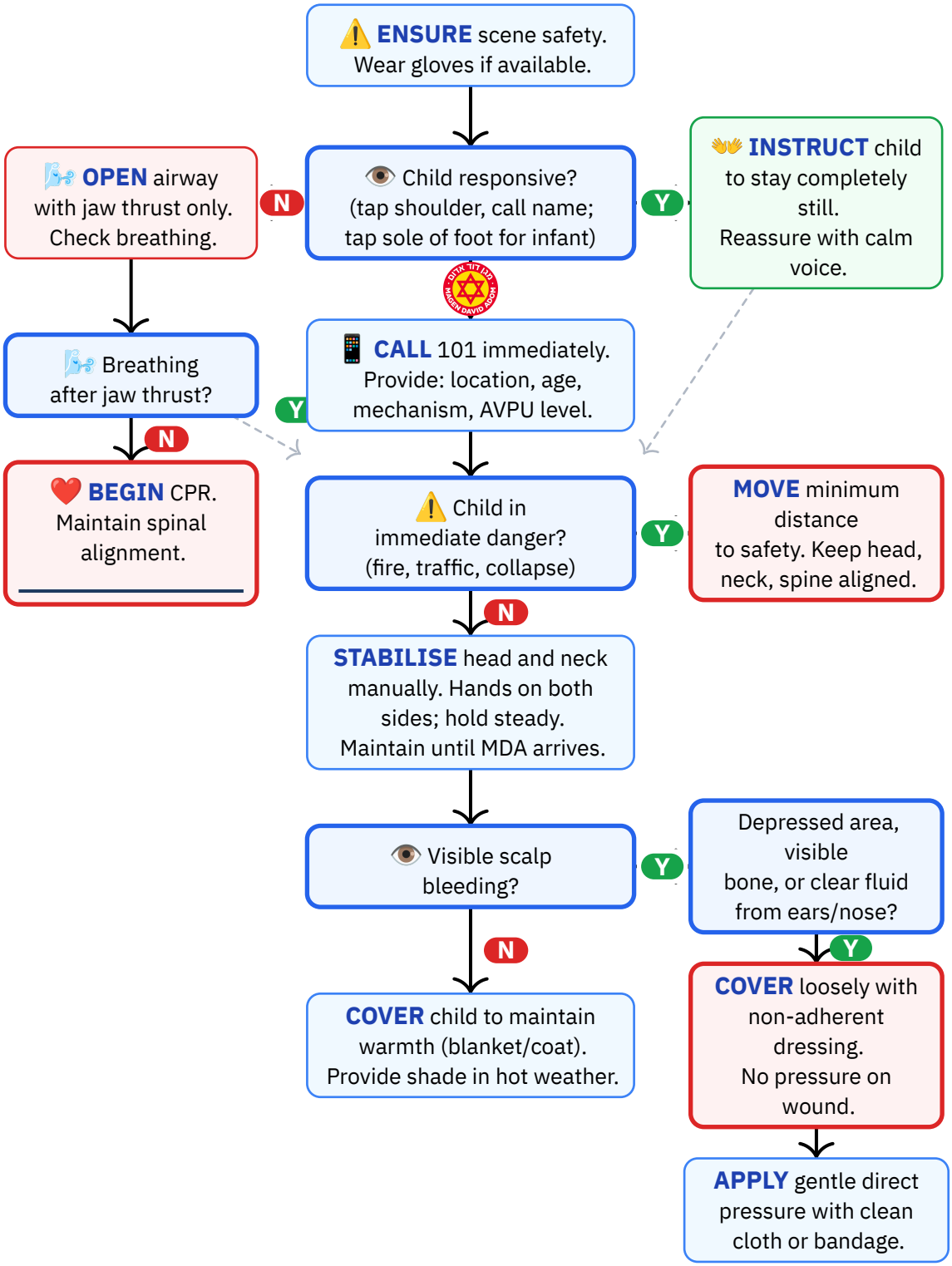
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Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Fracture / break	שבר	SHE-ver
Sprain	נקע	NE-ka
Splint	סד	SAD
Sling	מתלה	mit-LE
Bone	עצם	E-tsem
Swelling	נפיחות	ne-fi-KHUT
Pain	כאב	ke-EV
Growth plate	לוחית צמיחה	lu-KHIT tsmi-KHA
Open fracture	שבר פתוח	SHE-ver pa-TU-akh
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
It hurts here	כואב לי פה	ko-EV li PO
Can you move it?	אתה יכול לזוז?	a-TA ya-KHOL la-ZUZ?

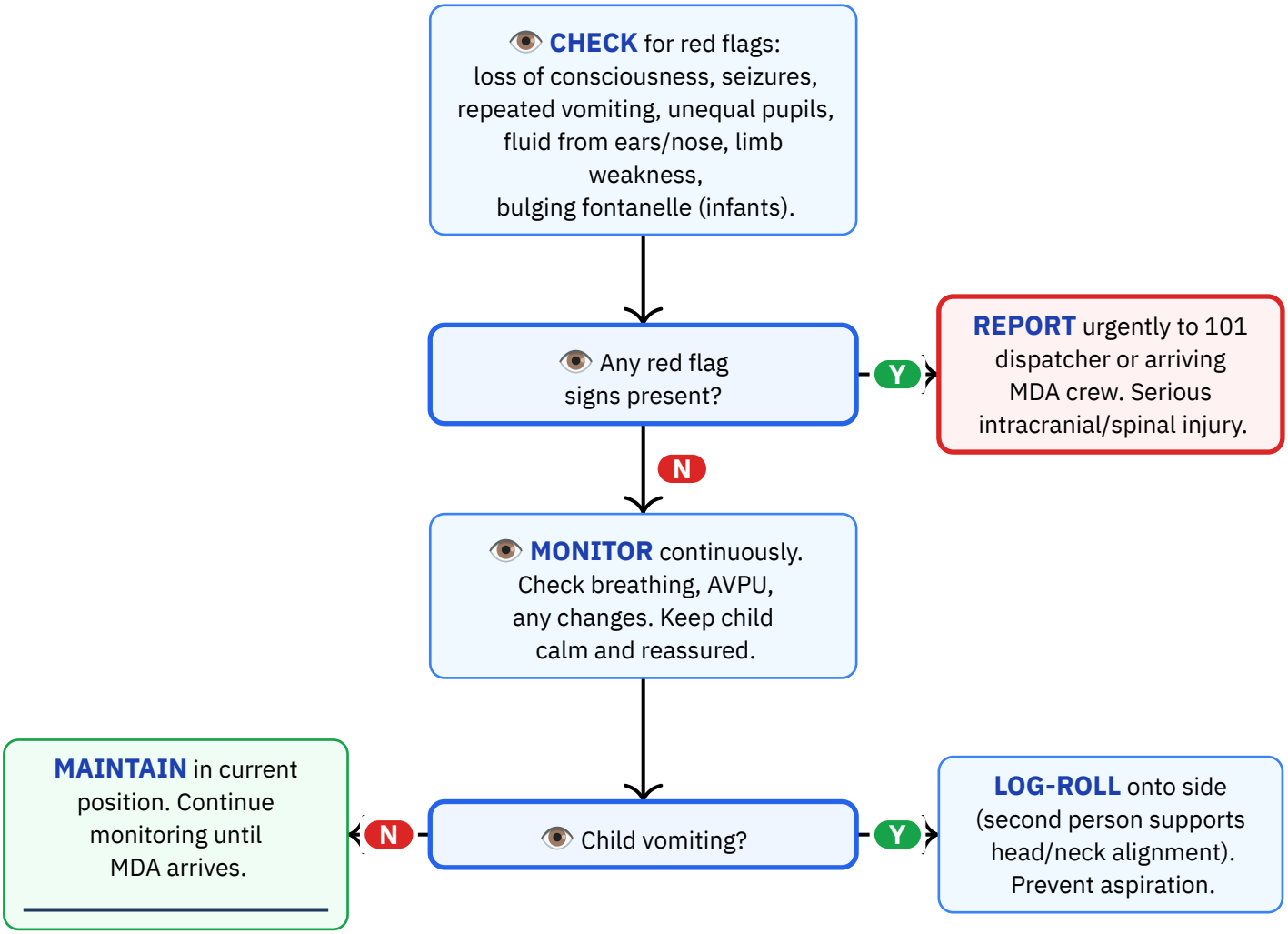
Source: Magen David Adom (MDA) — Fractures and Sprains First Aid Protocol — Children
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-16

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Continued Monitoring



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Reference Notes

DO NOT:

- ✕ **Do NOT move the child** unless in immediate life-threatening danger (fire, traffic, structural collapse).
- ✕ **Do NOT remove a helmet** — leave for MDA paramedics (requires two trained rescuers).
- ✕ **Do NOT allow the child to stand or walk** — standing/walking does NOT exclude spinal injury.
- ✕ **Do NOT give food or drink** — imaging or surgery under anaesthesia may be needed.
- ✕ **Do NOT apply pressure to a depressed skull fracture** or push back any protruding matter.
- ✕ **Do NOT insert anything into a wound.**
- ✕ **Do NOT attempt to straighten the neck** if in an abnormal position — hold as found.
- ✕ **Do NOT use head-tilt chin-lift** to open the airway — use jaw thrust only.
- ✕ **Do NOT shake the child** to assess responsiveness.
- ✕ **Do NOT leave the child alone** — continuous monitoring is essential.
- ✕ **Do NOT stop the flow of clear fluid** from ears or nose (may be cerebrospinal fluid).

Equipment needed:

— Disposable gloves
— Clean cloth or sterile bandages for wound control
— Blanket or coat for warmth

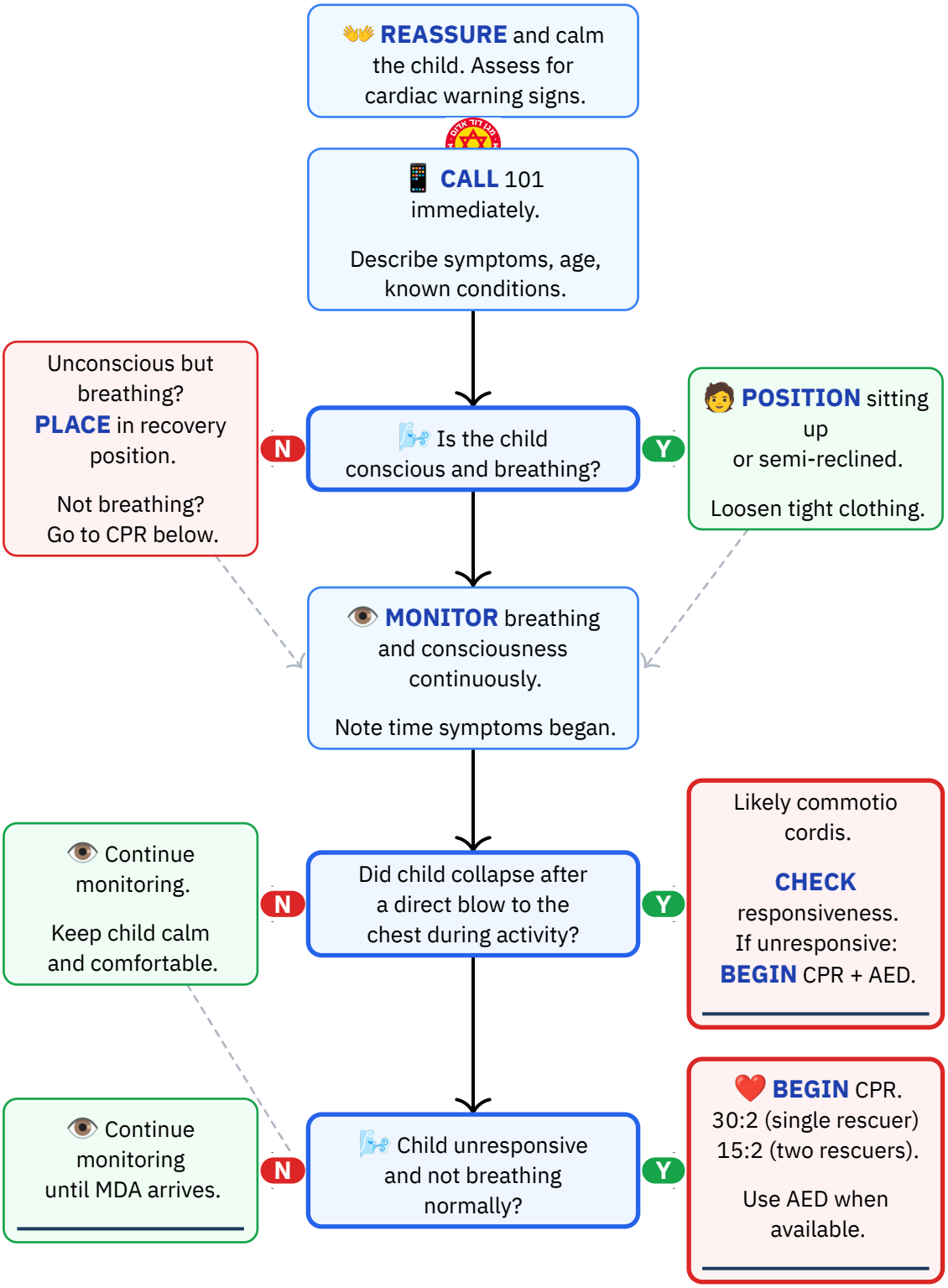
Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Head injury	פגיעת ראש	pgi-AT ROSH
Spinal injury	פגיעת עמוד שדרה	pgi-AT a-MUD shid-RA
Spine	עמוד שדרה	a-MUD shid-RA
Neck	צוואר	tsa-VAR
Helmet	קסדה	kas-DA
Consciousness	הכרה	ha-ka-RA
Unconscious	חסר הכרה	kha-SAR ha-ka-RA
Vomiting	הקאות	ha-ka-OT
Seizure	פרכוס	pir-KUS
Fontanelle	מרפס	mar-PES
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Do not move the child	אל תזיזו את הילד	al ta-ZI-zu et ha-YE-led
The child fell	הילד נפל	ha-YE-led na-FAL

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Source: Magen David Adom (MDA) — MDA BLS Guide (2016), MDA ALS Professional Manual (2024)
URL: https://www.mdais.org/101/first-aid
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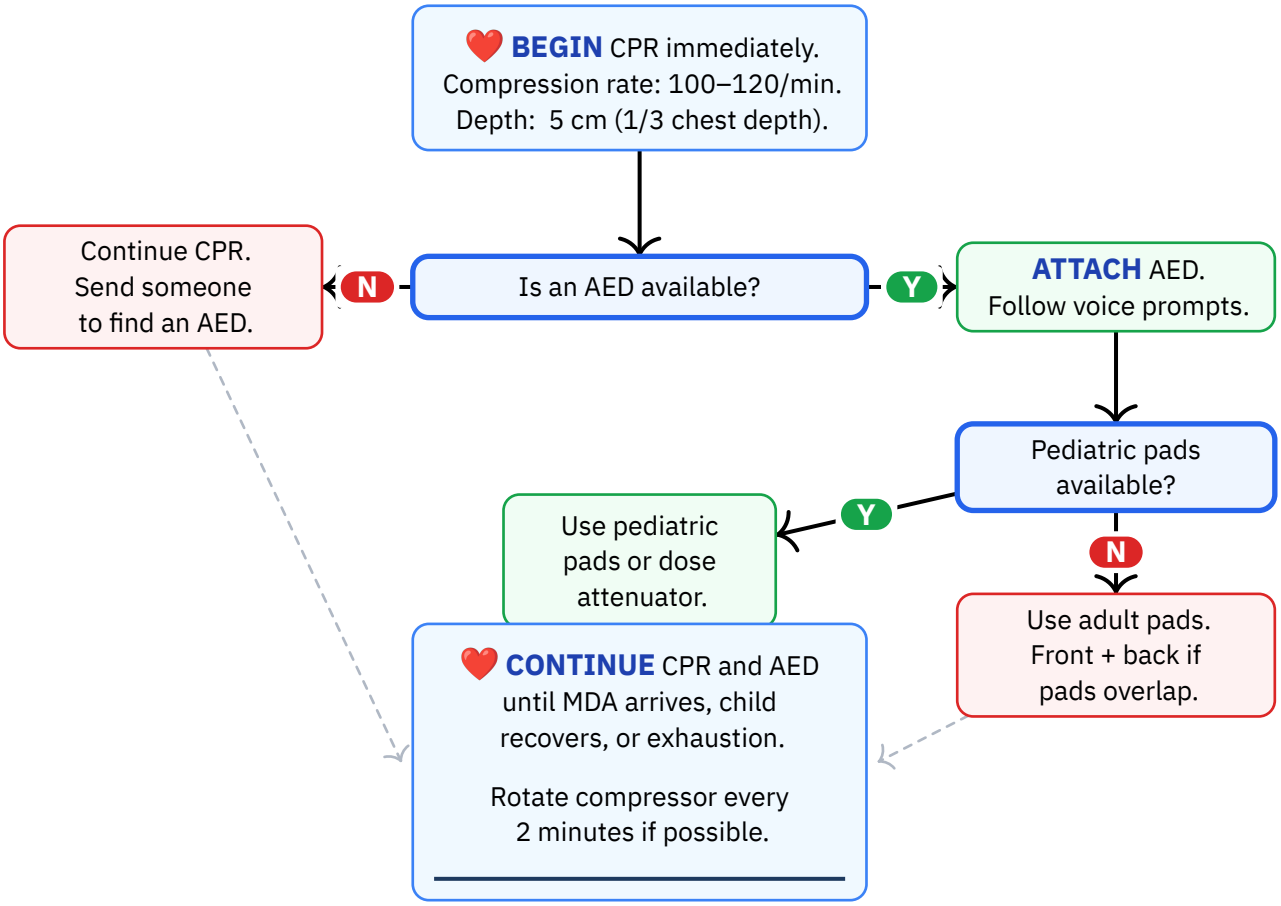
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CPR and AED Use



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Reference Notes

DO NOT:

✕ **Do NOT give aspirin to a child** — not indicated for pediatric chest pain; risk of Reye's syndrome.
✕ **Do NOT give nitroglycerin or cardiac medication** unless prescribed and directed by a physician.
✕ **Do NOT dismiss chest pain** in a child who has been ill, has a known heart condition, collapsed during activity, or has family history of sudden cardiac death.
✕ **Do NOT drive the child to hospital yourself** if cardiac arrest is suspected — call 101 for an equipped ambulance.
✕ **Do NOT leave the child unattended** while waiting for the ambulance.
✕ **Do NOT give the child food or drink** (in case hospital procedures are needed).
✕ **Do NOT give any medications** unless instructed by the 101 dispatcher or a physician.

Equipment needed:

— AED with pediatric pads or dose attenuator (if available)
— Phone to call 101 (MDA) or 1221 (United Hatzalah)
— MDA app (for AED locator and emergency calling)

Hebrew Terminology — מונחים בעברית

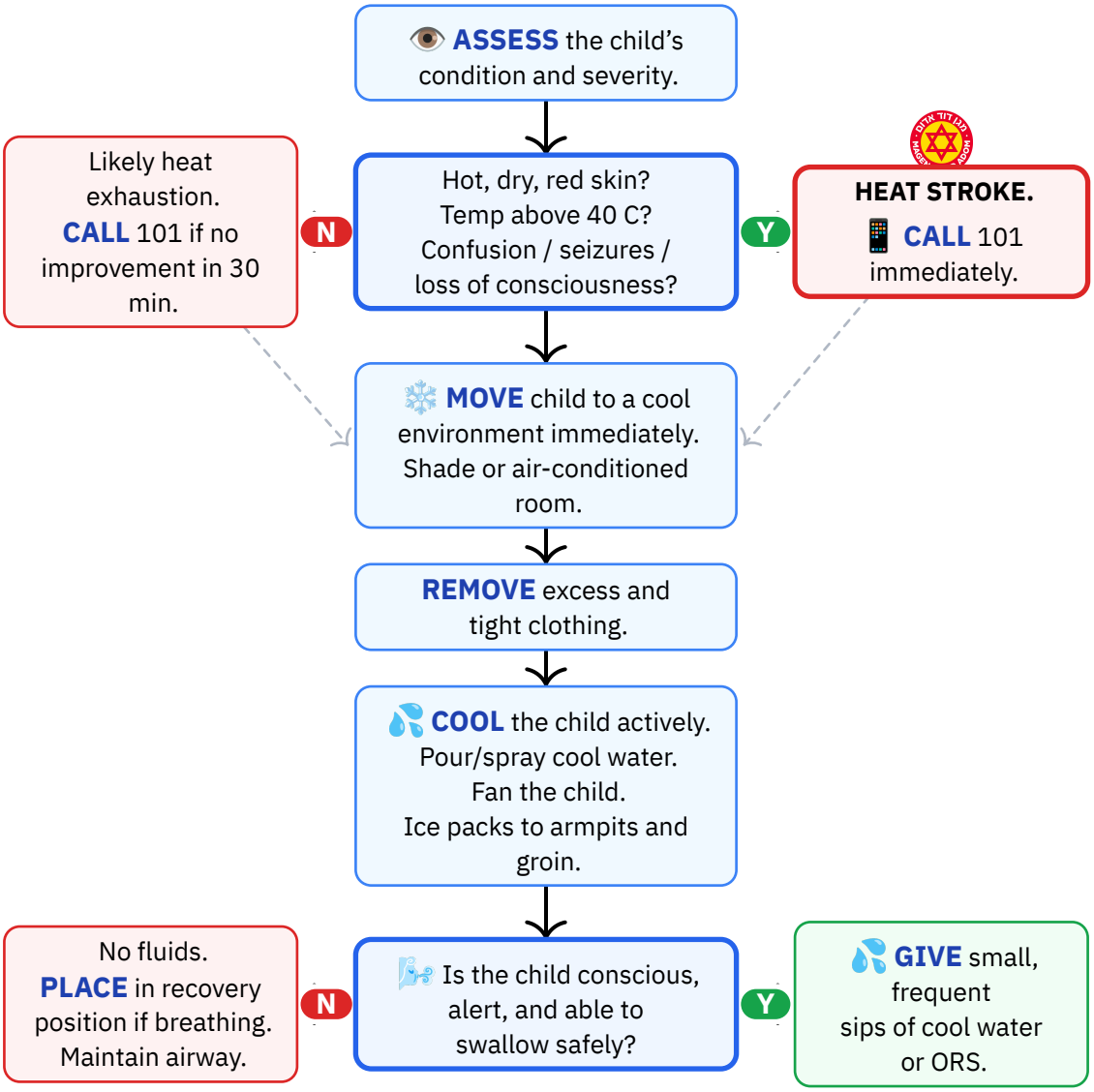
English	עברית	Pronunciation
Heart attack	התקף לב	het-KEF LEV
Chest pain	כאב בחזה	ke-EV ba-kha-ZE
Hard to breathe	קשה לנשום	ka-SHE lin-SHOM
CPR	החייאה	ha-kha-ya-A
Defibrillator	דפיברילטור	de-fib-ri-LA-tor
Unconscious	מחוסר הכרה	me-khu-SAR ha-ka-RA
Pulse	דופק	DO-fek
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Child	ילד / ילדה	YE-led / yal-DA
Does the chest hurt?	כואב לך בחזה?	ko-EV le-KHA ba-kha-ZE?
Can you breathe?	אתה יכול לנשום?	a-TA ya-KHOL lin-SHOM?

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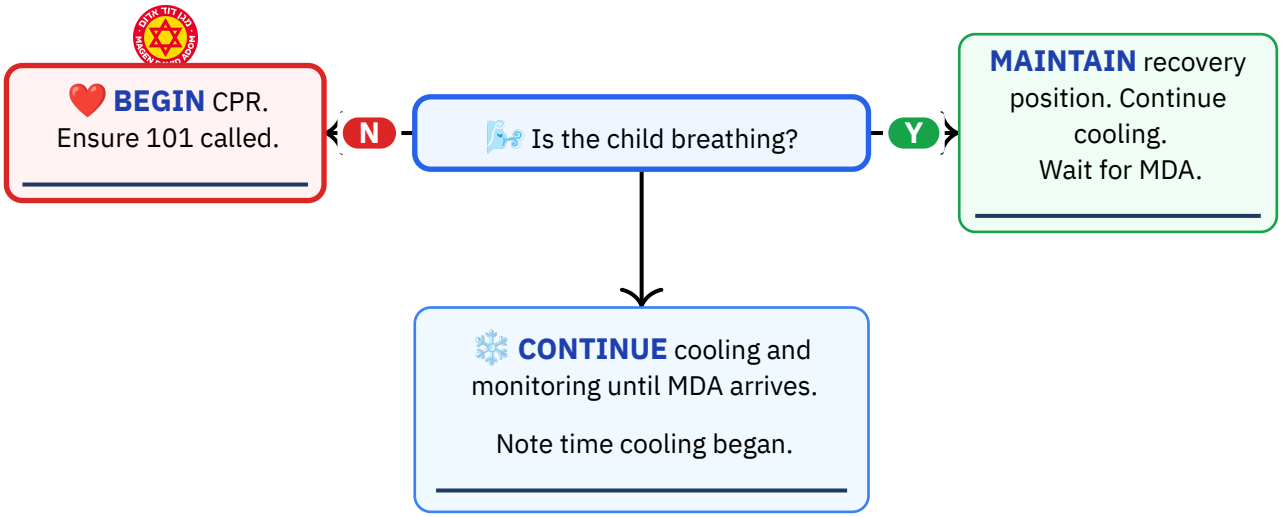
Source: Magen David Adom (MDA) — Heart Attack and Chest Pain in Children
URL: https://www.mdais.org/101/first-aid
Publication date: 2022-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-16

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👁️ ASSESS the child's condition and severity.
Hot, dry, red skin? Temp above 40 C? Confusion / seizures / loss of consciousness?
N
Likely heat exhaustion. CALL 101 if no improvement in 30 min.
Y
HEAT STROKE.
📱 CALL 101 immediately.
❄️ MOVE child to a cool environment immediately. Shade or air-conditioned room.
REMOVE excess and tight clothing.
💦 COOL the child actively.
Pour/spray cool water.
Fan the child.
Ice packs to armpits and groin.
🌬️ Is the child conscious, alert, and able to swallow safely?
N
No fluids. PLACE in recovery position if breathing. Maintain airway.
Y
💦 GIVE small, frequent sips of cool water or ORS.

Airway and Continued Monitoring



Child found in vehicle

Temperatures inside a locked car in Israeli summer can reach 70 C.
REMOVE the child from the vehicle immediately – break the window if necessary.
Report to MDA that the child was found in a vehicle, including estimated time of exposure.

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Reference Notes

DO NOT:

✕ **Do NOT give fever-reducing medications** (Acamol, Nurofen, naproxen) — these do not reduce heatstroke temperature and may cause harm.
✕ **Do NOT use ice-cold water immersion** for young children without medical supervision.
✕ **Do NOT force fluids** if the child is vomiting or has altered consciousness.
✕ **Do NOT leave the child unattended** at any point.
✕ **Do NOT give fluids to an unconscious child.**
✕ **Do NOT delay calling 101** if heat stroke is suspected.
✕ **Do NOT leave a child unattended in a vehicle** — even for a moment, even with windows open or A/C running.

Equipment needed:

— Cool water (for pouring or spraying over the body)
— Fan or means of fanning
— Ice packs or cold wet cloths
— Oral rehydration solution (ORS) or cool drinking water
— Thermometer (if available)

Hebrew Terminology — מונחים בעברית

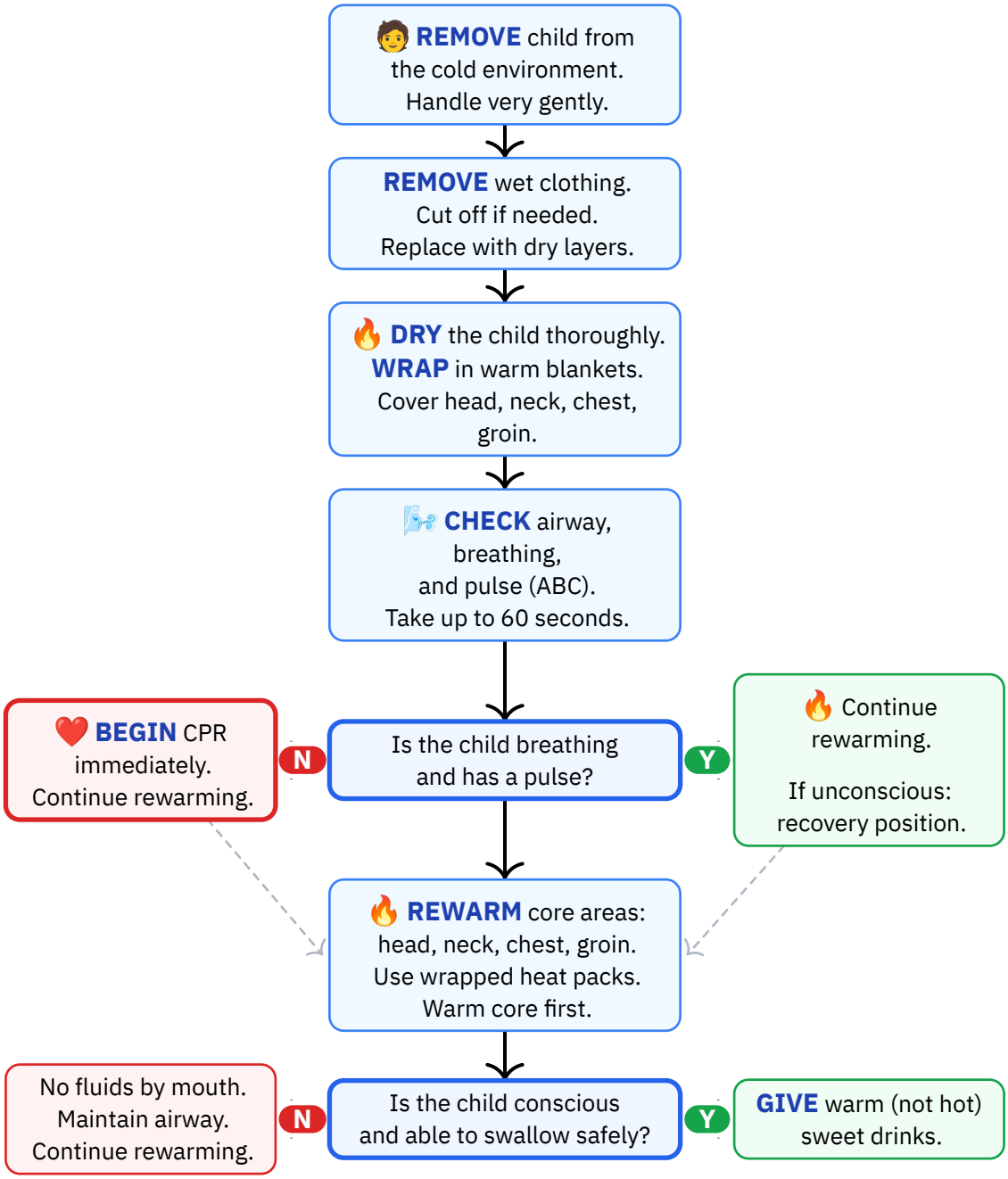
English	עברית	Pronunciation
Heatstroke	מכת חום	ma-KAT KHOM
Heat exhaustion	תשישות חום	tshi-SHUT KHOM
Water	מים	MA-yim
Shade	צל	TSEL
Hot	חם	KHAM
Cold water	מים קרים	MA-yim ka-RIM
Unconscious	מחוסר הכרה	me-khu-SAR ha-ka-RA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Child	ילד / ילדה	YE-led / yal-DA
Are you thirsty?	אתה צמא?	a-TA tsa-ME?
Drink water	תשתה מים	tish-TE MA-yim

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Source: Magen David Adom (MDA) — Heat Stroke Guidelines
URL: https://www.mdais.org/news/300820
Publication date: 2024-04-01
Imported: 2026-03-15 · **Last verified:** 2026-03-16

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Transport and Monitoring

📱 **CALL** 101 and transport urgently to hospital. Keep child horizontal.

↓

👁️ **MONITOR** breathing, pulse, and consciousness continuously until handover.

Be prepared to begin CPR if child deteriorates.

Hypothermia Severity Stages

Stage	Temperature	Signs
Stage 1	37–35 C	Shivering, normal judgement
Stage 2	35–32 C	Severe shivering, confusion, slurred speech, pallor
Stage 3	32–28 C	Deteriorating consciousness, muscle rigidity
Stage 4	Below 28 C	Unconsciousness, absent reflexes, absent breathing

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Reference Notes

DO NOT:

✕ **Do NOT give alcohol, caffeine, or allow smoking** — these worsen hypothermia.
✕ **Do NOT make excessive or rough movements** — risk of cardiac arrhythmias in a cold heart.
✕ **Do NOT apply direct heat to skin** without a cloth barrier — risk of burns.
✕ **Do NOT rewarm extremities before the core** — can cause dangerous drop in core temperature as cold blood returns to the heart.
✕ **Do NOT give fluids by mouth** to an unconscious or semi-conscious child — aspiration risk.
✕ **Do NOT assume a severely hypothermic child is dead** — "no one is dead until they are warm and dead."
✕ **Do NOT use hot water immersion** for rewarming in the field — risk of burns and cardiac arrhythmias.
✕ **Do NOT jostle or move the child unnecessarily** during transport.

Equipment needed:

— Warm blankets or sleeping bags
— Dry clothing
— Heat packs or warm water bottles (wrapped in cloth)
— Phone (to call 101)

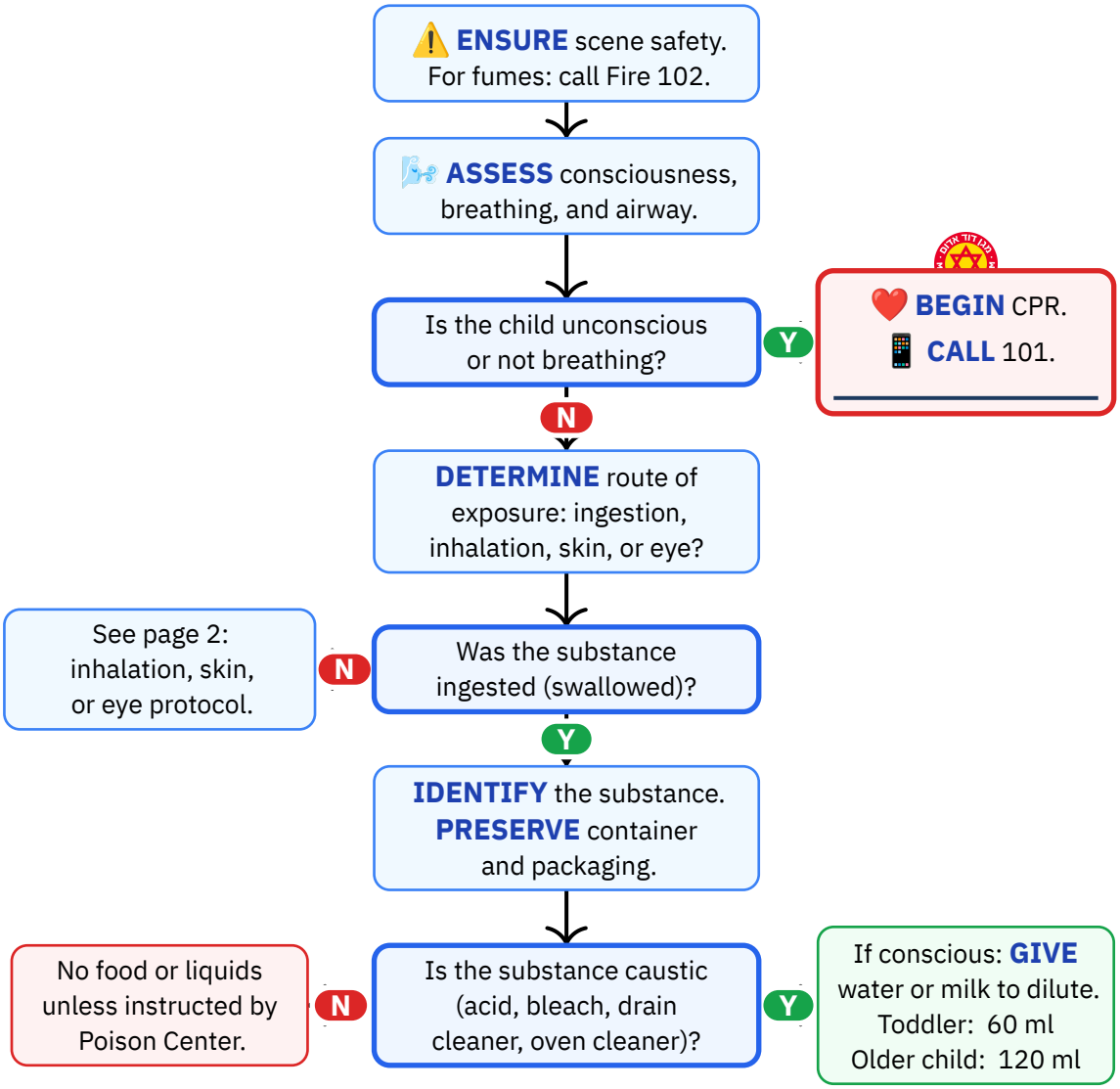
Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Hypothermia	היפותרמיה	hi-po-ter-MI-ya
Cold	קר	KAR
Blanket	שמיכה	smi-KHA
Shivering	רעד	RA-ad
Pulse	דופק	DO-fek
Breathing	נשימה	nshi-MA
Unconscious	מחוסר הכרה	me-khu-SAR ha-ka-RA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Child	ילד / ילדה	YE-led / yal-DA
Are you cold?	קר לך?	KAR le-KHA?
Warm drink	משקה חם	mash-KE KHAM

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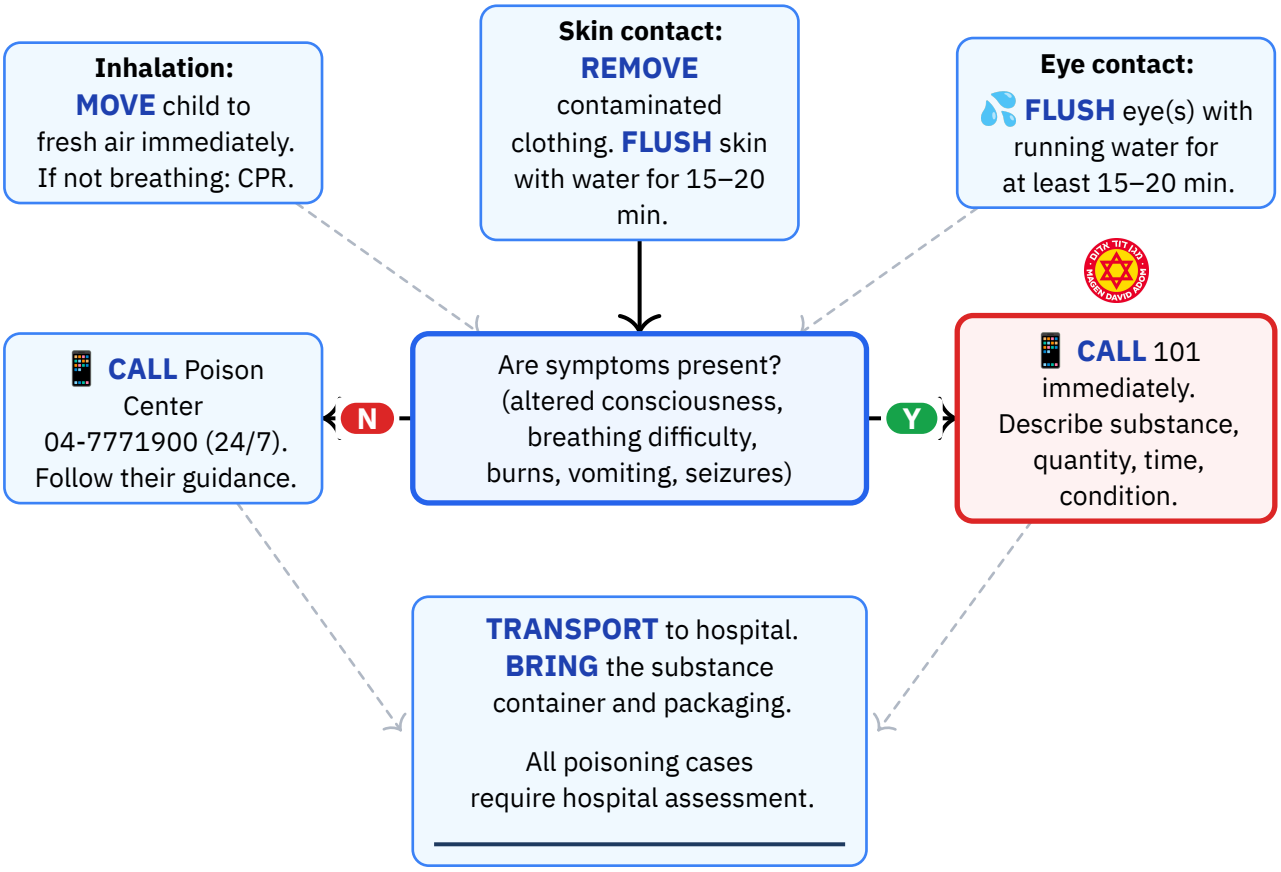
Source: Magen David Adom (MDA) — Hypothermia First Aid Guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16

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Non-Ingestion Routes and Calling for Help



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Reference Notes

DO NOT:

✕ **Do NOT induce vomiting** — regardless of the substance ingested. Causes additional oesophageal burns (caustic agents), aspiration risk, and ineffective removal.
✕ **Do NOT give activated charcoal at home** unless specifically instructed by the Poison Center or a physician.
✕ **Do NOT give food to the poisoned child.** Water or milk for caustic dilution only, when conscious.
✕ **Do NOT attempt home remedies** (raw eggs, olive oil, etc.).
✕ **Do NOT wait for symptoms** before calling for guidance — some toxic substances cause delayed organ damage (liver, kidneys).
✕ **Do NOT discard the container or packaging** — always bring it to the hospital.
✕ **Do NOT mix cleaning products** — especially bleach with acids (releases toxic chlorine gas).
✕ **Do NOT enter a toxic fume environment** without proper protection.

Equipment needed:

— Cloth (to wipe substance from child's mouth if present)
— Water or milk (for dilution of caustic substances only, age-appropriate amounts)
— Running water source (for skin and eye decontamination, 15--20 minutes flushing)
— Container or bag (to preserve substance packaging and any vomit for medical teams)

Key Phone Numbers

Service	Number
MDA (emergency)	**101**
Israel Poison Information Center (Rambam)	**04-7771900** (24/7)
United Hatzalah	**1221**
Fire Department	**102**

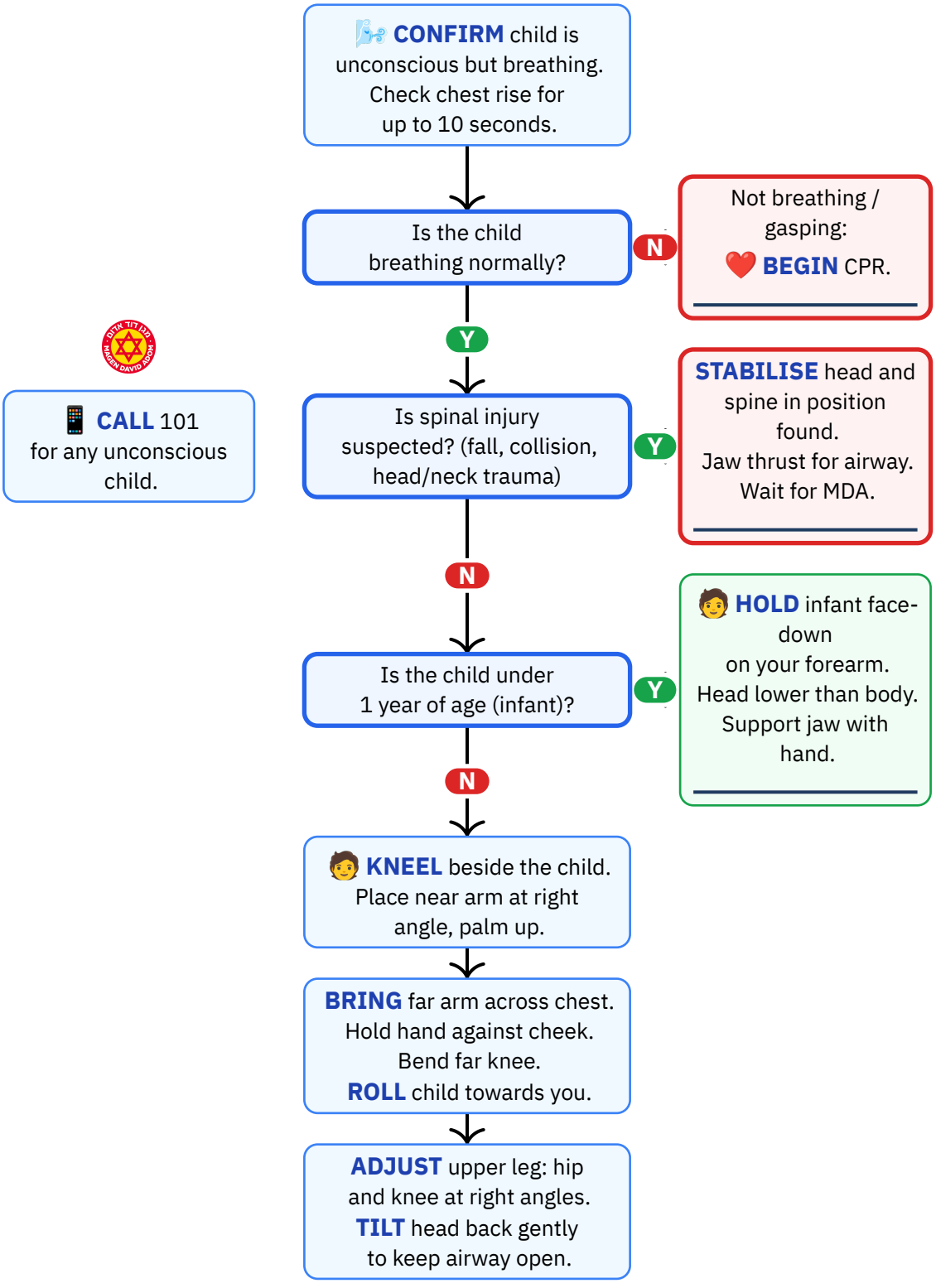
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English	עברית	Pronunciation
Poison / poisoning	הרעלה	har-a-LA
Poison center	מרכז הרעלות	mer-KAZ har-a-LOT
Swallowed	בלע	ba-LA
Cleaning product	חומר ניקוי	KHO-mer ni-KUY
Bleach (economica)	אקונומיקה	e-ko-NO-mi-ka
Vomiting	הקאה	ha-ka-A
Unconscious	מחוסר הכרה	me-khu-SAR ha-ka-RA
Child	ילד / ילדה	YE-led / yal-DA
What did they swallow?	מה הוא בלע?	MA HU ba-LA?
How much?	כמה?	KA-ma?

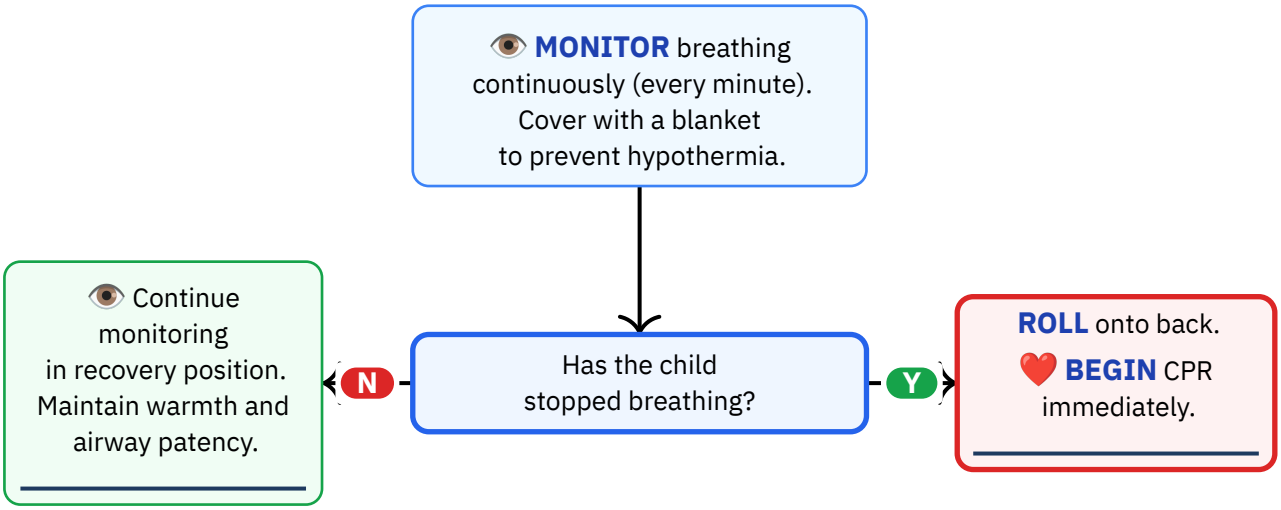
Source: Magen David Adom (MDA) — Poisoning First Aid Guidelines; Rambam National Poison Information Center
URL: https://www.mdais.org/101/poisoningn
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-16

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Monitoring



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Reference Notes

DO NOT:

✕ **Do NOT use the recovery position if the child is not breathing** — begin CPR instead.
✕ **Do NOT use the recovery position if spinal injury is suspected** — stabilise the spine.
✕ **Do NOT place infants under 1 year in the standard lateral recovery position** — use the forearm cradle position.
✕ **Do NOT leave the child unattended** in the recovery position — monitor breathing continuously.
✕ **Do NOT allow the child's head to flex forward** (especially in children with larger heads) — this can obstruct the airway.
✕ **Do NOT forget to cover the child** to prevent hypothermia.

Equipment needed:

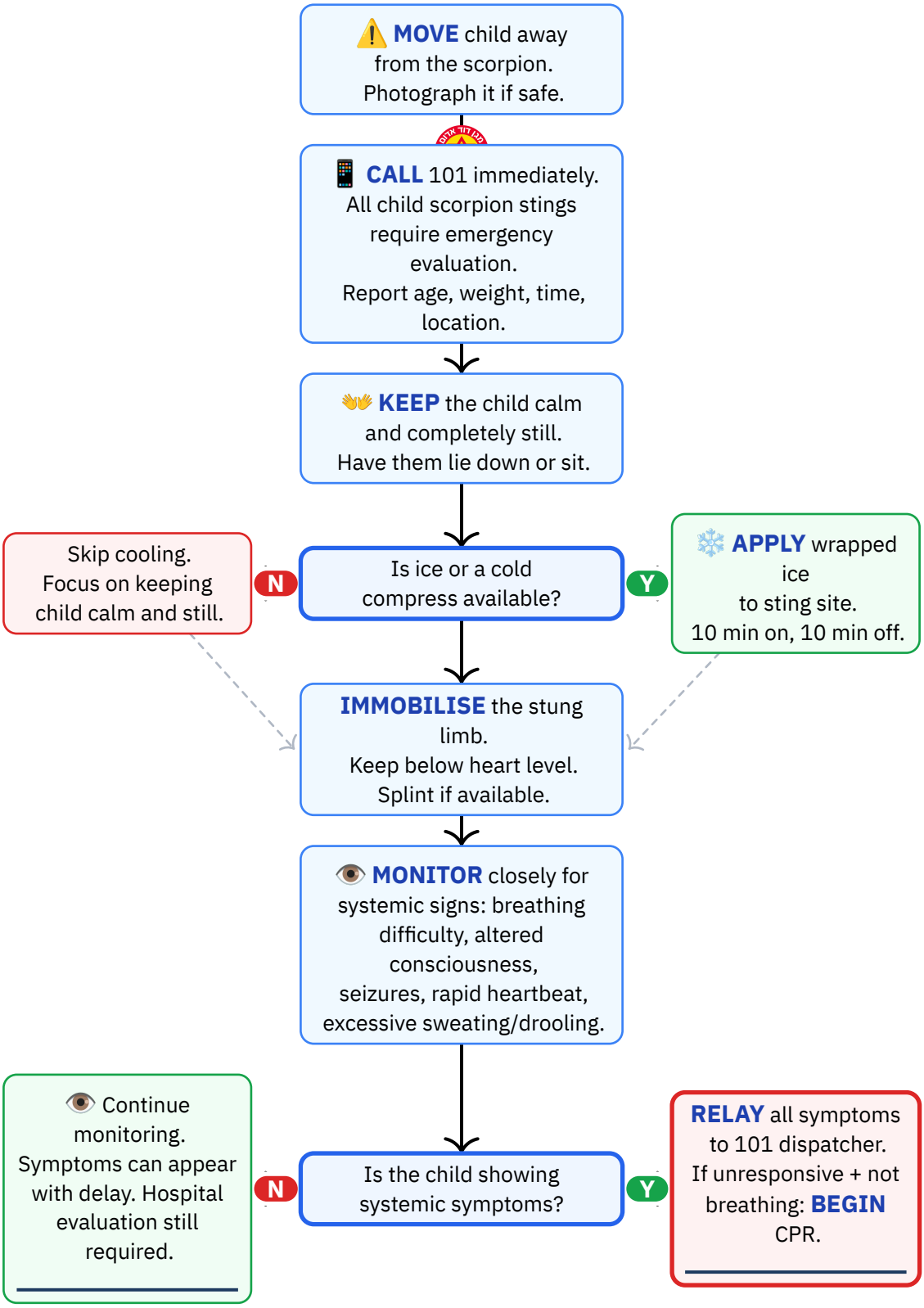
— Blanket or coat (to prevent hypothermia)
— Phone (to call 101)

Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Recovery position	תנוחת החלמה	tnu-KHAT ha-khla-MA
Unconscious	מחוסר הכרה	me-khu-SAR ha-ka-RA
Breathing	נשימה	nshi-MA
Airway	דרכי אוויר	dar-KHEY a-VIR
On their side	על הצד	al ha-TSAD
Infant	תינוק	ti-NOK
Spinal injury	פגיעה בעמוד השדרה	pgi-A be-a-MUD ha-shid-RA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Child	ילד / ילדה	YE-led / yal-DA
Is the child breathing?	הילד נושם?	ha-YE-led no-SHEM?
Roll them on their side	תגלגל אותו על הצד	te-gal-GEL o-TO al ha-TSAD

Source: Magen David Adom (MDA) — Recovery Position Guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2016-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16

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Reference Notes

DO NOT:

✕ **Do NOT apply a tourniquet** — concentrates venom in the limb and causes tissue damage.
✕ **Do NOT cut or incise the sting site** — causes additional injury and does not remove venom.
✕ **Do NOT attempt to suck out the venom** — ineffective and risks infection.
✕ **Do NOT apply alcohol or antiseptic** to the wound — interferes with medical assessment.
✕ **Do NOT give the child medication** without medical advice — may interact with venom effects or mask symptoms.
✕ **Do NOT underestimate a sting based on scorpion colour** — black scorpions can be as dangerous as yellow ones.
✕ **Do NOT delay calling 101 for children** — children deteriorate faster due to higher venom-to-body-weight ratio.
✕ **Do NOT apply ice directly to skin** — always wrap in cloth.
✕ **Do NOT allow the child to walk or move** — movement accelerates venom distribution.
✕ **Do NOT "wait and see" with children** — all scorpion stings in children require emergency medical evaluation.

Equipment needed:

— Ice pack or cold compress (wrapped in cloth)
— Splint or rigid object for limb immobilisation
— Phone (to call 101 / 04-7771900 and photograph the scorpion)
— Cloth or towel (to wrap ice pack)
— Blanket or clothing (to keep child comfortable while lying still)

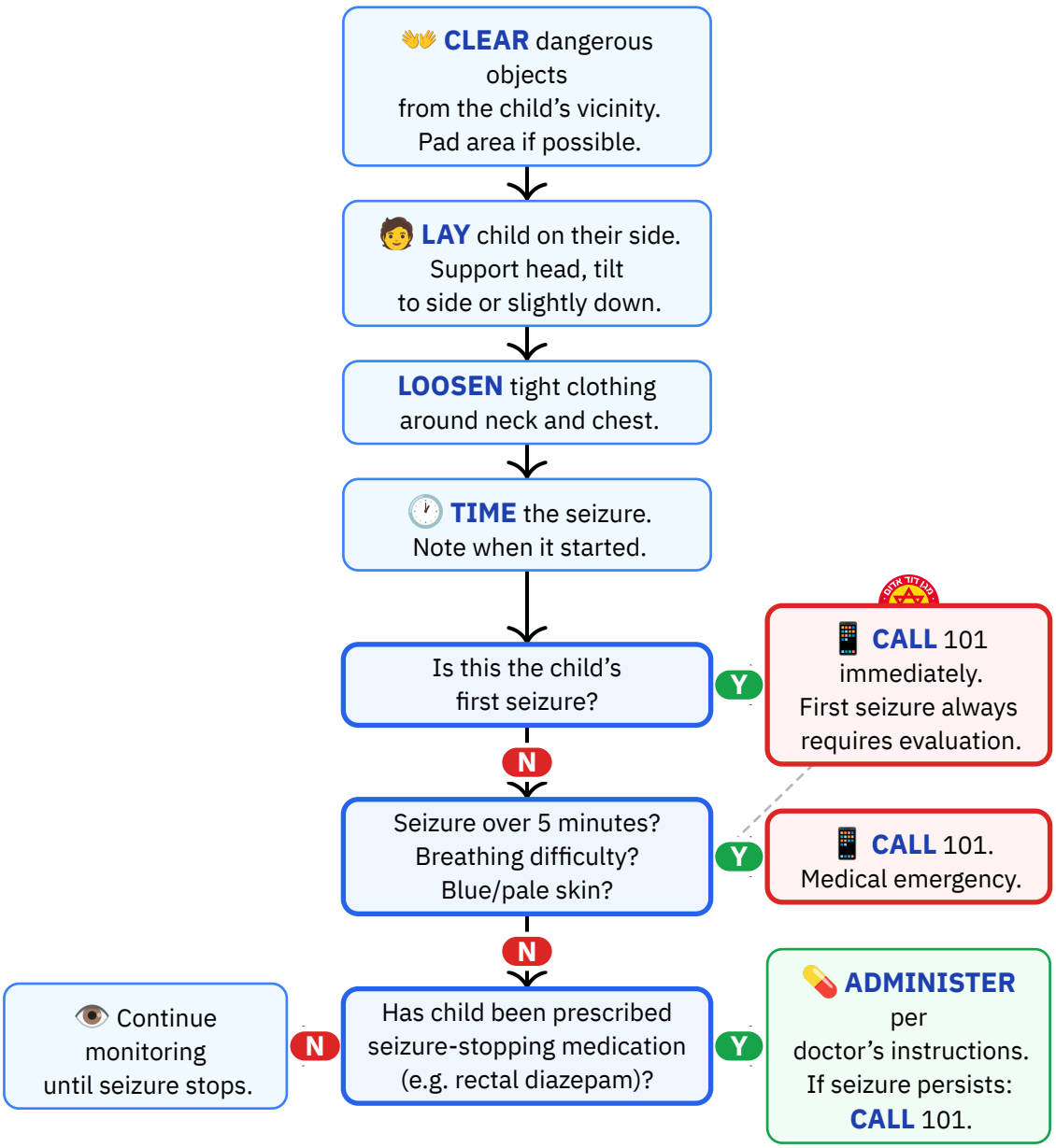
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Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Scorpion	עקרב	ak-RAV
Sting	עקיצה	a-ki-TSA
Venom / poison	ארס	E-res
Antivenom	נוגדן	no-ge-DAN
Swelling	נפיחות	ne-fi-KHUT
Pain	כאב	ke-EV
Ice	קרח	KE-rakh
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Child	ילד / ילדה	YE-led / yal-DA
A scorpion stung the child	עקרב עקץ את הילד	ak-RAV a-KATS et ha-YE-led
Where is the sting?	איפה העקיצה?	EI-fo ha-a-ki-TSA?

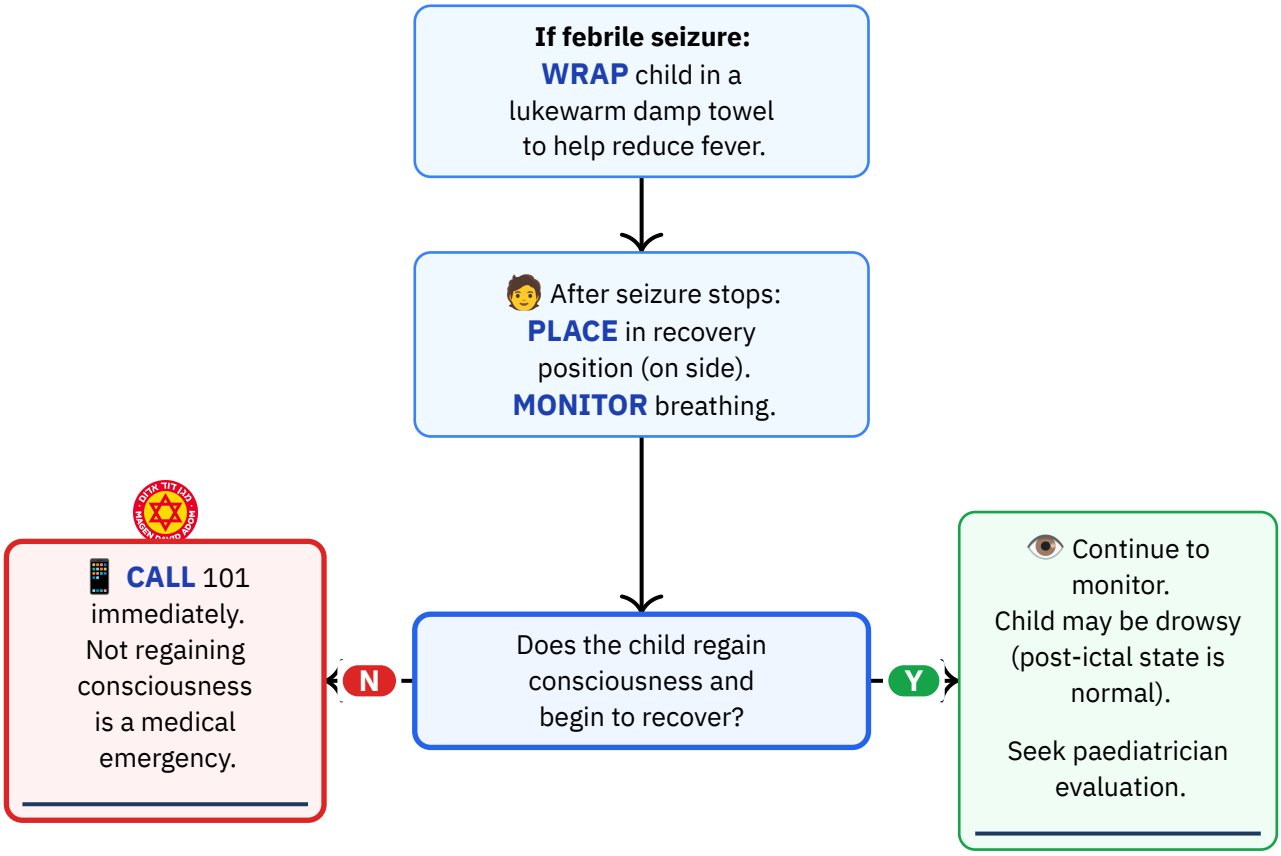
Source: Magen David Adom (MDA) — Scorpion Sting First Aid Guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-16

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Post-Seizure and Febrile Management



Post-Seizure Fever Management (Febrile Seizures)

Once the child has recovered and is no longer actively seizing:

- **ADMINISTER** age-appropriate fever-reducing medication (paracetamol or ibuprofen) as directed by the family doctor.
- Dress child in light clothing. Ventilate the room.
- Give plenty of fluids.
- A lukewarm bath (20 min) may be given only when NOT actively seizing.

Note: Antipyretics improve comfort but have NOT been proven to prevent febrile seizures (Israeli Ministry of Health).

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Reference Notes

DO NOT:

✕ **Do NOT force the child's mouth open** — the child will not swallow their tongue.
✕ **Do NOT insert anything into the mouth** — this can injure teeth or jaw.
✕ **Do NOT attempt CPR during the seizure** — this can only cause harm.
✕ **Do NOT put the child in a bath or shower during the seizure.**
✕ **Do NOT splash water on the child** during the seizure.
✕ **Do NOT try to wake the child** or stop the seizure by restraining or holding them down.
✕ **Do NOT give medications by mouth** during the seizure.
✕ **Do NOT attempt to cool the child rapidly** with cold water or ice.
✕ **Do NOT insert an airway device** during an active seizure.
✕ **Do NOT attempt forced airway opening** during an active seizure.

Equipment needed:

— Lukewarm damp towel (for febrile seizures)
— Pillows or blankets (to pad surroundings)
— Prescribed seizure-stopping medication if available (e.g., rectal diazepam)
— Fever-reducing medication for post-seizure fever management (paracetamol or ibuprofen, as directed by doctor)

Hebrew Terminology — מונחים בעברית

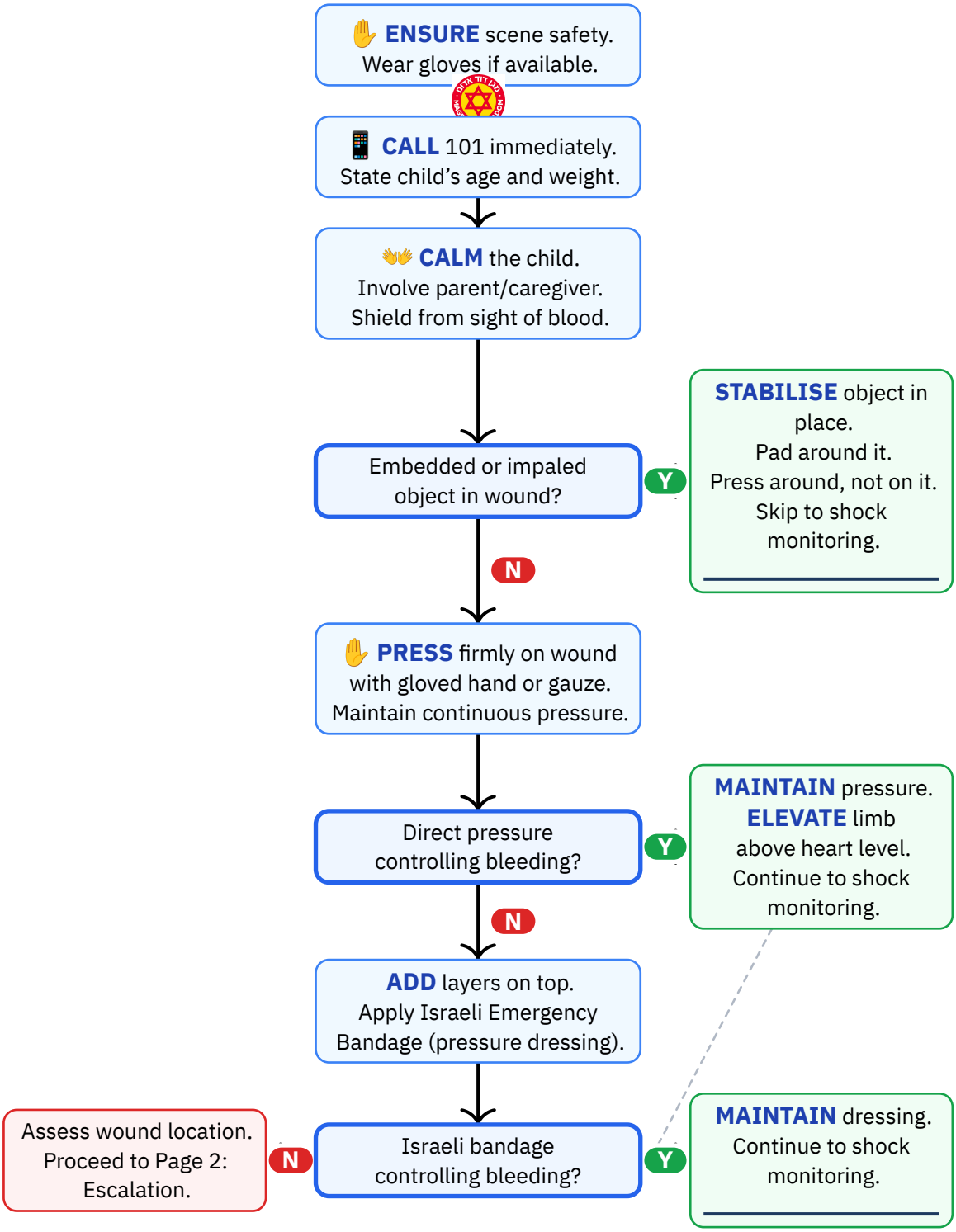
English	עברית	Pronunciation
Seizure / convulsion	פרכוס	pir-KUS
Febrile seizure	פרכוס חום	pir-KUS KHOM
Fever	חום	KHOM
Unconscious	מחוסר הכרה	me-khu-SAR ha-ka-RA
Breathing	נשימה	nshi-MA
Medication	תרופה	tru-FA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Child	ילד / ילדה	YE-led / yal-DA
The child is having a seizure	הילד מפרכס	ha-YE-led me-far-KES
How long has the seizure lasted?	כמה זמן הפרכוס?	KA-ma ZMAN ha-pir-KUS?
Does the child have a fever?	יש לילד חום?	YESH la-YE-led KHOM?

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Source: Magen David Adom (MDA) — How to Treat Febrile Seizures
URL: https://www.mdais.org/101/seizures
Publication date: 2022-01-01
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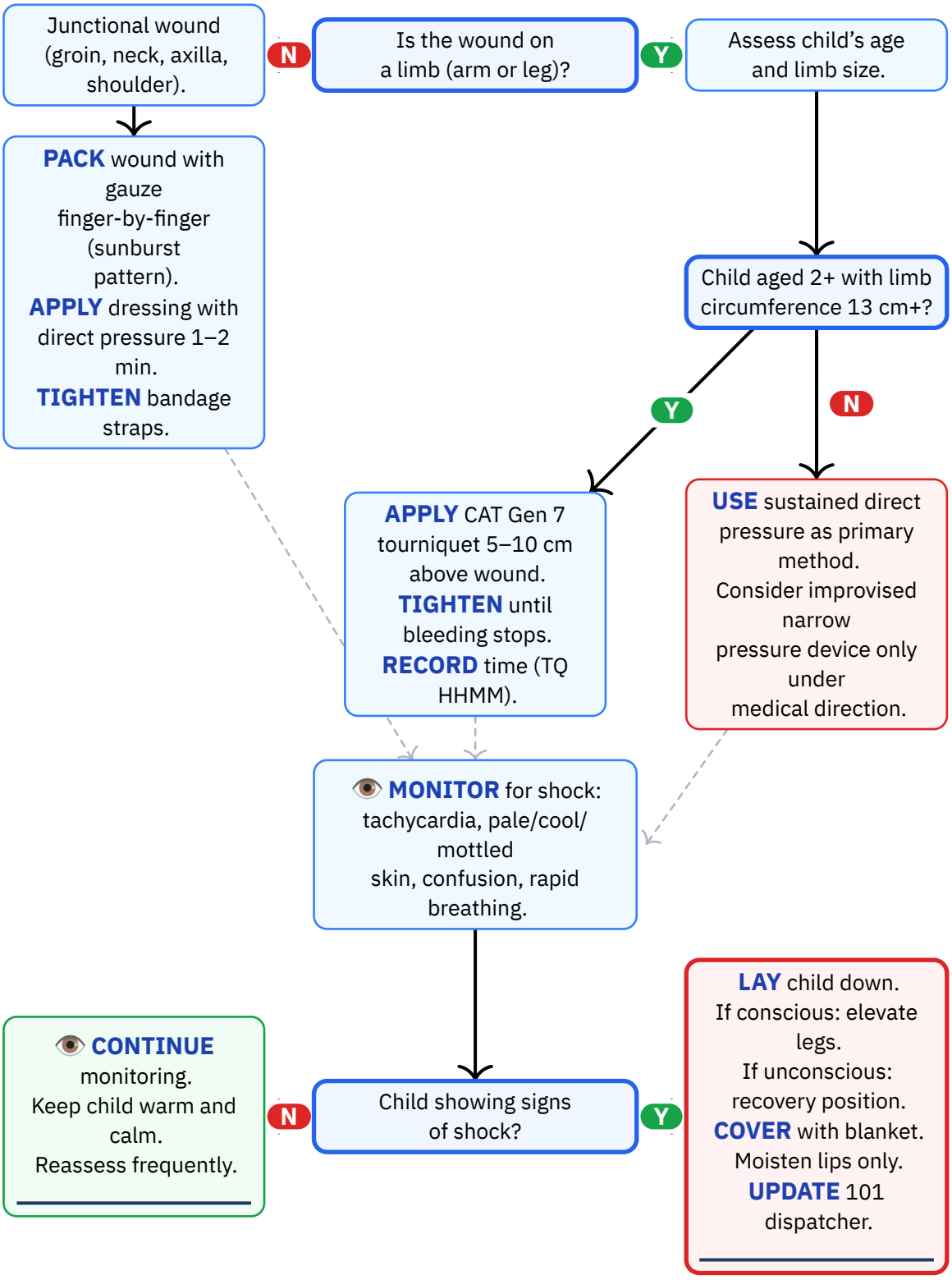
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Escalation — Wound Packing / Tourniquet



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Reference Notes

DO NOT:

- ✕ **Do NOT remove an embedded or impaled object** from the wound.
- ✕ **Do NOT lift or remove a blood-soaked dressing** — add layers on top.
- ✕ **Do NOT remove or loosen a tourniquet** once applied unless instructed by a paramedic or physician.
- ✕ **Do NOT loosen a tourniquet because the child is screaming** — pain is expected.
- ✕ **Do NOT give food or drink** to a child with severe bleeding or suspected shock.
- ✕ **Do NOT use wound packing for penetrating stab wounds.**
- ✕ **Do NOT enlarge wound openings** to facilitate wound packing.
- ✕ **Do NOT assume tachycardia is only from anxiety or pain** — suspect blood loss.
- ✕ **Do NOT wait for hypotension** to suspect shock in a child — it is a late, pre-arrest sign.
- ✕ **Do NOT delay calling 101** for severe or uncontrolled bleeding in a child.
- ✕ **Do NOT apply a tourniquet over a joint** — place 5–10 cm above the wound.
- ✕ **Do NOT use a manufactured tourniquet on a child under 2** — use sustained direct pressure.

Equipment needed:

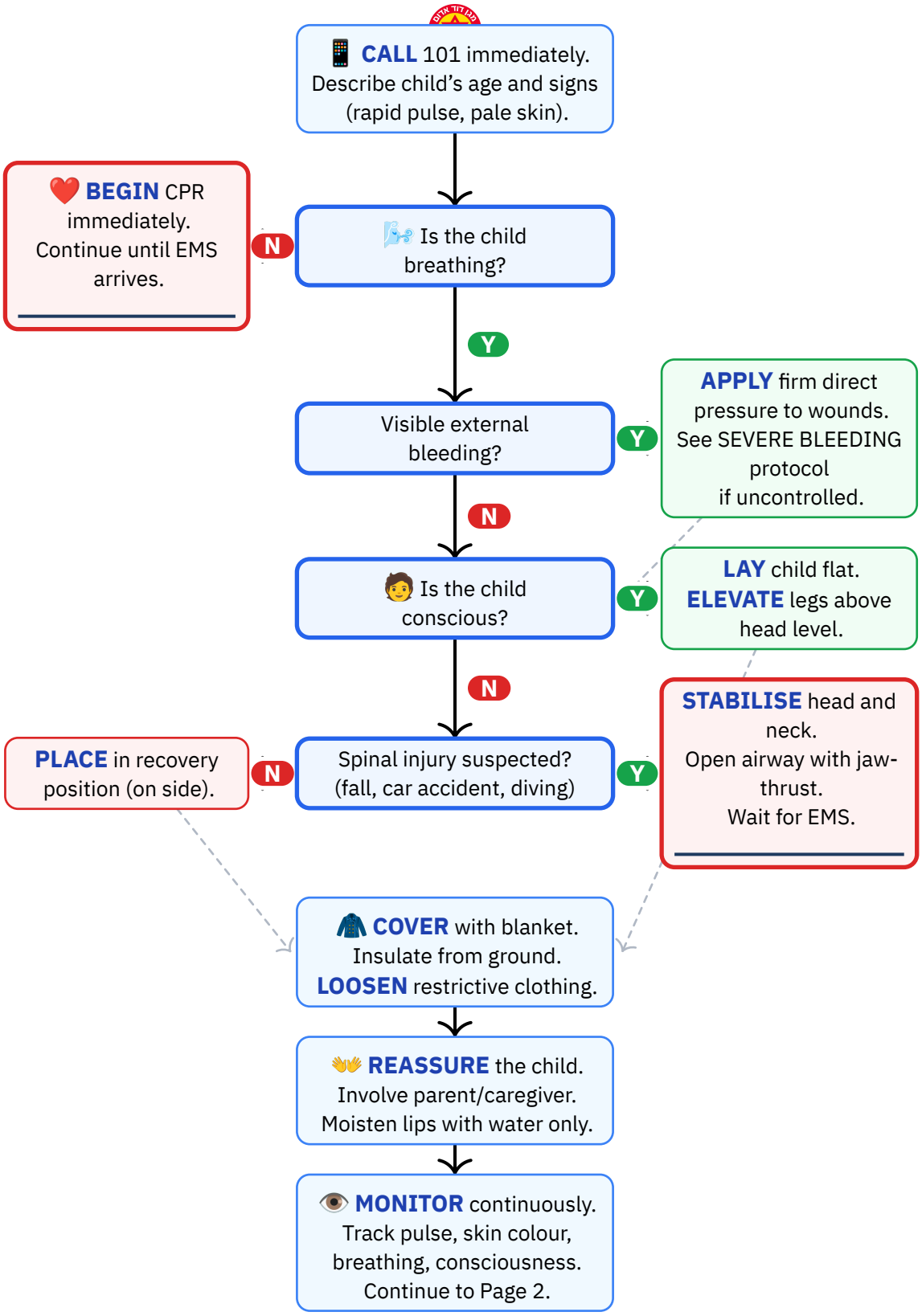
— Disposable gloves
— Sterile gauze or clean cloth
— Israeli Emergency Bandage (pressure dressing)
— Tourniquet (CAT Gen 7 — for children aged 2+ with limb circumference 13 cm+)
— Wound packing material (hemostatic gauze preferred)
— Blanket or coat (for shock management and warmth)
— Comfort item for the child (toy, stuffed animal) if available

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Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Bleeding	דימום	di-MUM
Tourniquet	חוסם עורקים	kho-SEM or-KIM
Bandage	תחבושת	takh-BO-shet
Pressure	לחץ	LA-khats
Wound	פצע	PE-tsa
Shock	הלם	HE-lem
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
Where is the bleeding?	איפה הדימום?	EI-fo ha-di-MUM?
The child is bleeding	הילד מדמם	ha-YE-led me-da-MEM
Apply pressure	תלחץ	til-KHATZ
Gloves	כפפות	kfa-FOT

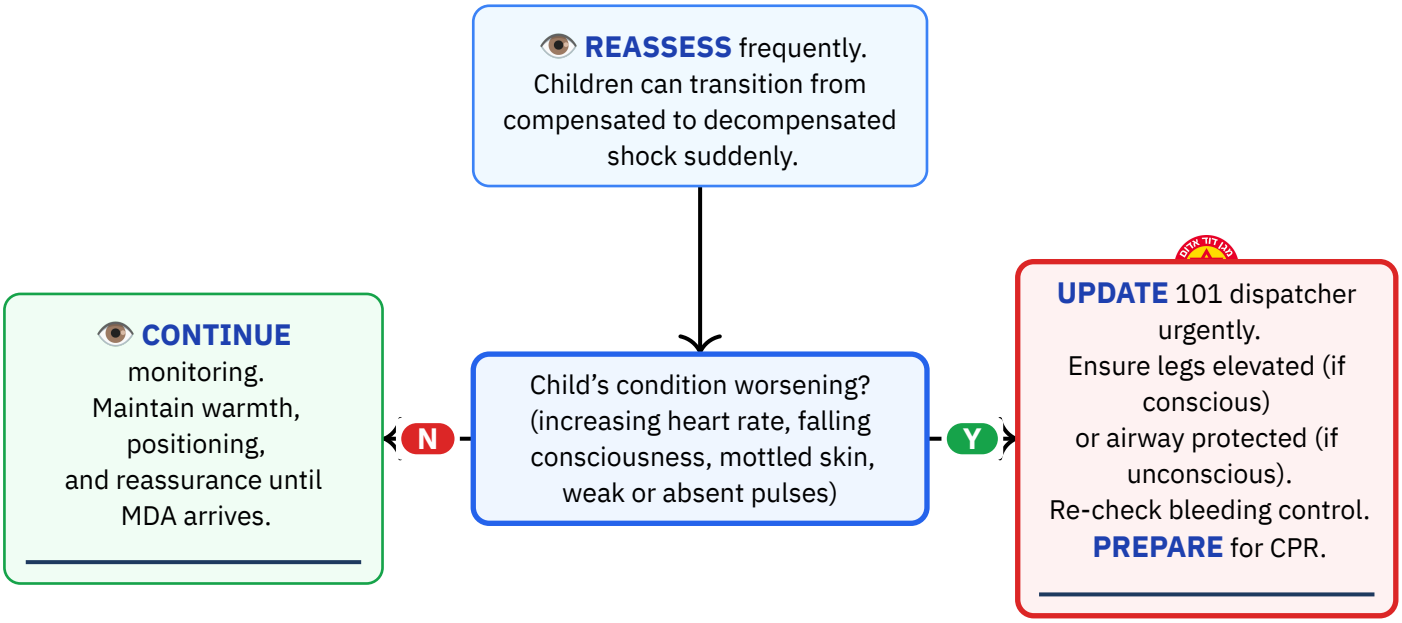
Source: Magen David Adom (MDA) — MDA 101 First Aid Public Guidance (Bleeding Control) — Pediatric Adaptation
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
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Continued Monitoring



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Reference Notes

DO NOT:

✕ **Do NOT sit the child upright** — this reduces blood flow to the brain and worsens shock.
✕ **Do NOT give the child anything to eat or drink** — moisten lips only.
✕ **Do NOT leave the child unattended** — continuous monitoring is essential due to risk of sudden deterioration.
✕ **Do NOT delay calling 101** — early hospital treatment with IV fluids is critical for survival.
✕ **Do NOT assume normal blood pressure means no shock** — children maintain BP through strong compensatory mechanisms until sudden decompensation.
✕ **Do NOT attribute tachycardia solely to anxiety, fear, or pain** — suspect shock until proven otherwise.
✕ **Do NOT move a child with suspected spinal injury** — stabilise and wait for EMS.
✕ **Do NOT remove blood-soaked dressings** — add additional layers on top.

Equipment needed:

— Blanket or coat (for warmth and insulation from ground)
— Sterile gauze or clean cloth (for bleeding control)
— Disposable gloves
— Pillow or folded blanket (for leg elevation)

Hebrew Terminology — מונחים בעברית

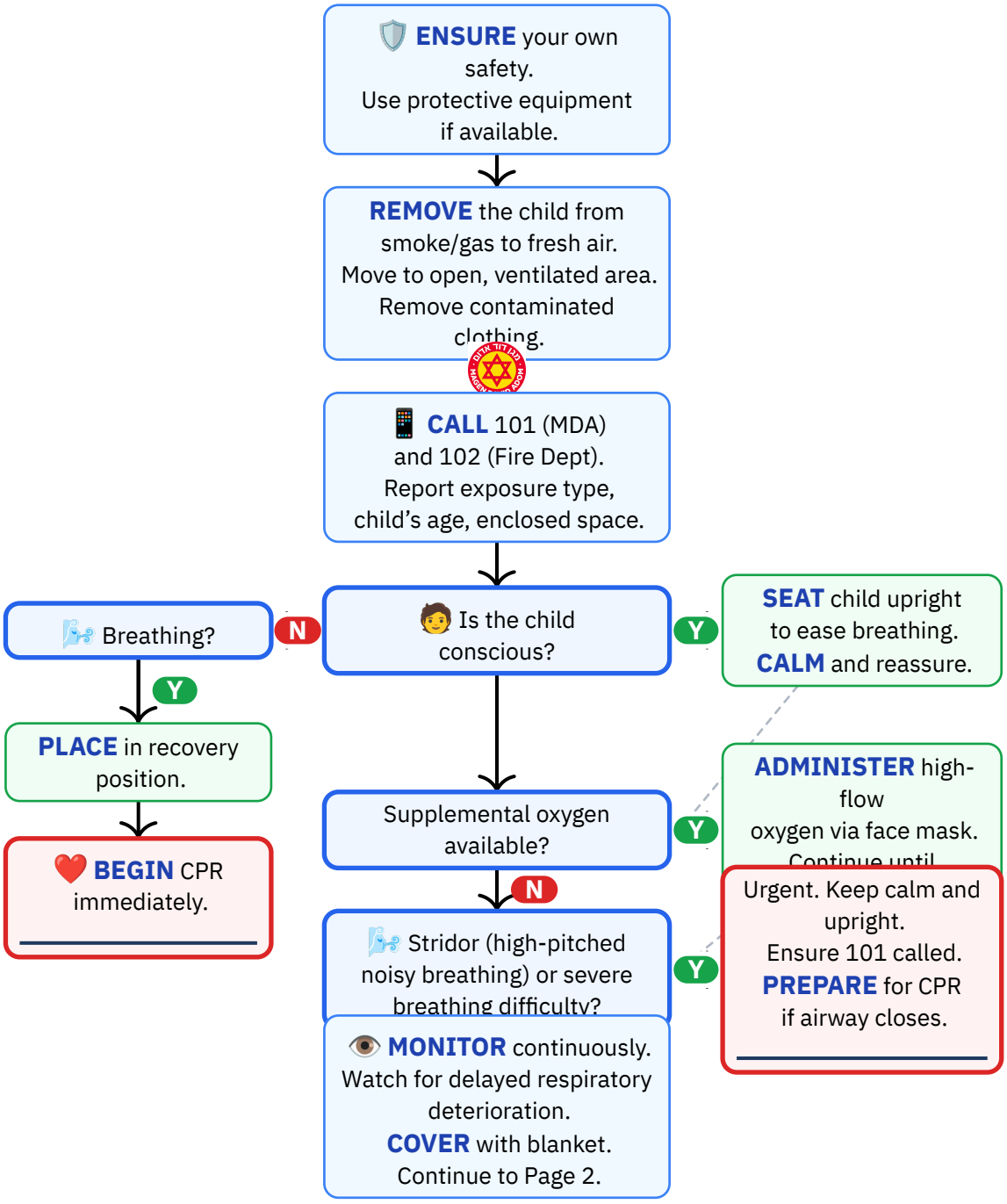
English	עברית	Pronunciation
Shock	הלם	HE-lem
Pulse / heart rate	דופק	DO-fek
Blood pressure	לחץ דם	LA-khats DAM
Pale	חיוור	khi-VER
Cold	קר	KAR
Breathing	נשימה	ne-shi-MA
Consciousness	הכרה	ha-ka-RA
Recovery position	תנוחת התאוששות	tnu-KHAT hit-o-she-SHUT
Blanket	שמיכה	smi-KHA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child is in shock	הילד בהלם	ha-YE-led be-HE-lem

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Source: Magen David Adom (MDA) — MDA BLS Guide (2016) and MDA 101 First Aid Public Guidance — Pediatric Shock
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-15

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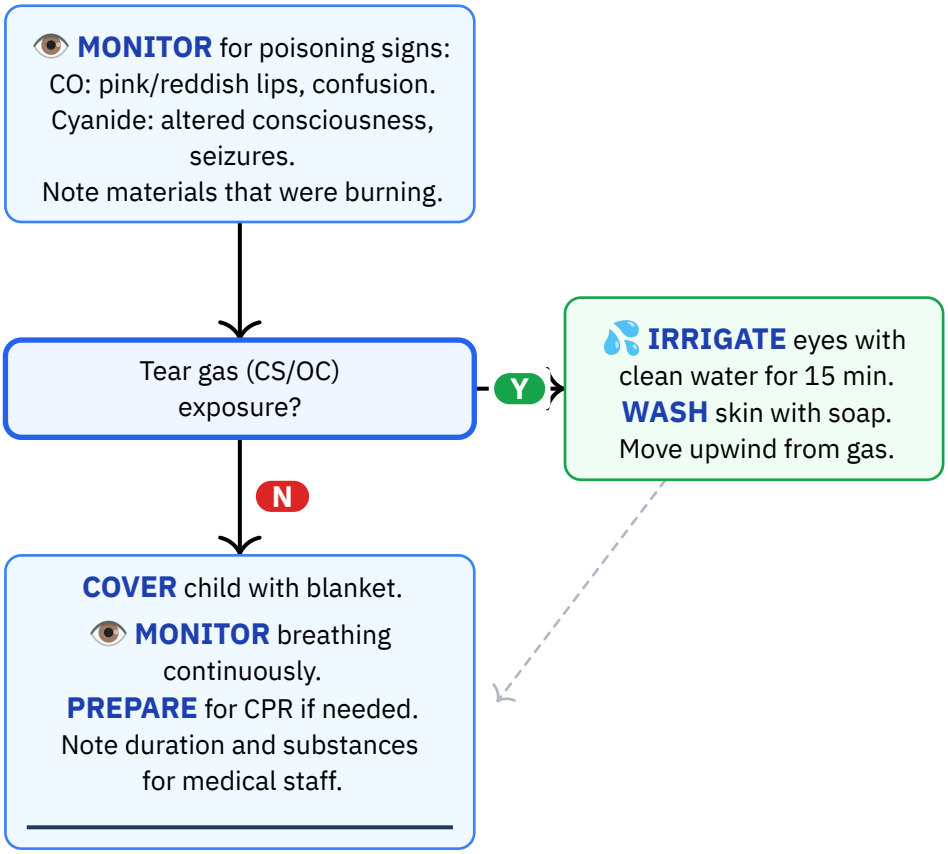
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Tear Gas Exposure & Poisoning Signs



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Reference Notes

DO NOT:

✕ **Do NOT re-enter a burning building or hazardous area** — wait for Fire Department rescue teams.
✕ **Do NOT ignore light smoke exposure in a child** — children should be assessed even with mild exposure.
✕ **Do NOT assume a well-appearing child is uninjured** — respiratory deterioration can be delayed by hours.
✕ **Do NOT rub the eyes after tear gas exposure** — this worsens irritation.
✕ **Do NOT give food or drink to a child with stridor or breathing difficulty** — aspiration risk.
✕ **Do NOT examine the throat of a child with stridor** — this can worsen airway swelling.
✕ **Do NOT leave the child unattended** — monitor breathing continuously.
✕ **Do NOT position a child with breathing difficulty lying flat** — keep upright.

Equipment needed:

— Supplemental oxygen and face mask (if available)
— Clean water (for eye irrigation after tear gas)
— Blanket (to prevent hypothermia)
— Cool mist humidifier (if available, for stridor management)
— Phone (to call 101 and 102)

Hebrew Terminology — מונחים בעברית

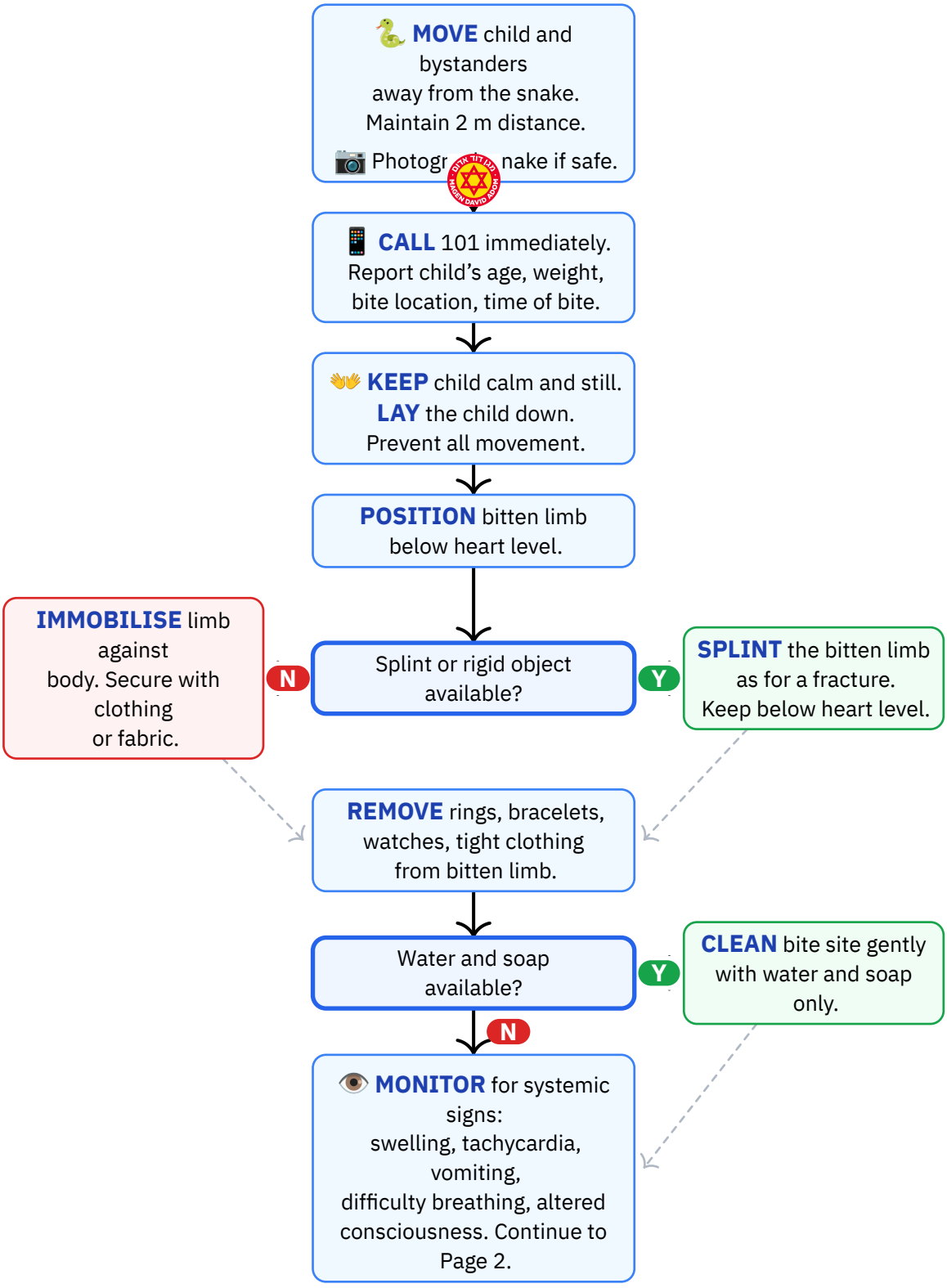
English	עברית	Pronunciation
Smoke	עשן	a-SHAN
Fire	שריפה	sre-FA
Tear gas	גז מדמיע	GAZ mad-MI-a
Hard to breathe	קשה לנשום	ka-SHE lin-SHOM
Oxygen	חמצן	kham-TSAN
Cough	שיעול	shi-UL
Stridor	סטרידור	STRI-dor
Fire department	כיבוי אש	ki-BUY ESH
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child inhaled smoke	הילד שאף עשן	ha-YE-led sha-AF a-SHAN
Can you breathe?	אתה יכול לנשום?	a-TA ya-KHOL lin-SHOM?

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Source: Magen David Adom (MDA) — MDA Smoke and Inhalation Injury First Aid Guidelines — Pediatric
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
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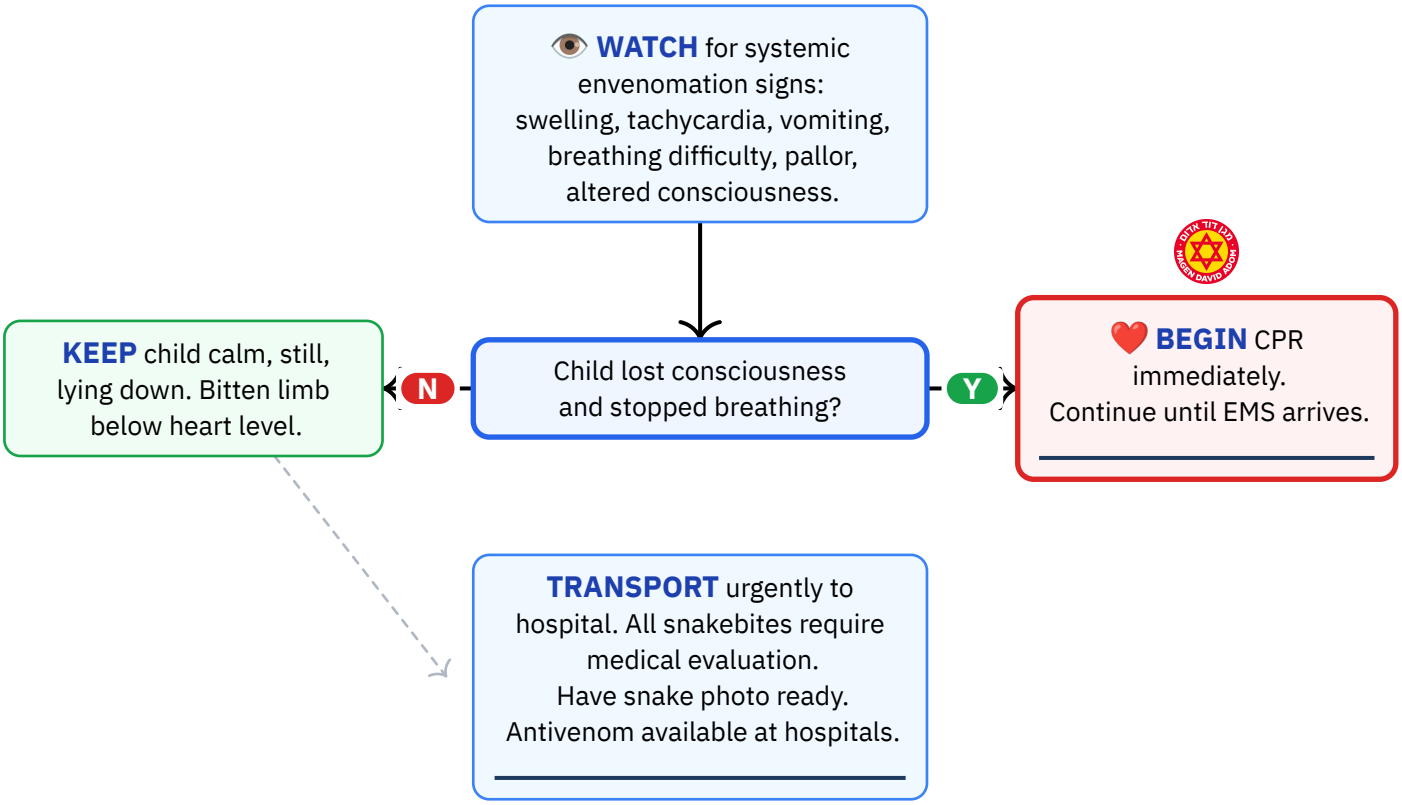
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Monitoring & Transport



Reference Notes

DO NOT:

- ✕ **Do NOT apply a tourniquet** — concentrates venom and causes tissue damage.
- ✕ **Do NOT cut the bite site** — causes additional injury and does not remove venom.
- ✕ **Do NOT attempt to suck out the venom** — ineffective and risks infection.
- ✕ **Do NOT apply ice or cold compresses** — can cause tissue damage and does not slow absorption.
- ✕ **Do NOT apply alcohol, antiseptic, or any substance** to the wound.
- ✕ **Do NOT give the child food or drink** — risk of vomiting; may need anaesthesia.
- ✕ **Do NOT chase, catch, kill, or handle the snake** — risk of additional bites.
- ✕ **Do NOT use a pressure immobilisation bandage** — not recommended for viper bites.
- ✕ **Do NOT allow the child to walk or move** — movement accelerates venom spread.
- ✕ **Do NOT wait for symptoms** before seeking emergency care.

Equipment needed:

— Splint or rigid object for limb immobilisation
— Water and soap (for wound cleaning)
— Phone (to call 101 and photograph the snake)
— Blanket or clothing (to keep child warm while lying still)

Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Snake	נחש	na-KHASH
Snakebite	הכשת נחש	ha-ka-SHAT na-KHASH
Viper	צפע	TSE-fa
Palestine viper	צפע ארצישראלי	TSE-fa er-ets yis-re-e-LI
Venom	ארס	E-res
Antivenom	נוגדן נגד ארס	nog-DAN NE-ged E-res
Swelling	נפיחות	ne-fi-KHUT
Splint	סד	SAD
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child was bitten by a snake	הילד הוכש על ידי נחש	ha-YE-led hu-KASH al ye-DEI na-KHASH
Keep still	תישאר במקום	ti-sha-ER ba-ma-KOM

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Source: Magen David Adom (MDA) — MDA Snakebite First Aid Guidelines — Pediatric
URL: https://www.mdais.org/101/snake-bite
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🕷️ **MOVE** child away from spider.
📷 Safely capture or photograph spider for ID.

↓

💦 **WASH** bite area with water and soap.

↓

❄️ **APPLY** cold compress wrapped in cloth, 10–15 min. Remove 10 min, reapply.

↓

IMMOBILISE bitten limb at heart level.
KEEP child still.

↓

👁️ Systemic symptoms? (severe pain, abdominal cramps, breathing difficulty, sweating, muscle cramps)

Y → 📱 **CALL** 101 immediately. Possible black widow bite. Keep child calm and still. **PREPARE** for transport.

N ↓

🕷️ Spider identified as or suspected Black Widow?

Y → 📱 **CALL** 101 immediately. Antivenom available at Israeli hospitals. **TRANSPORT** urgently.

N ↓

SEEK medical evaluation for all spider bites in children. Bring captured/photographed spider. First 6–8 hours are critical.

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Reference Notes

DO NOT:

✕ **Do NOT apply a tourniquet** — tourniquets worsen envenomation outcomes by concentrating venom.
✕ **Do NOT cut the bite site** — causes additional injury and does not remove venom.
✕ **Do NOT attempt to suck out the venom** — ineffective and risks infection.
✕ **Do NOT apply antiseptic, alcohol, or any substance** other than water and soap.
✕ **Do NOT bandage tightly** over the bite site.
✕ **Do NOT delay medical evaluation** for any spider bite in a child.
✕ **Do NOT ignore a painless bite** — brown recluse bites are often painless initially but develop serious necrotic wounds hours later.
✕ **Do NOT give the child medications** (painkillers, antihistamines) without medical direction.

Equipment needed:

— Water and soap (for wound cleaning)
— Ice or cold compress wrapped in cloth
— Container or glass (to safely capture the spider for identification)
— Phone (to call 101 and photograph the spider)

Venomous Spiders in Israel

Brown Recluse (shshan chum)
Small, brown, long legs
Lives indoors (wardrobes, shoes)
Bite barely felt initially
Effects appear hours later
Causes local necrotic wound

Black Widow (almana shchora)
Black with markings
Found outdoors (gardens, walls)
Immediate pain and burning
Rapid systemic symptoms
Female bite is dangerous
Antivenom available

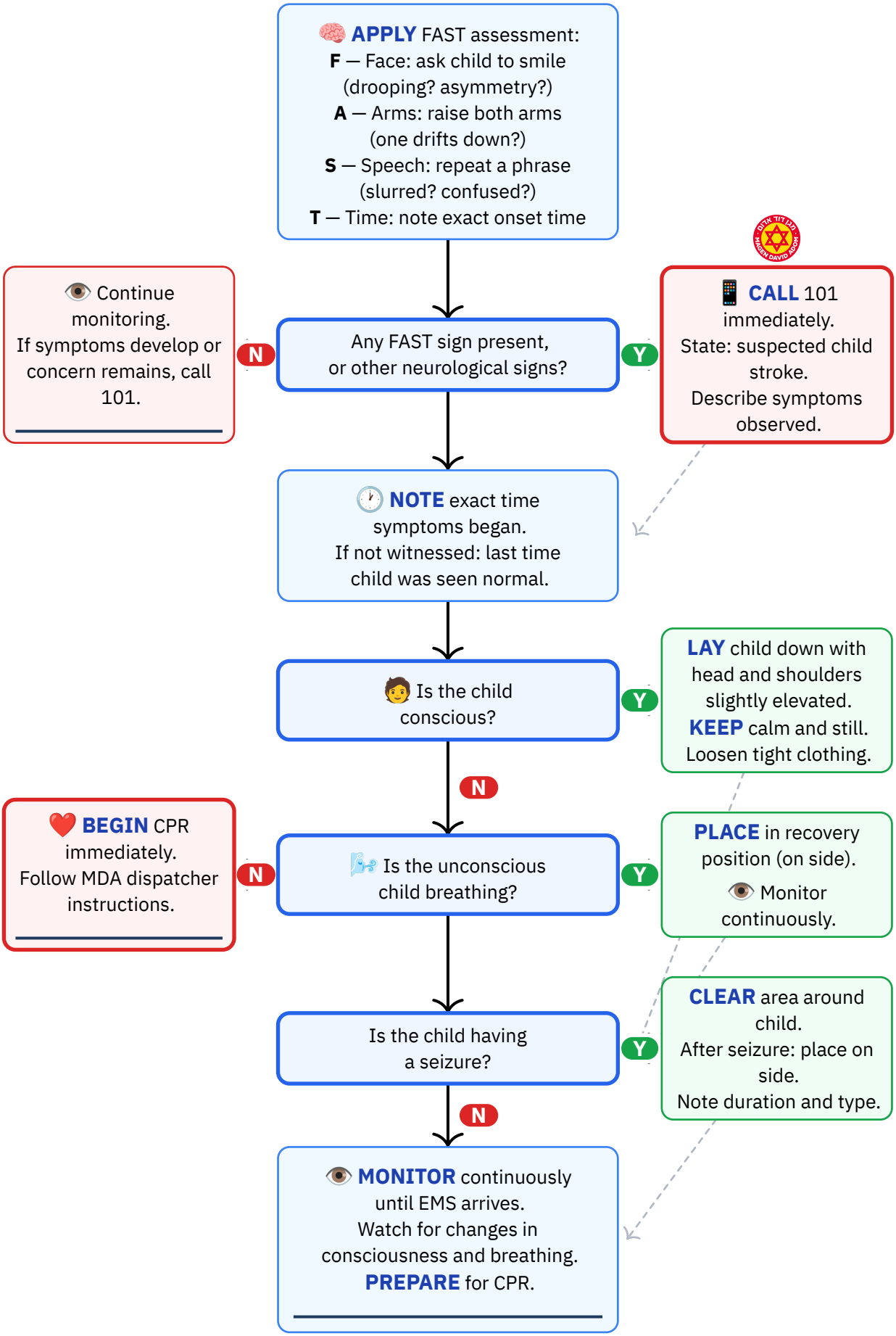
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Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Spider	עכביש	ak-a-VISH
Spider bite	עקיצת עכביש	a-ki-TSAT ak-a-VISH
Black widow	אלמנה שחורה	al-ma-NA shcho-RA
Brown recluse	שושן חום	sho-SHAN KHUM
Venom	ארס	E-res
Antivenom	נוגדן נגד ארס	nog-DAN NE-ged E-res
Swelling	נפיחות	ne-fi-KHUT
Pain	כאב	ke-EV
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child was bitten by a spider	הילד נעקץ על ידי עכביש	ha-YE-led ne-e-KATS al ye-DEI ak-a-VISH

Source: Magen David Adom (MDA) — MDA Spider Bite First Aid Guidelines — Pediatric
URL: https://www.mdais.org/101/first-aid
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🧠 APPLY FAST assessment:
F — Face: ask child to smile (drooping? asymmetry?)
A — Arms: raise both arms (one drifts down?)
S — Speech: repeat a phrase (slurred? confused?)
T — Time: note exact onset time
Any FAST sign present, or other neurological signs?
N
👁️ Continue monitoring. If symptoms develop or concern remains, call 101.
Y
📱 CALL 101 immediately. State: suspected child stroke. Describe symptoms observed.
🕐 NOTE exact time symptoms began. If not witnessed: last time child was seen normal.
🧒 Is the child conscious?
Y
LAY child down with head and shoulders slightly elevated. KEEP calm and still. Loosen tight clothing.
N
🌬️ Is the unconscious child breathing?
N
❤️ BEGIN CPR immediately. Follow MDA dispatcher instructions.
Y
PLACE in recovery position (on side). 👁️ Monitor continuously.
Is the child having a seizure?
Y
CLEAR area around child. After seizure: place on side. Note duration and type.
N
👁️ MONITOR continuously until EMS arrives. Watch for changes in consciousness and breathing. PREPARE for CPR.

Reference Notes

DO NOT:

✕ **Do NOT delay calling 101** to wait and see if symptoms resolve.
✕ **Do NOT give the child any food or drink** — stroke may impair swallowing reflex, creating aspiration and choking risk.
✕ **Do NOT give any medications, especially not aspirin** — if the stroke is haemorrhagic, aspirin will worsen it.
✕ **Do NOT drive the child to hospital yourself** unless MDA explicitly advises it. MDA ambulances begin treatment en route.
✕ **Do NOT assume a child cannot have a stroke** — children can have strokes at any age.
✕ **Do NOT restrain the child during a seizure.**
✕ **Do NOT put anything in the child's mouth** during a seizure.
✕ **Do NOT rely solely on FAST to rule out stroke in children** — adult screening tools are not validated for the pediatric population.

Pediatric Stroke — Key Differences from Adults

- Seizures at onset are more common in children than adults
- Symptoms may appear gradually rather than suddenly
- Very young children may show only irritability, feeding difficulty, or lethargy
- FAST assessment is not validated for the pediatric population
- Treatment window: alteplase effective within 4.5 hours (ages 28 days–18 years)
- Mechanical clot removal may be effective for ages 6+ within 6–24 hours

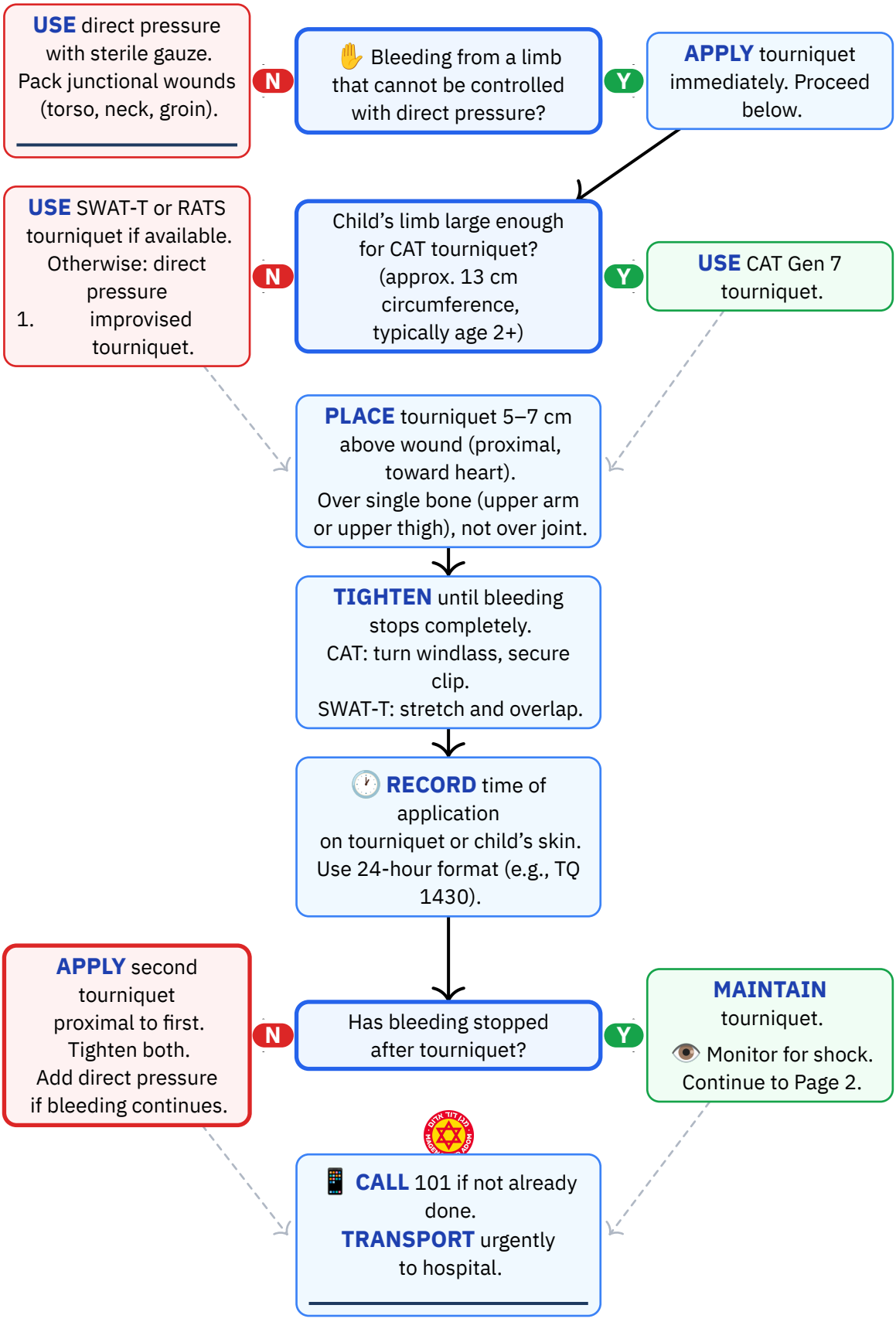
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Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Stroke	שבץ מוחי	SHEV-ets mo-KHI
Face	פנים	pa-NIM
Arm	זרוע	zro-A
Speech	דיבור	di-BUR
Seizure	פרכוס	pir-KUS
Headache	כאב ראש	ke-EV ROSH
Consciousness	הכרה	ha-ka-RA
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child may be having a stroke	ייתכן שהילד סובל משבץ	yi-ta-KHEN she-ha-YE-led so-VEL mi-SHEV-ets
Can you smile?	אתה יכול לחייך?	a-TA ya-KHOL le-kha-YEKH?
Raise both arms	תרים את שתי הידיים	ta-RIM et shTEI ha-ya-DA-yim

Source: Magen David Adom (MDA) — MDA Stroke Recognition Protocol (FAST) and AHA 2024 First Aid Guidelines — Pediatric Stroke Considerations
URL: https://www.mdais.org/101/stroke
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Monitoring & Transport

👁 **CHECK** tourniquet has not loosened.
CHECK bleeding has stopped.
MONITOR for shock: rapid weak pulse, pallor, cool mottled skin, altered consciousness.

↓

KEEP child warm to prevent hypothermia.
REASSURE child and parent.
Report time, device, and condition to arriving EMS.

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Reference Notes

DO NOT:

✕ **Do NOT remove a tourniquet once applied** — only hospital personnel should remove it.
✕ **Do NOT apply a tourniquet for venomous animal bites** (snakes, scorpions, spiders) — tourniquets worsen envenomation outcomes.
✕ **Do NOT apply a tourniquet over a joint** (elbow, knee) — place on upper arm or upper thigh.
✕ **Do NOT apply a tourniquet too loosely** — an insufficiently tight tourniquet causes venous bleeding without arterial occlusion, worsening blood loss.
✕ **Do NOT use a standard CAT tourniquet on very small infant limbs** (under 13 cm circumference) — it will not be effective.
✕ **Do NOT delay tourniquet application** to attempt prolonged direct pressure on massive haemorrhage.
✕ **Do NOT forget to record the time of application.**

Equipment needed:

— CAT (Combat Application Tourniquet) Gen 7 — for children aged ~2+ with sufficient limb size
— SWAT-T tourniquet — for all pediatric sizes including very small limbs
— RATS tourniquet — alternative for all pediatric sizes
— Improvised tourniquet materials (triangular bandage + rigid windlass object)
— Marker or pen (to record time of application)
— Sterile gauze (for wound packing if needed)
— Phone (to call 101)

Pediatric Tourniquet Sizing Guide

Age / Size	Device	Notes
Age 2+, limb 13 cm+	CAT Gen 7	Upper + lower extremity
Age 6+, limb 16 cm+	CAT Gen 7	Reliable both extremities
All pediatric sizes	SWAT-T / RATS	Effective on very small limbs
Infants < 2 years	Direct pressure	No evidence for manufactured TQ

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Hebrew Terminology — מונחים בעברית

English	עברית	Pronunciation
Tourniquet	חוסם עורקים	kho-SEM or-KIM
Bleeding	דימום	di-MUM
Tighten	הדק	ha-DEK
Windlass	מוט סיבוב	MOT si-BUV
Above the wound	מעל הפצע	me-AL ha-PE-tsa
Time of application	זמן הנחה	ZMAN ha-na-KHA
Shock	הלם	HE-lem
Ambulance	אמבולנס	am-bu-LANS
Emergency	חירום	khe-RUM
The child needs a tourniquet	הילד צריך חוסם עורקים	ha-YE-led tsa-RIKH kho-SEM or-KIM
The bleeding won’t stop	הדימום לא נעצר	ha-di-MUM LO ne-e-TSAR

Source: Magen David Adom (MDA) — MDA Tourniquet Application Guidelines — Pediatric
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